Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

3/13/2026 19:23 Joseph Wong, MD Note Discipline: Physician Tag: History & Physical

Form: Rehab History and Physical [PHP]

Reason for Admission

Rehab Diagnosis:

Vertigo

Date of Onset

3/12/2026

Etiologic Diagnosis-Impairment

bpppv

Secondary Impairments and Complications

History

History of Present Illness

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

Comorbidities

- [ ] Cellulitis
- [ ] C. Difficile
- [ ] Pseudomonas
- [ ] UTI

Other Infection:

- [ ] Diabetic Neuropathy
- [ ] Diabetic Myopathy
- [ ] Acute Kidney Failure
- [ ] Chronic Kidney Failure
- [ ] Aspiration Pneumonia
- [ ] Pneumonitis
- [ ] Pneumonia
- [ ] BMI >40
- [ ] Dysphagia
- [ ] Hemiparesis
- [ ] Diastolic Heart Failure
- [ ] Systolic Heart Failure

Other:

Impairment Group Code:

Brain Dysfunction

Subjective/Chief Complaint

- [ ] Patient's Own Words
- [ ] Non-Verbal
- [ ] Caregiver

Other:

No acute events overnight. Denies headache at present. Acute rehab expectations reviewed.

Denies fever/chills, chest pain, SOB, cough, nausea, vomiting, diarrhea, constipation, dysuria, new weakness or changes in sensation. Complete ROS negative unless mentioned above.

Interpreter required:

- [ ] Yes
- [x] No

Language: English

Other:

History of Immunizations

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- [ ] DPT
- [ ] MMR
- [ ] Hepatitis B

Tetanus: _____

- [ ] Influenza

Influenza last given: _____

Pneumococcal: _____

Past Medical History

Add comments to any medical history that needs further details

- [x] None
- [ ] AIDS/HIV
- [ ] A-Fib
- [ ] Allergic Rhinitis
- [ ] Alzheimer's
- [ ] Anemia
- [ ] Anxiety
- [ ] Arthritis (Osteo)
- [ ] Arthritis (Rheumatoid)
- [ ] Benign Prostate Hypertrophy
- [ ] CAD
- [ ] Cancer
- [ ] CHF
- [ ] Chronic Back Pain
- [ ] Cirrhosis
- [ ] Compression Fracture
- [ ] Dementia
- [ ] DJD
- [ ] Diabetes
- [ ] Diverticulitis/losis
- [ ] Depression
- [ ] Deep Vein Thrombosis
- [ ] Epilepsy
- [ ] GERD
- [ ] Glaucoma
- [ ] GI Bleed
- [ ] Gout
- [ ] Heart Disease
- [ ] Hiatal Hernia
- [ ] HTN
- [ ] Hyperlipidemia
- [ ] Kidney/Bladder
- [ ] Liver Disease
- [ ] Lung Disease
- [ ] Macular Degeneration
- [ ] MI (Not Primary Med Condtn)
- [ ] Obesity
- [ ] Orthopedic
- [ ] Osteoporosis
- [ ] Peptic Ulcer
- [ ] Peripheral Vascular Disease
- [ ] Pulmonary Embolis
- [ ] Seizures
- [ ] Sleep Apnea
- [ ] Stroke
- [ ] TB History
- [ ] Thyroid Disease
- [ ] TIA

Other Medical History: _____

Additional Comments: _____

Past Surgical History

Add comments to any surgical history that needs further details

- [x] None
- [ ] AICD
- [ ] Amputation(s)
- [ ] Anesthesia difficulties
- [ ] Angioplasty
- [ ] Appendectomy
- [ ] Biopsy (site)
- [ ] Bronchoscopy
- [ ] CABG
- [ ] Carotid (CEA)
- [ ] Carpal Tunnel
- [ ] Cataract(s)
- [ ] Cholecystectomy
- [ ] Craniotomy
- [ ] Esophageal Surgery
- [ ] Gastric Surgeries (Type)
- [ ] Heart Valve Surgery
- [ ] Hemorrhoidectomy
- [ ] Hernia Repair (location)
- [ ] Hysterectomy
- [ ] Joint Replacement (location)
- [ ] Lithotripsy
- [ ] Mastectomy (R/L/B)
- [ ] Orthopedic
- [ ] Ostomies (Describe)

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- [ ] Pacemaker
- [ ] PEG
- [ ] Pneumonectomy (lobes)
- [ ] Spinal Surgery
- [ ] Stent Placement (location)
- [ ] Tonsillectomy
- [ ] Thyroid Surgery
- [ ] Tracheostomy

Other Surgical History:

Additional Comments:

Family Medical History

Have any of these conditions occurred on either side of the patient's family? (Check if yes)

- [x] Not applicable
- [ ] Allergies
- [ ] Cardiovascular disease
- [ ] Gastrointestinal disease
- [ ] Pulmonary disease
- [ ] Bleeding tendencies
- [ ] Tuberculosis
- [ ] Psychiatric illness
- [ ] Bone disease
- [ ] Kidney disease
- [ ] Congenital deformities
- [ ] Alcohol/drug dependence
- [ ] Cancer/Tumors
- [ ] Diabetes

Other:

Comments:

Social History:

Social History: no alcohol

Comments:

Marital Status

Marital Status: Widowed

Current or past use of alcohol?

- ( ) Yes
- (•) No

Recreational/Illicit Drugs

- [x] Denies Use
- [ ] Past History of Use
- [ ] Current Use

Type: Frequency:

Date last used:

Tobacco/Smoking

- [ ] Current Smoker Packs/day: How many years:
- [x] Non-Smoker

Previous History of Smoking (indicate last time used and previous PPD):

- [ ] Vapes Vapes Frequency:
- [ ] Marijuana Marijuana Frequency:

Other:

Allergies

No Known Drug Intolerances

- [x] Physician Reviewed Allergies
- [ ] Physician Unable to Review
- [ ] None Known

Medications

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

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atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

Review of Systems

Allergies:

- ☐ No Known Allergies
- ☐ Allergy Unspecified
- ☐ Desensitization to Allergies
- ☐ Hx-Penicillin
- ☐ Hx-Antibiotic NEC
- ☐ Hx-Sulfonamides
- ☐ Hx-Anti-Infectives
- ☐ Hx-Anesthetic
- ☐ Hx-Narcotic
- ☐ Hx-Analgesic
- ☐ Hx-Vaccine
- ☐ Hx-Drug NEC
- ☐ Hx-Peanut
- ☐ Hx-Milk Product
- ☐ Hx-Eggs
- ☐ Hx-Seafood
- ☐ Hx-Insects
- ☐ Hx-Latex
- ☐ Hx-Allergy NEC
- ☐ Hx-Other Non-Drug
- ☐ Hx-Drug NOS
- ☐ Hx-Other Drug

Other Allergy/Comments: _____

Review of Systems

_____

Vision:

- ☐ Negative
- ☐ Vision change
- ☐ Glasses/Contacts
- ☐ Cataracts
- ☐ Glaucoma
- ☐ Macular degeneration
- ☐ Legally blind

Other/Comments: _____

Hearing:

- ☐ Negative
- ☐ Hearing loss
- ☐ Hearing aides
- ☐ Other amplification device

Other/Comments: _____

Cardiovascular:

- ☐ Negative
- ☐ Shortness of Breath
- ☐ Orthopnea
- ☐ Dyspnea - Cardiac Etiology
- ☐ Dyspnea on exertion
- ☐ Supplemental O2 > 40%
- ☐ Chest pain
- ☐ Edema
- ☐ Palpitation
- ☐ Pacemaker
- ☐ AICD
- ☐ Arrhythmias
- ☐ Syncope
- ☐ Uncontrolled Hypertension
- ☐ Ejection Fraction < 40%
- ☐ O2 How many liters of O2: _____

Other/Comments: _____

Pulmonary:

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☐ Negative

BiPAP/CPAP:

○ Yes ○ No

BiPAP/CPAP Settings:

BiPAP I/E: _____ % of Oxygen

CPAP Settings: _____ % of Oxygen

Trach Airway

☐ N/A

Trach Type: _____ Trach Size: ____

Cuff Pressure (cm H2O): _____ Cuff Pressure (cc H2O): _____

Placement Date: _____ Down Sized/Change Date: _____

☐ No Cap ☐ Capped ☐ PMV

Gastrointestinal:

☐ Negative

☐ Nausea ☐ Vomiting ☐ Colostomy ☐ Ileostomy

Other/Comments: _____

H0400. Bowel Continence: Pre-Hospital

H0400. Bowel Continence: Current

Medication Related to Bowel Incontinence:

☐ Yes ☐ No

If yes, Medication type: _____

Date of Last BM:

_____ ☐ Prior to admission

Genitourinary:

☐ Foley Catheter

☐ Accidents

☐ UTI

☐ Intermittent Cath

☐ Urinary Retention

H0350. Bladder Continence: Pre-Hospital

H0350. Bladder Continence: Current

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Medication related to Bladder Incontinence:

☐ Yes ☐ No

If yes, Medication type: _____

Dialysis

☐ Not Applicable

☐ Established Dialysis ☐ New Dialysis Patient ☐ Hemodialysis ☐ Peritoneal Dialysis

Nephrologist: _____

Outpatient Dialysis Center: _____

☐ Dialysis at Home

What is Dialysis Schedule: _____

Musculoskeletal:

☐ Negative

_____

Skin:

☐ Negative

☐ Burns ☐ Incisions

☐ Wound ☐ Rash

☐ Ulcers Stages: _____

locations: _____

☐ Burns

Dressing Changes, etc: _____

☐ Dry skin

Other/Comments: _____

Neuro: Cognitive

☐ Alert ☐ Combative ☐ Confused ☐ Lethargic

☐ Nonresponsive ☐ Recognizes ☐ Short term memory deficit ☐ Vegetative/Comatose

Need for Low Stimulation or 1:1

_____

Neuro: Motor

☐ Follow Complex Commands ☐ Follows Simple Commands ☐ Withdraws from pain ☐ Spontaneous movement

☐ Posturing

Neuro: Verbal

☐ Appropriate ☐ Inappropriate ☐ Words/Sounds ☐ Sentences

☐ Nonverbal ☐ Receptive aphasia ☐ Expressive aphasia ☐ Follows 1-2 Stage Commands

Neuro Comments:

_____

Psych-Social Comments:

☐ Negative

☐ Crying ☐ Depression ☐ Severe anxiety

Other/Comments: _____

Endocrine:

☐ Negative

☐ Diabetes ☐ Hypothyroid ☐ Hyperthyroid ☐ Hot flashes

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- [ ] Hair loss
- [ ] Hirsutism
- [ ] Heat/Cold intolerance
- [ ] Hyperlipidemia

Other/Comments: _____

Bariatric Needs:

- [ ] None

Isolation Precautions:

- [ ] None

Anticoagulation:

- [ ] None
- [ ] Heparin
- [ ] Coumadin
- [ ] Currently Subtherapeutic
- [ ] Currently Therapeutic

Weight Bearing Status - Left Arm

- ○ Full
- ○ Partial
- ○ Touch Down/Toe Touch
- ○ WBAT
- ○ No Weight Bearing

Partial %: _____

Weight Bearing Status - Right Arm

- ○ Full
- ○ Partial
- ○ Touch Down/Toe Touch
- ○ WBAT
- ○ No Weight Bearing

Partial %: _____

Weight Bearing Status - Left Leg

- ○ Full
- ○ Partial
- ○ Touch Down/Toe Touch
- ○ WBAT
- ○ No Weight Bearing

Partial %: _____

Weight Bearing Status - Right Leg

- ○ Full
- ○ Partial
- ○ Touch Down/Toe Touch
- ○ WBAT
- ○ No Weight Bearing

Partial %: _____

Other/Notes:

Physical Exam

Blood Pressure:

Systolic (mmHg): 131 Diastolic (mmHg): 53 Position: Lying Side: Left

Comment: _____

Pulse:

B/min: 72 Position: Lying Site: Monitor Type: Irregular

Comment: _____

Respiration:

Per Minute: 18 Vent: No

Comment: _____

Weight:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

lb: 183.0 Scale: Bed
Comment:

Height:

Feet: 5 Inches: 1
Comment:

Physical Exam

General:

- [x] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments:

Skin:

- [x] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments:

- [ ] Abnormal

Abnormal findings:

HEENT:

- [x] Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: mm dry

- [ ] Abnormal

Abnormal findings:

Neck:

- [x] Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments:

- [ ] Abnormal

Abnormal findings:

Heart:

- [x] Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments:

- [ ] Abnormal

Abnormal findings:

Lungs/Chest:

- [x] Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments:

- [ ] Abnormal

Abnormal findings:

Abdomen:

- [ ] Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments:

- [x] Abnormal

Abnormal findings: obese
soft ntnd

Extremities:

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- [x] Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Back:

- [x] Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

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Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Gait and station are normal.

- [ ] Normal
- [x] Abnormal

Other:

fluent speech, follows commands
bilateral UE 5/5
bilateral LE 5-/5

Motor Functions: Romberg test is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Muscle tone is normal. No abnormal movements.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Plantar responses are down going.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Sensory: Sensory examination to pinprick and vibration is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed

Comment/Non-interfaced results:

Diagnostic Imaging Reviewed

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Comment/Non-interfaced results:

Microbiology Tests Reviewed

Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Medical Conditions that Require Daily Physician Oversight and Nursing Assessment

Medical Conditions/Risks for Clinical Complications that Cause Need for Hospital Level of Care

A rehabilitation physician, or other licensed treating physician with specialized training and experience in inpatient rehabilitation, will conduct face-to-face visits with the patient a minimum of at least 3 days per week throughout the patient's stay. The purpose of this is to assess the patient both medically and functionally, as well as to modify the course of treatment as needed to maximize the patient's capacity to benefit from the rehabilitation process.

- [ ] Infectious disease requiring monitoring, treatment, and isolation
- [ ] Frequency of wound assessment needs, ongoing intervention and evaluation to optimize wound healing and prevent potential or worsening infection
- [ ] Acute and/or chronic renal insufficiency which may require frequent monitoring of BP parameters, daily labs, hydration status, renal dose adjusted medications, fluid and electrolyte balance management
- [x] Fall risk due to balance deficits, decreased strength and mobility, and poor safety awareness
- [x] Bowel and/or bladder monitoring due to UTI risk; need for supervised voiding schedules
- [ ] Urinary retention requiring close monitoring of residuals, use of bladder scanner, intermittent catheterization as needed
- [ ] Seizure risk requiring close monitoring of seizure activity, worsening mental status, brain injury, change in respiratory status and medication management
- [ ] Extension of Stroke requiring close monitoring of blood pressure/ hear rate parameters, changes in cognition, dysphagia/aspiration, and risk for bleeding
- [ ] Cardiac Arrhythmias requiring close monitoring and medication regiment adjustment as needed
- [ ] Congestive Heart Failure requiring close monitoring of blood pressure, heart rate, weight and fluid balance, and oxygen saturation rates
- [ ] Respiratory complications requiring ongoing monitoring and assessment of supplemental O2 need, titration protocols, monitoring of spo2 saturation, RR and VS, imaging, SOB and/or confusion r/t acute or chronic condition
- [ ] Pulmonary Embolism requiring continued monitoring, anticoagulation treatment and/or bridge treatment regimen. Major potential clinical complications include recurrent thromboembolism, chronic thromboembolic pulmonary hypertension, right heart failure and cardiogenic shock
- [ ] Nutritional Needs including close monitoring of nutritional status, PO or enteral intake, routine calorie count, body weight assessment, vitamin sufficiency, evaluation of supplement need, loss of appetite, recent weight loss, and/or diagnosis of protein calorie malnutrition
- [ ] Post-Operative Anemia which may require frequent lab monitoring, enriched diet, use of supplements if applicable, and transfusion protocols for correction
- [x] Electrolyte Imbalance monitoring of fluid intake, need for IV fluids, medications or supplement protocols to replace lost electrolytes
- [ ] Dysphagia/Risk of Aspiration that requires assessment and monitoring of dietary consistency by certified specialist, maintaining aspiration precautions, assessment of feeding tube placement and frequent residual checks, routine oral care, lung auscultation, and dehydration status
- [ ] Diabetes Mellitus that requires frequent monitoring of blood sugar and potential use SSI to treat high levels of glucose and avoid potential DKA, neuropathy, and delayed healing
- [ ] Hypo/Hyperglycemia monitoring which includes frequent blood sugar monitoring, recognizing signs and symptoms of low/high blood glucose and its associated risk factors and initiating protocol for treatment to avoid rapid decline, loss of conscious, confusion and/or coma state
- [x] Hypertension/ Hypotension requiring frequent monitoring of BP parameters and ongoing medication management to avoid potential risks to include crisis, stroke, MI, syncopal episodes and falls
- [x] Pain Management requiring frequent pain assessment, pharmacologic/non- pharmacologic management, change in mobility, decreased PO intake, constipation, electrolyte imbalance, and BP/VS fluctuations.

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- [x] Anticoagulation Protocols to prevent deep vein thrombosis/ pulmonary embolism, stroke, MI, and lower extremity edema.
- [ ] Encephalopathy with continued monitoring for changes in/ worsening mental status, behaviors, LOC, seizure activity, and infection
- [ ] Morbid obesity with continued monitoring for skin breakdown/non- healing wounds, SOB, stroke/MI, and OSA.

Other: _____

Functional Abilities and Goals

Functional Abilities and Goals

	Eating	Toileting Hygiene	Shower/Bathing	Upper Body Dressing	Lower Body Dressing
Prior Level of Function	Independent	Independent	Independent	Independent	Independent
Current Level of Function	Independent	Partial/Moderate Assistance	Partial/Moderate Assistance	Partial/Moderate Assistance	Partial/Moderate Assistance

Functional Abilities and Goals (Cont.)

	Putting On/Taking Off Shoes	Sit to Lying	Lying to Sitting	Sit to Stand	Chair/Bed-to-Chair Transfer	Toilet Transfer
Prior Level of Function	Independent	Independent	Independent	Independent	Independent	Independent
Current Level of Function	Partial/Moderate Assistance	Partial/Moderate Assistance	Partial/Moderate Assistance	Partial/Moderate Assistance	Partial/Moderate Assistance	Partial/Moderate Assistance

Functional Abilities and Goals (Cont.)

	Ambulation
Prior Level of Function	Independent
Current Level of Function	Partial/Moderate Assistance

Functional Abilities Comments

Fall risk,

Distance:

Pre-Morbid/Prior LOF: _____

Now/Current: _____

Goal: _____

Strength Testing Evaluation

	Strength
Upper Right Extremity	Within Functional Limits
Upper Left Extremity	Within Functional Limits
Lower Right Extremity	Within Functional Limits
Lower Left Extremity	Within Functional Limits

Strength Testing Evaluation Comments

_____

Prior Device Use - Indicate devices/aids used prior to current illness/exacerbation/injury:

- [ ] None
- [ ] Manual wheelchair
- [ ] Motorized wheelchair and/or scooter
- [ ] Mechanical lift
- [ ] Orthotics/Prosthetics
- [ ] Crutches
- [x] Cane
- [ ] Hemi walker
- [ ] Walker

Type: SPC

Acute Therapy Comments

PT: _____

OT: _____

ST/Swallow: _____

Other: _____

History of Falls

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- [ ] No
- [x] Yes
- [ ] Unknown

Fall History Comments: _____

Other Safety Issues: _____

Anticipated Functional Outcomes: Self Care/Mobility

Self Care	Mobility
Independent	Modified Independent

Self Care/Mobility Comments

_____

General Rehabilitation Goals and Needs

Intensive Rehabilitation Nursing:

- [x] ADLs
- [x] Family Training/Education
- [x] Safety
- [x] Bladder Management
- [x] Medication Management
- [ ] Skin Integrity
- [x] Bowel Management
- [ ] Nutritional Deficits
- [ ] Nutrition
- [ ] Cognition
- [x] Pain Management
- [x] Transfers
- [ ] Communication
- [ ] Patient Education
- [ ] Wound, Skin or Stump Care
- [ ] Diabetes Management
- [x] Postioning
- [ ] Spasticity
- [ ] Disease Management
- [ ] Respiratory/Airway Management
- [ ] Cardiac Conditions
- [ ] Spinal orthosis or stabilization appliance use
- [ ] Spinal or post-surgical precautions
- [ ] Medically Complex Conditions
- [ ] Post-op Complications
- [ ] Coping Skills or Adjustment to Functional Loss
- [ ] Care Coordination needed and will be initiated on admission

Appropriate Therapy Needs:

The patient requires active and ongoing therapeutic interventions of at least two therapy disciplines (physical therapy and/or occupational therapy) in addition to Speech-Language Pathology or Prosthetic/Orthotics rehabilitation and training. REQUIRES TWO OR MORE THERAPIES:

- [x] Physical Therapy to see patient 0.5 - 1.5 hours per day, minimum of 5 days per week to address:
- [x] Occupational Therapy to see patient 0.5 - 1.5 hours per day, minimum of 5 days per week to address:
- [ ] Orthotics/Prosthetics to see patient 0.5 - 1.5 hours per day, minimum of 5 days per week to address:
- [ ] Speech and Language Pathology see patient 0.5 - 1 hours per day, 3-5 days a week to address:

Physical Therapy:

- [x] Physical Therapy to see patient 0.5 - 1.5 hours per day, minimum of 5 days per week to address:
- [x] Gait Training
- [ ] Pathfinding Activities
- [ ] Adaptive Equipment Training
- [x] Transfer Training
- [ ] Strengthening Neuro Re-Ed
- [x] Balance Training
- [ ] Wheelchair Mgt.
- [x] Monitoring of BP and O2 sats during/after functional activity
- [x] Community Reintegration

Occupational Therapy:

- [x] Occupational Therapy to see patient 0.5 - 1.5 hours per day, minimum of 5 days per week to address:
- [x] ADL Training
- [ ] Cognitive Training
- [ ] Adaptive Equipment Training
- [x] Transfer Training
- [ ] Compensatory Training
- [ ] IADLs
- [ ] Vision Therapy
- [ ] Fine Motor/Dexterity

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] Monitoring of BP and O2 sats during/after functional activity
- [x] Community Reintegration

Estimated LOS and Potential Discharge Location

Anticipated Length of Stay

10-12 days

Assessment/Plan

Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist

HTN
- lisinopril

HLD
- on statin

Skin
- monitor per nursing protocol for decubitus ulcers
- frequent offloading as indicated

Bowel - constipation
- colace/senna
- lactulose, dulcolax suppository PRN

Bladder
- monitor for retention
- check PVRs

DVT ppx
- SCDs

Post-Admission Physician Evaluation:
The patient has the potential to make improvement and is in need of physical, occupational, and/or speech therapy services. The patient may also need nutritional services. Given the patient's complex medical condition and risk of further medical complications, rehabilitative services cannot be safely provided at a lower level of care, such as a skilled nursing facility. I have reviewed the patient's functional and medical status at the time of the preadmission screening and they are the same as on the day of this admission. I acknowledge that I have personally performed a full physical examination on this patient within 24 hours of admission. The patient and/or family demonstrated understanding the rehabilitation program and the discharge process after we discussed them.

Agree in entirety: yes
Minor adaptions: none
Major changes: none

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Great than 50 minutes was spent on plan of care, including face to face discussion with patient.

Rehab Prognosis:

Medicare requires documentation beyond just a statement of "Good/Fair/Poor".
Provide detailed explanation below as to "why" a patient's potential is "Good/Fair/Poor".

- (●) Good (anticipate managing medical conditions without interruption to intensive therapy plan)
- ( ) Fair (special prophylaxis required to mitigate elevated risks)
- ( ) Poor (High risk of unavoidable complications during this stay)

Medical Prognosis:

Medicare requires documentation beyond just a statement of "Good/Fair/Poor".
Provide detailed explanation below as to "why" a patient's potential is "Good/Fair/Poor".

- ( ) Good (anticipate managing medical conditions without interruption to intensive therapy plan)
- (●) Fair (special prophylaxis required to mitigate elevated risks)
- ( ) Poor (High risk of unavoidable complications during this stay)

Rehab/Medical Prognosis - Explain rationale for admission despite risks:

Anticipate managing medical risks

Anticipated Discharge Location

Home with Home Health

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

Physician Addendum

Electronically Signed By:

Joseph Wong, MD, 03/14/2026 08:24

3/13/2026	21:03	Peter Campanella, PharmD	Note Discipline: Pharmacist	Tag: Admission

On 03/13/2026 approx 2103, I messaged Dr Hasson, "Hello Dr Hasson. There is a new pt admit at PHX - Pt is currently on her way to hospital.
Pt Phillips, Priscila R . Is it ok to continue all of the pt's medications on the depart med rec?"

On 03/13/2026 approx 2103, Dr Hasson replied, "Sure"

Medications from the pt's depart med rec were entered and activated.

Electronically Signed By:

Peter Campanella, PharmD, 03/13/2026 21:05

3/14/2026	7:16	Chelsea Olson, OT	Note Discipline: Occupational Therapist	Tag: Therapy Evaluation

Form: OT Initial Evaluation [P12]

Session Time

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Session Time for Evaluation

Time In: 07:30 AM
Time Out: 09:00 AM
Total Time of Session: 90
Session Comments:

Late Entry

☐ This is a late entry

OT Patient History

Subjective/Observations:

Pt sleeping in bed upon arrival but agreeable to OT eval and reporting no pain. "My memory is really going, I can't even take a guess at the year"

Precautions:

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity

Weight Bearing:

☑ Full ☐ Partial Partial %:
☐ Touch Down/Toe Touch ☐ WBAT ☐ No Weight Bearing

History of Current Illness:

Patient is a 72 year old female with no know PMH initially transfer from Valleywise Maryvale ED to Valleywise Hospital on 3/12 with concerns of persistent dizziness and headaches. Patient states her symptoms have been ongoing for about 7 weeks but have recently increase to the point where she is unable to complete ADLS and the only thing that helps her dizzy spells is laying in a dark room. Vital signs in the ED temp 36.4, heart rate 64, respiratory rate 18, SP02 98% and BP 157/67. CMP bicarb 20 otherwise unremarkable. CTA of head and neck showed no acute no acute intracranial abnormality. CT chest and abdomen and pelvis showed no acute abnormality with no active pneumonia, diverticulosis without diverticulitis, small hiatal hernia and fusiform ectasia of the abdominal aorta measuring 2.6 cm. She was then admitted to OBs with Medicine for further monitoring and evaluation of Vertigo. She was started on Lisinopril. ECHO findings; Left ventricle is normal in size. Sigmoid shape septum with increased basal left ventricular septal wall thickness. Normal. Left ventricular systolic function with an EF of 54-74%. Biplane LVEF is calculated at 74%. PLOF independent with ADLs and mobility. CLOF Min to mod assist with ADLs and Ambulation mod assist for 2 (lateral steps).

Social History:

Patient lives in with son in a SLH with no STE and was previously independent in all ADLs, functional transfers, and ambulation. She is retired and was previously driving herself for leisure activities.

Recreational/Leisure Activities

○ None

List Recreational Activities and Hobbies: hangout with grandchildren

Has the patient had two or more falls in the past year or any fall with injury in the past year?

○ Yes ◉ No ○ Unknown

Home Environment: (Stairs, Tub, Shower)

Patient lives in a SLH with son and has a tub/shower combo with grab bars

Current Adaptive Equipment:

Pt owns a cane that she does not use often

Transportation: Has lack of transporation kept you from medical appointments, meetings, work or from getting things needed for daily living?

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- [ ] A. Yes, it has kept me from medical appointments of from getting my medications
- [ ] B. Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [x] C. No
- [ ] X. Patient not able to respond
- [ ] Y. Patient declines to respond

Preferred Language:

- ○ English
- ○ Spanish
- ○ American Sign Language
- ○ Braille
- ○ Korean
- ○ German

Other: _____

Hearing Functional:

- ◉ Yes
- ○ No
- ○ Hearing Aid

Hearing: Ability to hear (with hearing aid or hearing appliances if normally used)

Adequate - no difficulty in normal conversation, social interaction, listening to TV

Vision Functional:

- ○ Yes
- ○ No
- ◉ Glasses
- ○ Reading Glasses Only

Vision: Ability to see in adequate light (with glasses or other visual appliances)

Adequate - sees fine detail, such as regular print in newspapers/ books

Expression of ideas and wants (consider both verbal and non-verbal expression and excluding language barriers)

3. Exhibits some difficulty with expressing needs and ideas (e.g., some words or finishing thoughts) or speech is not clear

Understanding verbal and non-verbal content (with hearing aid or device, if used, and excluding language barriers)

4. Understands: Clear comprehension without cues or repetitions

Dentures:

- ○ Yes
- ○ No

Current Diet Orders:

Diet Order Every Shift, Request Type: Routine, Diet Consistency: Regular, Liquid Consistency: Thin (0), Therapeutic Diets: General Healthful

Communication Barriers:

_____

Does the patient have any sexual health questions or concerns?

- ○ Yes
- ◉ No

Would the patient like additional information on sexual health?

- ○ Yes
- ◉ No

Comments:

_____

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Vitals Taken

Did you take Vitals Pre-Treatment?

- ○ Yes
- ○ No

Did you take Vitals During Treatment?

- ○ Yes
- ○ No

Did you take Vitals Post-Treatment?

- ○ Yes
- ○ No

UE Strength and ROM

Dominant Hand:

Right

UE ROM - Left

- ○ WNL
- ◉ WFL
- ○ Impaired
- ○ DNT

UE ROM - Right

- ○ WNL
- ◉ WFL
- ○ Impaired
- ○ DNT

Grasp

Right	Left	Comments

Comments for UE ROM

UE Strength Left

- ○ WNL
- ◉ WFL
- ○ Impaired
- ○ DNT

UE Strength Right

- ○ WNL
- ◉ WFL
- ○ Impaired
- ○ DNT

Skin Issues?

- ☑ No
- ☐ Yes

If Yes, Comment:

OT Additional Assessment Comments:

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Comments for UE Strength

UE Neurological

UE Tone

Body Part/Motion	Left	Right

UE Tone (Cont.)

Comments

Balance:

Static Sitting	Static Standing	Dynamic Sitting

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Balance: (Cont.)

Dynamic Standing

Head Control:

Trunk Control:

Balance/Posture Comments:

Sensory Perception

	Right Scale	Left Scale	Comments
Stereognosis			
Barognosis			
Graphesthesia			
Proprioception			

Cervical Dermatones

	Test	Right
C2: Back of Head/Top of Neck		
C3: Lower Neck		
C4: Clavicle (Just Below)		
C5: Lat Upr Arm Above Elbow		
C6: Lateral Forearm		
C6: Radial Part of Hand (Thumb		
C7: Middle Finger		
C8: Lateral Part Hand (Pinky)		
T1: Medial Forearm		
T2: Axillary Area		
T3: Upper Pectoral Area		
T4: Nipple Area		
T5: Lower Pectoral Area		
T6/T7: Upper Abdominal		
T8/T9: Middle Abdominal		
T10: Umbilicus Area (Belly Btn		
T11: Lower Abdominal		
T12: Hip Girdle (Just Above)		
L1: Hip Girdle/Groin Area		
L2/L3: Front Thighs		
L4: Medial Lower Leg		
L5: Lateral Lower Leg/Big Toe		

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S1: Heel/Middle Back of Leg		
S2: Back of Thighs		
S3: Medial Buttocks		
S4/S5: Perineal Area		

Cervical Dermatones (Cont.)

	Left	Comments
C2: Back of Head/Top of Neck		
C3: Lower Neck		
C4: Clavicle (Just Below)		
C5: Lat Upr Arm Above Elbow		
C6: Lateral Forearm		
C6: Radial Part of Hand (Thumb		
C7: Middle Finger		
C8: Lateral Part Hand (Pinky)		
T1: Medial Forearm		
T2: Axillary Area		
T3: Upper Pectoral Area		
T4: Nipple Area		
T5: Lower Pectoral Area		
T6/T7: Upper Abdominal		
T8/T9: Middle Abdominal		
T10: Umbilicus Area (Belly Btn		
T11: Lower Abdominal		
T12: Hip Girdle (Just Above)		
L1: Hip Girdle/Groin Area		
L2/L3: Front Thighs		
L4: Medial Lower Leg		
L5: Lateral Lower Leg/Big Toe		
S1: Heel/Middle Back of Leg		
S2: Back of Thighs		
S3: Medial Buttocks		
S4/S5: Perineal Area		

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Thoracic Dermatones

	Test	Right
T1: Medial Forearm		
T2: Axillary Area		
T3: Upper Pectoral Area		
T4: Nipple Area		
T5: Lower Pectoral Area		
T6/T7: Upper Abdominal		
T8/T9: Middle Abdominal		
T10: Umbilicus Area (Belly Btn		
T11: Lower Abdominal		
T12: Hip Girdle (Just Above)		
L1: Hip Girdle/Groin Area		
L2/L3: Front Thighs		
L4: Medial Lower Leg		
L5: Lateral Lower Leg/Big Toe		
S1: Heel/Middle Back of Leg		
S2: Back of Thighs		
S3: Medial Buttocks		
S4/S5: Perineal Area		

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Thoracic Dermatones (Cont.)

	Left	Comments
T1: Medial Forearm		
T2: Axillary Area		
T3: Upper Pectoral Area		
T4: Nipple Area		
T5: Lower Pectoral Area		
T6/T7: Upper Abdominal		
T8/T9: Middle Abdominal		
T10: Umbilicus Area (Belly Btn		
T11: Lower Abdominal		
T12: Hip Girdle (Just Above)		
L1: Hip Girdle/Groin Area		
L2/L3: Front Thighs		
L4: Medial Lower Leg		
L5: Lateral Lower Leg/Big Toe		
S1: Heel/Middle Back of Leg		
S2: Back of Thighs		
S3: Medial Buttocks		
S4/S5: Perineal Area		

UE Reflexes

	Left	Right
C5/C: Biceps		
C5/C6: Brachioradialis		
C6/C7: Triceps		
C6/T1: Finger Flexors		
L4: Quadriceps/Patellar		
S1: Plantarflexors/Achilles		

Comments for UE Reflexes

Activity Tolerance

Activity Tolerance:

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BIMS

Can Brief Interview for Mental Status be completed?

If no, complete Memory/Recall Ability category instead.

1. Yes (patient is able to be assessed)

Repetition of Three Words - Number of words repeated after first attempt

Ask patient: I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: sock, blue and bed. Now tell me the three words.

After the patient's first attempt, repeat the words using cues ("sock, something to wear; blue, a color; bed, a piece of furniture"). You may repeat the words up to two more times.

Three (3)

words repeated after first attempt Total Score: .00

Temporal Orientation - Able to report correct year

Ask patient: Please tell me what year it is right now.

Missed by > 5 years or no answer (0)

correct year Total Score: .00

Temporal Orientation - Able to report correct month

Ask patient: What month are we in right now?

Missed by > 1 month or no answer (0)

correct month Total Score: .00

Temporal Orientation - Able to report correct day of the week

Able to report correct day of the week

Incorrect or no answer (0)

correct day of the week Total Score: .00

Recall - Able to recall "sock"

Ask patient: Let's go back to an earlier question. What were those three words that I asked you to repeat? If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word.

No - could not recall (0)

Recall - Able to recall "sock" Total Score: .00

Recall - Able to recall "blue"

No - could not recall (0)

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Recall - Able to recall "blue" Total Score: .00

Recall - Able to recall "bed"

No - could not recall (0)

Recall - Able to recall "bed" Total Score: .00

Brief Interview for Mental Status (BIMS) Score

Score: 3

- [ ] Patient was unable to complete the interview
- [x] 0 to 7 = severely impairment
- [ ] 8 to 12 = moderately impaired
- [ ] 13 to 15 = cognitively intact

BIMS Total Score: 3.00

Cognition

Does the patient have Cognitive Deficits?

- ○ Yes
- ○ No

Health Literacy: How often do you have someone help you when you read instructions, pamphlets, or other written material from you doctor or pharmacy?

0. Never

A. Acute Onset Mental Status Change: Is there evidence of an acute change in mental status from the patient's baseline?

1. Yes

B. Inattention: Did the patient have difficulty focusing attention, for example being easily distractible or having difficulty keeping track of what was being said?

0. Behavior not present

C. Disorganized Thinking: Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

2. Behavior present, fluctuates (comes and goes, changes severity)

D. Altered Level of Consciousness: Did the patient have altered level of consciousness as indicated by any of the following criteria?

vigilant - startled easily to any sound or touch lethargic - repeatedly dozed off when being asked questions, but responded to voice or touch stuporous - very difficult to arouse and keep aroused for the interview comatose - could not be aroused

0. Behavior not present

OT Self Care

Did OT/COTA shower/bathe the patient today?

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(●) Yes ( ) No

Activities of Daily Living

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Self Feeding	SBA					
Oral Hygiene	Min					
Grooming	CGA					
Toilet Hygiene	CGA					
Bathing/Shower	CGA					
Dressing UE	SBA					
Dressing LE	CGA					
Putting On/Taking Off Footwear	SBA					

Comments for ADLs for Weekly Team Conference

OT Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Bed						
Toileting						
Tub						
Shower						

Comments for OT Mobility for Team Conference

Wheelchair Mobility not assessed

☐ On Admission ☐ Week 1 ☐ Week 2
☐ Week 3 ☐ Week 4

Wheelchair Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance						

Type of Wheelchair

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

Level of Assistance with Wheelchair Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						

Wheelchair Management

	Assistance Level	Comment
Propulsion in Room		
Propulsion in Facility		
Propulsion Outdoors		
Positioning for Txfr		
Negotiating Obstacles		
W/C Management		
Brakes		
Armrests		
Footrests		
Pressure Relief		

Comments for Wheelchair Mobility

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GG Scores - Self Care

(GG Score) PLOF - Self-Care:

3 - Independent

PLOF - Functional Cognition:

8 - Unknown

Eating: The ability to use suitable utensils to bring food and/or liquid to the mouth and swallow food and/ or liquid once the meal is placed before the patient

5. Setup or clean-up assistance

Eating Comments:

Oral Hygiene: The ability to use suitable items to clean teeth. Dentures (if applicable): The ability to insert and remove dentures into and from the mouth, and manage denture soaking and rinsing with use of equipment

3. Partial/moderate assistance

Oral Hygiene Comments:

Toileting Hygiene: The ability to maintain perineal hygiene, adjust clothes before and after voiding or having a bowel movement. If managing an ostomy, include wiping the opening but not managing equipment

4. Supervision or touching assistance

Toileting Hygiene Comments:

Toilet Transfer: The ability to get on and off a toilet or commode

4. Supervision or touching assistance

Toilet Transfer Comments:

Shower/Bathe Self: The ability to bathe self, including washing, rinsing, and drying self (excludes washing of back and hair). Does not include transferring in/out of tub/shower

4. Supervision or touching assistance

Shower/Bathe Self Comments:

Upper Body Dressing: The ability to dress and undress above the waist; including fasteners, if applicable

4. Supervision or touching assistance

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Upper Body Dressing Comments:

Lower Body Dressing: The ability to dress and undress below the waist, including fasteners; does not include footwear

4. Supervision or touching assistance

Lower Body Dressing Comments:

Putting On/Taking Off Footwear: The ability to put on and take off socks shoes or other footwear that is appropriate for safe mobility; including fasteners, if applicable

4. Supervision or touching assistance

Putting On/Taking Off Footwear Comments:

Activities of Daily Living Comments:

GG Scores - Transfers

Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed

Roll Left and Right Comments:

Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed

Sit to Lying Comments:

Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with no back support

Lying to Sitting on Side of Bed Comments:

Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed

Sit to Stand Comments:

Chair/bed-to-chair transfer: The abilityto transfer to and from a bed to a chair (or wheelchair)

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Chair/Bed-to-Chair Transfer Comments:

Bed to Wheelchair:

Bed to Wheelchair Comments:

OT Special Tests

OT Special Tests

	Admission Score	Week 1 Score	Week 2 Score	Week 3 Score
Actv Measure Post Acute Care				
Catherine Bergego Scale				
Function in Sitting Test				
Functional Reach				
Hand Grip Dynamometer (L)				
Hand Grip Dynamometer (R)				
Katz Index Independence ADLs	3/6			
Lawton IADL Scale				
Modified Barthel				
Nine Hole Peg Test				
Trail Making Test Parts A & B				
Other (Specify in Comments)				

OT Special Tests (Cont.)

	Week 4 Score	Week 5 Score	Discharge Score
Actv Measure Post Acute Care			
Catherine Bergego Scale			
Function in Sitting Test			
Functional Reach			
Hand Grip Dynamometer (L)			
Hand Grip Dynamometer (R)			
Katz Index Independence ADLs			
Lawton IADL Scale			
Modified Barthel			
Nine Hole Peg Test			
Trail Making Test Parts A & B			

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Other (Specify in Comments)			

OT Test Interpretation

Katz Index
Katz - Bathing
Need help with bathing more than one part of the body, getting in or out of the tub or shower. Requires total bathing (0)
Bathes self completely or needs help in bathing only a single part of the body such as the back, genital area or disabled extremity. (1)
Katz - Bathing Total Score: 0

Katz - Dressing
Needs help with dressing self or needs to be completely dressed. (0)
Get clothes from closets and drawers and puts on clothes and outer garments complete with fasteners. May have help tying shoes. (1)
Katz - Dressing Total Score: 0

Katz - Toileting
Needs help transferring to the toilet, cleaning self or uses bedpan or commode. (0)
Goes to toilet, gets on and off, arranges clothes, cleans genital area without help. (1)
Katz - Toileting Total Score: 1

Katz - Transferring
Needs help in moving from bed to chair or requires a complete transfer. (0)
Moves in and out of bed or chair unassisted. Mechanical transfer aids are acceptable (1)
Katz - Transferring Total Score: 0

Katz - Continence
Is partially or totally incontinent of bowel or bladder (0)
Exercises complete self control over urination and defecation. (1)
Katz - Continence Total Score: 1

Katz - Feeding
Needs partial or total help with feeding or requires parenteral feeding. (0)
Gets food from plate into mouth without help. Preparation of food may be done by another person. (1)
Katz - Feeding Total Score: 1

Total Score: 3
Interpretation:
6= High, full functional independence.
4= Moderate impairment.
2 or below= low, severe functional impairment, dependence

OT Education

Education Provided On:

- ☐ Fall Precautions
- ☐ Goals for Inpatient Rehab / Plan of Care
- ☐ Therapy Schedule
- ☐ Safe Mobility
- ☐ Amputation Program Handbook
- ☐ SCI Program Handbook
- ☐ Stroke Program Handbook
- ☐ TBI Program Handbook
- ☐ Cardiac/Sternal Precautions
- ☐ Posterior Hip Precautions
- ☐ Pressure Relieving Technique
- ☐ Spinal Precautions
- ☐ Autonomic Dysreflexia
- ☐ Compensatory Strategies
- ☐ Energy Conservation
- ☐ Home Exercise Program

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule
- [ ] Sexual Health
- [ ] Vision Changes

Use of Equipment:

Other:

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other:

OT Assessment

Occupational Therapy Assessment:

Pt presents with limitations to strength, standing/seated static/dynamic balance, endurance, limited activity tolerance, lack of safety awareness, cognitive limitations, needs increased time to perform activities that impact pt's ability to perform ADLs safely is limited by dizziness and light sensitivity and is not at PLOF. Pt was independent prior to this hospital admission with independence for functional mobility, transfers and ADLs. Pt now requiring CGA for ADLs, functional transfers, and ambulation. Pt is a high fall risk and does not have ample assistance available at home and a high risk for re-injury, would be unable to safely perform transfers and perform daily ADLs safely and functionally at this time. Pt would benefit from skilled occupational therapy services to address aforementioned deficits, increase functional independence & safety, prevent deconditioning, reduce caregiver reliance provide ongoing education regarding compensatory/ adaptive strategies/HEP/safety and return closer to prior functional baseline.

OT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Impaired Safety Awareness
- [ ] Impaired Vision/Perception
- [x] Decreased Strength
- [ ] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [ ] Impaired Communication

Discharge Planning

Discharge Plans:

DME Needed:

- [ ] None

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers:

Discharge Planning Comments:

OT Plan of Care

OT Plan of Care STG & LTG

Outcomes/Goals:
1. STG: Patient will complete standing grooming tasks at sink for 5 min with SBA and no LOB to improve endurance for ADL participation
2. LTG: Patient will complete grooming standing at the sink for greater than 5 minutes with no LOB, demonstrating improved activity tolerance for self care.
3. STG: Patient will complete functional reaching outside BOS during standing ADL task with CGA and no LOB to improve dynamic balance for daily activities.
4. LTG: Patient will perform functional mobility during ADL tasks using AD safely and with no LOB to improve safety awareness and independence with ADLs.
5. STG: Patient will complete LB dressing in seated position with SBA and minimal vc for safety awareness to reduce fall risk during ADLs.
6. LTG: Patient will complete UB/LB dressing independently in seated position without LOB or unsafe standing to maximize independence and reduce fall risk.

OT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [ ] 1.5 hours/day minimum of 5 days per week

Other Intensity/Frequency: 30-180 mins/ 5-7 days/wk

Total number of days anticipated: 14

Patient/Family Goals:

Not really I think I'm doing okay, I can go home and do whatever I need to do

OT Participated in Development of POC

Occupational Therapist who participated in the development of the Individualized Plan of Care:

Olson, Chelsea

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Score: 3.00

Electronically Signed By:
Chelsea Olson, OT, 03/14/2026 12:13

3/14/2026 8:15 Michelle Perry, PT, DPT Note Discipline: Physical Therapist Tag: Therapy Evaluation

Form: PT Initial Evaluation [P10]

Session Time

Session Time for Evaluation

Time In: 09:30 AM
Time Out: 10:30 AM
Total Time of Session: 60
Session Comments:

Late Entry

☐ This is a late entry

PT Patient History

Precautions:

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity

Weight Bearing:

☑ Full ☐ Partial Partial %:
☐ Touch Down/Toe Touch ☐ WBAT ☐ No Weight Bearing

History of Current Illness:

Patient is a 72 year old female with no know PMH initially transfer from Valleywise Maryvale ED to Valleywise Hospital on 3/12 with concerns of persistent dizziness and headaches. Patient states her symptoms have been ongoing for about 7 weeks but have recently increase to the point where she is unable to complete ADLS and the only thing that helps her dizzy spells is laying in a dark room. Vital signs in the ED temp 36.4, heart rate 64, respiratory rate 18, SP02 98% and BP 157/67. CMP bicarb 20 otherwise unremarkable. CTA of head and neck showed no acute no acute intracranial abnormality. CT chest and abdomen and pelvis showed no acute abnormality with no active pneumonia, diverticulosis without diverticulitis, small hiatal hernia and fusiform ectasia of the abdominal aorta measuring 2.6 cm. She was then admitted to OBs with Medicine for further monitoring and evaluation of Vertigo. She was started on Lisinopril. ECHO findings; Left ventricle is normal in size. Sigmoid shape septum with increased basal left ventricular septal wall thickness. Normal. Left ventricular systolic function with an EF of 54-74%. Biplane LVEF is calculated at 74%. PLOF independent with ADLs and mobility. CLOF Min to mod assist with ADLs and Ambulation mod assist for 2 (lateral steps).

Has the patient had two or more falls in the past year or any fall with injury in the past year?

○ Yes ◉ No ○ Unknown

Home Environment: (Stairs, Tub, Shower)

Patient lives in a SLH with son and has a tub/shower combo with grab bars

Social History:

Patient lives in with son in a SLH with no STE and was previously independent in all ADLs, functional transfers, and ambulation. She is retired and was previously driving herself for leisure activities.

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Subjective/Observations:

Pt presents supine in bed endorsing "room was spinning" and she was dizzy 3 days ago. Pt amicable and agreeable to education presented on Dix Hallpike testing to Tx vertigo. She endorses nausea as well

Alert and Oriented:

- [x] To Person
- [x] To Place
- [x] To Time
- [x] To Situation
- [ ] Disoriented x4

Safety Awareness:

- ○ Good
- ○ Fair
- ○ Poor

Do you need or want an interpreter to communicate with a doctor or healthcare staff?

- ○ Yes
- ○ No
- ○ Unable to determine

What form of Interpreter was utilized?

- [ ] By Phone (list Interpreter ID number in comments)
- [ ] By Certified Interpreter (list name or Interpreter ID number in comments)
- [ ] Patient agreed to use family member interpreter (non-medical terminology used)
- [ ] Patient agreed to use bi-lingual staff interpreter (non-medical terminology used)
- [ ] Other (see comments)

Comments: _____

Preferred Language:

- ○ English
- ○ Spanish
- ○ American Sign Language
- ○ Braille
- ○ Korean
- ○ German

Other: _____

Hearing Functional:

- ◉ Yes
- ○ No
- ○ Hearing Aid

Vision Functional:

- ○ Yes
- ○ No
- ○ Glasses
- ○ Reading Glasses Only

Communication Barriers:

- [ ] N/A

Current Adaptive Equipment:

Cane that she didn't use often

Comments:

Vitals Taken

Did you take Vitals Pre-Treatment?

- ○ Yes
- ○ No

Did you take Vitals During Treatment?

- ○ Yes
- ○ No

Did you take Vitals Post-Treatment?

- ○ Yes
- ○ No

LE Strength and ROM

LE MMT - Left

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- ○ Impaired
- ◉ WFL
- ○ WNL
- ○ DNT

LE MMT - Right

- ○ Impaired
- ◉ WFL
- ○ WNL
- ○ DNT

LE ROM - Left

- ○ Impaired
- ○ WFL
- ○ WNL
- ○ DNT

LE ROM - Right

- ○ Impaired
- ○ WFL
- ○ WNL
- ○ DNT

Comments for LE ROM

Cervical/Thoracic/Lumbar

Side Bending Left

Forward Bending

Side Bending Right

Left Rotation

Bending Backwards

Rotation Right

MMT Upper Trapezius

Left:

Right:

MMT Mid Trapezius

Left:

Right:

MMT Lower Trapezius

Left:

Right:

Posture:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ WFL ☐ Impaired

Comment:

Neurological

Tone:

☐ LUE - Normal	☐ LLE - Normal	☐ RUE - Normal	☐ RLE - Normal
☐ LUE - Flaccid	☐ LLE - Flaccid	☐ RUE - Flaccid	☐ RLE - Flaccid
☐ LUE - Hypo	☐ LLE - Hypo	☐ RUE - Hypo	☐ RLE - Hypo
☐ LUE - Hyper	☐ LLE - Hyper	☐ RUE - Hyper	☐ RLE - Hyper
☐ LUE - Clonus	☐ LLE - Clonus	☐ RUE - Clonus	☐ RLE - Clonus

Sitting-Static:

Sitting-Dynamic:

Standing-Static:

Standing-Dynamic:

Sensation

	Light Touch	Localization of Touch
Left Hip		
Right Hip		
Left Thigh		
Right Thigh		
Left Knee		
Right Knee		
Left Shin		
Right Shin		
Left Foot		
Right Foot		
Left Toes		
Right Toes		

Comments for Sensation

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Lumbar Dermatones

	Test	Right
L1: Hip Girdle/Groin Area		
L2/L3: Front Thighs		
L4: Medial Lower Leg		
L5: Lateral Lower Leg/Big Toe		
S1: Heel/Middle Back of Leg		
S2: Back of Thighs		
S3: Medial Buttocks		
S4/S5: Perineal Area		

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Lumbar Dermatones (Cont.)

	Left	Comments
L1: Hip Girdle/Groin Area		
L2/L3: Front Thighs		
L4: Medial Lower Leg		
L5: Lateral Lower Leg/Big Toe		
S1: Heel/Middle Back of Leg		
S2: Back of Thighs		
S3: Medial Buttocks		
S4/S5: Perineal Area		

Deep Tendon Reflexes

L5, S1, S2 (Hamstring): _____

L2, L3, L4 (Quad): _____

S1. S2 (Achilles): _____

Comments for LE Reflexes

What neurological, involuntary, abnormal and/or associated movement patterns are present? (i.e. tremors - intention or resting, dystonia, ataxia, athetoid-choreform, pusher syndrome, pill rolling, retropulsion in sitting, etc.):

Activity Tolerance

Activity Tolerance:

poor d/t dizziness and nausea

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Functional Mobility

Functional Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Rolling Left						
Rolling Right						
Supine to Sit	CGA					
Sit to Supine	CGA					
Bed to Chair						
Sit to Stand	Min					
Toilet Transfer						

Comments for Functional Mobility

No bed rolling assessed since upon subjective report, this writer is confident Pt presents with vertigo. LOB recovered upon standing

Ambulation Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance						

Ambulation Device

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

Level of Assistance with Ambulation

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						
Rolling Right						
Supine to Sit						
Sit to Supine						
Bed to Chair						
Sit to Stand						

Gait Deviations:

Comments for Ambulation

Stairs not assessed

- ☐ On Admission
- ☐ Week 1
- ☐ Week 2
- ☐ Week 3
- ☐ Week 4

Number of Steps

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
# of Steps						

Device for Stairs

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

Level of Assistance with Stairs

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						

Comments for Stairs

Wheelchair Mobility not assessed

- ☐ On Admission
- ☐ Week 1
- ☐ Week 2

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☐ Week 3 ☐ Week 4

Wheelchair Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance						

Type of Wheelchair

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

Level of Assistance with Wheelchair Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						

Wheelchair Management

	Assistance Level	Comment
Propulsion in Room		
Propulsion in Facility		
Propulsion Outdoors		
Positioning for Txfr		
Negotiating Obstacles		
W/C Management		
Brakes		
Armrests		
Footrests		
Pressure Relief		

Comments

GG Mobility Scores

Prior Device Use:

☑ None

☐ Manual wheelchair ☐ Motorized wheelchair and/or scooter ☐ Mechanical lift ☐ Walker

☐ Orthotics/Prosthetics ☐ Cane

Other:

(GG Score) PLOF - Indoor Mobility (Ambulation):

3 - Independent

(GG Score) PLOF - Stairs:

3 - Independent

Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed

88. Not attempted due to medical condition or safety concerns

Roll Left and Right Comments:

Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

3. Partial/moderate assistance

Sit to Lying Comments:

Mod for Dix hallpike testing

Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with no back support

4. Supervision or touching assistance

Lying to Sitting on Side of Bed Comments:

Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed

3. Partial/moderate assistance

Sit to Stand Comments:

Min d/t LOB from dizziness

Chair/bed-to-chair transfer: The abilityto transfer to and from a bed to a chair (or wheelchair)

3. Partial/moderate assistance

Chair/Bed-to-Chair Transfer Comments:

Mod c FWW

Car transfer: The ability to transfer and out of a car or van on the passengerside. Does not include the ability to open/close door or fasten seat belt

88. Not attempted due to medical condition or safety concerns

Car Transfer Comments:

Walk 10 feet: Once standing, the abilityto walk at least 10 feet in a room, corridor, or similar space

3. Partial/moderate assistance

Walk 10 Feet Comments:

Mod LOB Fww

Walk 50 feet with two turns: Once standing, the ability to walk at least 50 feet and make two turns

3. Partial/moderate assistance

Walk 50 Feet with Two Turns Comments:

ModA fww

Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space

88. Not attempted due to medical condition or safety concerns

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Walk 150 Feet Comments:

Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel

88. Not attempted due to medical condition or safety concerns

Walking 10 Feet on Uneven Surfaces Comments:

1 step (curb): The ability to go up and down a curb or up and down one step

88. Not attempted due to medical condition or safety concerns

1 Step (Curb) Comments:

4 steps: The ability to go up and down four steps with or without a rail

88. Not attempted due to medical condition or safety concerns

4 Steps Comments:

12 steps: The ability to go up and down 12 steps with or without a rail

88. Not attempted due to medical condition or safety concerns

12 Steps Comments:

Picking up object: The ability to bend/ stoop from a standing position to pick up a small object, such as a spoon, fromthe floor

88. Not attempted due to medical condition or safety concerns

Picking Up Object Comments:

Does the patient use a wheelchair and/or scooter?

○ Yes ◉ No

Functional Limitations/Skin/LE Comments

Impairments/Functional Limitations:

Inability to bend over and stand erect as a result of nausea

Skin Issues?

☑ No ☐ Yes

If Yes, Comment:

PT - Lower Extremity Comments:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT Special Tests

PT Special Tests

	Admission Score	Week 1 Score	Week 2 Score	Week 3 Score
2 Minute Walk Test				
5 Times Sit to Stand				
6 Minute Walk Test				
10 Meter Walk Test				
Activities-Specific Balance				
Actv Measure Post Acute Care				
Amputee Predictor Assess Tool				
Berg Balance Test				
Elderly Mobility Scale				
Function in Sitting Test				
Functional Gait Assessment				
Timed up and Go Test				
Tinetti Test				
Other (Specify in Comments)	Dix Hallpike			

PT Special Tests (Cont.)

	Week 4 Score	Week 5 Score	Discharge Score
2 Minute Walk Test			
5 Times Sit to Stand			
6 Minute Walk Test			
10 Meter Walk Test			
Activities-Specific Balance			
Actv Measure Post Acute Care			
Amputee Predictor Assess Tool			
Berg Balance Test			
Elderly Mobility Scale			
Function in Sitting Test			
Functional Gait Assessment			
Timed up and Go Test			
Tinetti Test			
Other (Specify in Comments)			

PT Test Interpretation

+ Dix on Right for BPPV

PT Education

Education Provided On:

- [x] Fall Precautions
- [x] Goals for Inpatient Rehab / Plan of Care
- [x] Therapy Schedule
- [x] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [x] Energy Conservation
- [ ] Weight Bearing Precautions
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule

Use of Equipment:

Other:

Education Provided To:

- [x] Patient
- [x] Family
- [ ] Caregiver

Other: Go to PT after D/c to increase cervical and thoracic spine mobility to decrease reoccurrence of BPPV

Response to Education:

- [x] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other:

PT Assessment

Physical Therapy Assessment:

Priscilla presents to physical therapy at Reunion rehab following prior admission at Valleywise as a result of a recent bout of dizziness and HA lasting 7 weeks. Pertinent objective measures include +R Dix Hallpike testing, nausea and dizziness with ambulation. Functional limitations include inability to participate in bed mobility, transfers and ambulation free of dizziness and transfers prior to Dix Hallpike. She was educated to sleep with HOB elevated to 45 degrees for next 24hrs as a result of Epley maneuver completed. She was educated to limited bending forward and picking up objects, donning pants with cervical spine flexion and flexion during toileting and other ADLs for the next 48hrs as a result of BPPV. She responded very well to Epley maneuver and has been educated that she might need maneuver in 24-48hrs by another PT at reunion if she is not 100%

PT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Decreased Strength
- [x] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [x] Impaired Safety Awareness
- [ ] Impaired Communication
- [ ] Impaired Vision/Perception

Discharge Planning

Discharge Plans:

return home to live Indep

Discharge Planning Comments:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

DME Needed:

- [ ] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: _____

DME Providers: _____

PT Plan of Care

PT Plan of Care STG & LTG

Problem: Mobility Impairment

Outcomes/Goals:
1. STG: She will demonstrate Indep with Amb 250' on uneven terrain c FWW within 5 days free of VC and MI.
2. LTG: She will demonstrate ability to amb 350' free o AD, no VC and even and uneven surfaces in order to return home to live Indep
3. STG: She will participate in all transfers free of dizziness, nausea and room spinning in order to return home free of challenges
4. LTG: She will demonstrate all bed mobility Indep and free of dizziness in order to return home Indep

Interventions/Education:
1. Family/Caregiver training.
2. Safety Education.
3. Home exercise program instruction.
4. Bed mobility training.
5. Transfer training.
6. Gait training.
7. Therapeutic exercise.
8. Aerobic exercise.
9. Endurance training.
10. Soft tissue mobilization.
11. Neuromuscular re-education.
12. Balance training.

PT Intensity/Frequency

- [ ] Not required

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- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency: _____

Total number of days anticipated: 14

Patient/Family Goals:

Return to PLOF of Indep c ADLs, amb and grooming

PT Participated in Development of POC

Physical Therapist who participated in the development of the Individualized Plan of Care:

Perry, Michelle

Score: 0.00

Electronically Signed By:

Michelle Perry, PT, DPT, 03/14/2026 10:37

3/14/2026 9:42 Liviu Vladoianu, NP Note Discipline: Nurse practitioner Tag: Consult Note

Form: Provider Consult Note [39]

Coded Diagnoses

Admitting Diagnosis

#Debility - vertigo
#HTN
#HLD

Review of History

History of Present Illness

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

PMH/SH/FH reviewed

- [ ] No change since admission
- [ ] Changes/updates documented below

Past Medical History

Add comments to any medical history that needs further details

- [x] None
- [ ] AIDS/HIV
- [ ] A-Fib
- [ ] Allergic Rhinitis
- [ ] Alzheimer's
- [ ] Anemia
- [ ] Anxiety
- [ ] Arthritis (Osteo)
- [ ] Arthritis (Rheumatoid)
- [ ] Benign Prostate Hypertrophy
- [ ] CAD
- [ ] Cancer
- [ ] CHF
- [ ] Chronic Back Pain
- [ ] Cirrhosis
- [ ] Compression Fracture
- [ ] Dementia

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] DJD
- [ ] Diabetes
- [ ] Diverticulitis/losis
- [ ] Depression
- [ ] Deep Vein Thrombosis
- [ ] Epilepsy
- [ ] GERD
- [ ] Glaucoma
- [ ] GI Bleed
- [ ] Gout
- [ ] Heart Disease
- [ ] Hiatal Hernia
- [ ] HTN
- [ ] Hyperlipidemia
- [ ] Kidney/Bladder
- [ ] Liver Disease
- [ ] Lung Disease
- [ ] Macular Degeneration
- [ ] MI (Not Primary Med Condtn)
- [ ] Obesity
- [ ] Orthopedic
- [ ] Osteoporosis
- [ ] Peptic Ulcer
- [ ] Peripheral Vascular Disease
- [ ] Pulmonary Embolis
- [ ] Seizures
- [ ] Sleep Apnea
- [ ] Stroke
- [ ] TB History
- [ ] Thyroid Disease
- [ ] TIA

Other Medical History: _____

Additional Comments: _____

Past Surgical History

Add comments to any surgical history that needs further details

- [x] None
- [ ] AICD
- [ ] Amputation(s)
- [ ] Anesthesia difficulties
- [ ] Angioplasty
- [ ] Appendectomy
- [ ] Biopsy (site)
- [ ] Bronchoscopy
- [ ] CABG
- [ ] Carotid (CEA)
- [ ] Carpal Tunnel
- [ ] Cataract(s)
- [ ] Cholecystectomy
- [ ] Craniotomy
- [ ] Esophageal Surgery
- [ ] Gastric Surgeries (Type)
- [ ] Heart Valve Surgery
- [ ] Hemorrhoidectomy
- [ ] Hernia Repair (location)
- [ ] Hysterectomy
- [ ] Joint Replacement (location)
- [ ] Lithotripsy
- [ ] Mastectomy (R/L/B)
- [ ] Orthopedic
- [ ] Ostomies (Describe)
- [ ] Pacemaker
- [ ] PEG
- [ ] Pneumonectomy (lobes)
- [ ] Spinal Surgery
- [ ] Stent Placement (location)
- [ ] Tonsillectomy
- [ ] Thyroid Surgery
- [ ] Tracheostomy

Other Surgical History: _____

Additional Comments: _____

Family Medical History

Have any of these conditions occurred on either side of the patient's family? (Check if yes)

- [x] Not applicable
- [ ] Allergies
- [ ] Cardiovascular disease
- [ ] Gastrointestinal disease
- [ ] Pulmonary disease
- [ ] Bleeding tendencies
- [ ] Tuberculosis
- [ ] Psychiatric illness
- [ ] Bone disease
- [ ] Kidney disease
- [ ] Congenital deformities
- [ ] Alcohol/drug dependence

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Cancer/Tumors
- [ ] Diabetes

Other: _____

Comments: _____

Marital Status

Marital Status: Widowed

Current or past use of alcohol?

- ( ) Yes
- (•) No

Recreational/Illicit Drugs

- [x] Denies Use
- [ ] Past History of Use
- [ ] Current Use

Type: _____ Frequency: _____

Date last used: _____

Tobacco/Smoking

- [ ] Current Smoker Packs/day: _____ How many years: _____
- [x] Non-Smoker

Previous History of Smoking (indicate last time used and previous PPD): _____

- [ ] Vapes Vapes Frequency: _____
- [ ] Marijuana Marijuana Frequency: _____

Other: _____

Consult ROS

Review of System Status Update

- [ ] N/A (Admission Review of Systems)
- [ ] NO CHANGES in Condition Since Last System Review.
- [ ] CHANGES HAVE Occurred & Amended Below to Reflect an Update to the Patient's Current Condition.
- [ ] Unable to Obtain Review of Systems. Reason: _____

Review of Systems

Denies fever/chills, chest pain, SOB, cough, nausea, vomiting, diarrhea, constipation, dysuria, new weakness or changes in sensation.
Complete ROS negative unless mentioned above.

Complications HEENT:

Check All that Apply.

- [ ] Jaundice
- [ ] Diplopia
- [ ] Photophobia
- [ ] Rhinitis
- [ ] Nose Bleeds
- [ ] Packed Left Nostril
- [ ] Packed Rt. Nostril
- [ ] Tooth ache
- [ ] Ear ache
- [ ] Sore throat
- [ ] Sinus tender
- [ ] Lymphadenopathy

Other: _____

- [ ] None of the Above

Complications of Respiratory System:

Check All that Apply.

- [ ] Intubated
- [ ] SOB at Rest
- [ ] SOB with Exertion
- [ ] Orthopnea
- [ ] Snores
- [ ] Hemoptysis
- [ ] Chills
- [ ] Night Sweats
- [ ] Fatigue

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Recent Weight Loss
- [ ] Ineffective & Non-Productive Cough
- [ ] Frequent Suctioning
- [ ] Persistent cough less than 2 weeks
- [ ] Persistent cough greater than 2 weeks
- [ ] Tracheostomy
- [ ] Sleep Apnea
- [ ] Oxygen Dependent

Other: _____

- [ ] None of the Above

Complications of Circulatory System:

Check all that Apply.

- [ ] HTN
- [ ] Hypotension
- [ ] Orthostatic Hypotension
- [ ] Tachycardia
- [ ] Bradycaria
- [ ] Dysrhythmia
- [ ] Edema
- [ ] History of Chest Pains at Rest
- [ ] History of Chest Pains with Activity

Other: _____

- [ ] None of the Above

Complications GI System:

Check all that Apply.

- [ ] Vomiting in Past 72 Hours
- [ ] Diarrhea Within past 72 Hours
- [ ] Chronic Constipation
- [ ] Weight Loss
- [ ] Weight Gain
- [ ] Indigestion
- [ ] Reflux
- [ ] Rectal Bleeding
- [ ] Tarry Stools
- [ ] Rectal Tube
- [ ] Gastric Decompression
- [ ] Jaundice
- [ ] Encephalopathy

Other: _____

- [ ] None of the Above

Complications Endocrine System:

Check all that Apply.

- [ ] Episodes of Hypoglycemia
- [ ] Episodes of Hyperglycemic
- [ ] Frequent Urination
- [ ] Excessive Thirst
- [ ] Fatigue/Malaise
- [ ] Hyperactivity

Other: _____

- [ ] None of the Above

Complications Genitourinary System:

Check all that Apply.

- [ ] Incontinent
- [ ] Neurogenic Bladder
- [ ] Anuria
- [ ] Hematuria
- [ ] Cloudy
- [ ] Odor
- [ ] Urgency
- [ ] Retention
- [ ] Difficulty starting stream
- [ ] Vaginal/Penile Bleeding or Drainage

Other: _____

- [ ] None of the Above

Complications Musculoskeletal/Mobility:

Check all that Apply.

- [ ] Fall Risk
- [ ] Activity Intolerance
- [ ] Poor Balance
- [ ] Weakness/Fatigue
- [ ] Joint Swelling
- [ ] Joint Pain
- [ ] Contractures
- [ ] Dizziness
- [ ] Amputation
- [ ] Hemiplegia
- [ ] Monoplegia
- [ ] Quadrapeligia

Other: _____

- [ ] None of the Above

Complications of Pain:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Check all that Apply.

- [ ] Chronic Pain
- [ ] Intractable Pain
- [ ] Decreased Mental Status
- [ ] Agitation
- [ ] Inability to Concentrate
- [ ] Anxiety
- [ ] Nausea/Vomiting
- [ ] Constipation
- [ ] Itching

Other: _____

- [ ] None of the Above

Pain Management:

- [ ] N/A
- [ ] Heat
- [ ] Ice
- [ ] TENS Unit
- [ ] Positioning
- [ ] Biofeedback
- [ ] Medication

Comments: _____

Complications Skin/Wound:

Check all that Apply.

- [ ] Urinary Incontinence
- [ ] Fecal Incontinence
- [ ] Wound Drainage
- [ ] Erythema
- [ ] Skin Maceration
- [ ] Skin Tear
- [ ] Incision Dehiscense
- [ ] Incision Evisceration
- [ ] Stoma Care
- [ ] Poor Nutrition
- [ ] Immobility

Other: _____

- [ ] None of the Above

Complication Neuro/Mental Status:

Check all that Apply.

- [ ] Altered Mental Status
- [ ] Dementia Confusion
- [ ] Emotional Liability/Depression
- [ ] Seizures
- [ ] Headache
- [ ] Dizziness/Balance Loss
- [ ] Speech Problem
- [ ] Loss of Gag Reflex
- [ ] Loss of Memory
- [ ] Loss of Safety Awareness
- [ ] Paralysis - Location: _____

Other: _____

- [ ] None of the Above

All Recent Investigations Noted

- ( ) All Recent Investigations Noted

Consult PE

Blood Pressure:

Systolic (mmHg): 132 Diastolic (mmHg): 58 Position: Sitting Side: Left

Comment: _____

Pulse:

B/min: 72 Position: Sitting Site: Monitor Type: Regular

Comment: _____

Respiration:

Per Minute: 18 Vent: No

Comment: _____

Weight:

lb: 183.0 Scale: Bed

Comment: _____

Height:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Feet: 5 Inches: 1
Comment:

Physical Exam

General:

- [ ] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments:

Skin:

- [ ] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments:

- [ ] Abnormal

Abnormal findings:

HEENT:

- [ ] Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments:

- [ ] Abnormal

Abnormal findings:

Neck:

- [ ] Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments:

- [ ] Abnormal

Abnormal findings:

Heart:

- [ ] Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments:

- [ ] Abnormal

Abnormal findings:

Lungs/Chest:

- [ ] Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments:

- [ ] Abnormal

Abnormal findings:

Abdomen:

- [ ] Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments:

- [ ] Abnormal

Abnormal findings:

Extremities:

- [ ] Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments:

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- [ ] Abnormal

Abnormal findings: _____

Back:

- [ ] Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

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Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Gait and station are normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Romberg test is normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Plantar responses are down going.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Sensory: Sensory examination to pinprick and vibration is normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed _____

Comment/Non-interfaced results: _____

Diagnostic Imaging Reviewed

_____

Comment/Non-interfaced results: _____

Microbiology Tests Reviewed

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Consult Impression/Plan

Action Plan

☐ New Action Plan Initiated ☐ Continue Same Course of Care ☐ Updated Action Plan Initiated

Plan:

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

#Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist
#HTN
- lisinopril
#HLD
- on statin
#Skin
- monitor per nursing protocol for decubitus ulcers
- frequent offloading as indicated
#Bowel - constipation
- colace/senna
- lactulose, dulcolax suppository PRN
#Bladder
- monitor for retention
- check PVRs
#DVT ppx
- SCDs

Impression

#Debility - vertigo
#HTN
#HLD
99223 and 99497
Patient is Full Code

Neuro/Psych Assessment and Plan

Respiratory Assessment and Plan

Cardiovascular Assessment and Plan

Renal/Genitourinary Assessment and Plan

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Infectious Disease Assessment and Plan

Gastrointestinal/Stress Ulcer prophylaxis/Nutrition Assessment and Plan

DVT prophylaxis Assessment and Plan

Wounds/Pressure ulcer Assessment and Plan

Medications Reviewed

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

I have Reviewed the Following Patient Data:

- ☐ Prior Medical Records & Test Results
- ☐ Medication Reconciliation
- ☐ Allergies
- ☐ Consultant Report(s)
- ☐ Current/New Medications
- ☐ Current Lab Result & Diagnostic Test
- ☐ I Have Personally Discussed Code Status with this Patient or their Surrogate Decision Maker and Believe they Understand their Medical Condition, Prognosis and the Consequences of their Code Status Decision.
- ☐ All of the Above

Anticipated Post Hospital Service Needs:

- ☐ Home with Help
- ☐ Home with Home Health
- ☐ Assisted Living Facility
- ☐ SNF
- ☐ Acute Inpatient Rehab
- ☐ Home with Outpatient Therapy
- ☐ Hospice
- ☐ Group Home
- ☐ Post Acute Vent Care Unit

Other: _____

Coordinated care with

- ☐ Consultants
- ☐ Nursing
- ☐ Nutrition
- ☐ Therapist
- ☐ Family

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☐ Attending ☐ PA Other: _____

Plan Discussed in Detail with:

☐ Patient ☐ Family ☐ Nursing Staff ☐ Case Manager

Comment: _____

Other

_____

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

_____

Physician Addendum

_____

Electronically Signed By:

Liviu Vladoianu, NP, 03/15/2026 17:49

3/14/2026 11:46 Peter Campanella, PharmD Note Discipline: Pharmacist Tag: RX - Admission Medication Review

Pt was alert and orientated and lying in bed. I went over all the medications with the patient. Patient verbalized that those were the correct medications and dosages and frequencies.

Pt had no additional questions

ALLERGIES: pt reports NKDA
Mentioned to the patient that PRN medication (especially pain medications) need to be requested from the nurses

Told the patient to have the nurses contact the pharmacy if she had any additional medication related questions.

Electronically Signed By:

Peter Campanella, PharmD, 03/14/2026 11:46

3/14/2026 11:48 Peter Campanella, PharmD Note Discipline: Pharmacist Tag: Query

EPCS updated to:

CVS/pharmacy #9222
NCPDP:
0353792
Address:
4275 W Thomas Rd
43rd Ave & Thomas
Phoenix, AZ 850194334
Phone:
WORK: (602) 272-5601
FAX: (602) 455-8153

Electronically Signed By:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Peter Campanella, PharmD, 03/14/2026 11:48

3/14/2026 11:48 Peter Campanella, PharmD Note Discipline: Pharmacist Tag: Admission

Form: Drug Regimen Review - Admission [P91]

Drug Regimen Review

1. Did a complete drug regimen review identify potentially clinically significant medication issue?
   - (●) No - No issues found during review
   - ( ) Yes - Issues found during review
   - ( ) N/A - Patient is not taking any medications

2. If YES - Did the facility contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?
   - ( ) Yes
   - (●) No

Comments:

n/a

3. Is the patient female and between the ages of 14-50 years old?
   - ( ) Yes
   - (●) No

High Risk Drug Classes

1. Is taking - Indicate if the patient is taking any medications by pharmacological classification, not how it is used. 2. Indication noted - if medication is in use, check for indication on the order. (Order Reporting: Active Medications or Active Medication Order Listing report can be used to find these medications.)

High-Risk Drug Classes: Use and Indication

- [ ] Antipsychotic - Is Taking
- [ ] Antipsychotic - Indication Noted
- [ ] Antipsychotic - No Indication Noted
- [x] Anticoagulant - Is Taking
- [x] Anticoagulant - Indication Noted
- [ ] Anticoagulant - No Indication Noted
- [ ] Antibiotic - Is Taking
- [ ] Antibiotic - Indication Noted
- [ ] Antibiotic - No Indication Noted
- [ ] Opioid - Is Taking
- [ ] Opioid - Indication Noted
- [ ] Opioid - No Indication Noted
- [ ] Antiplatelet - Is Taking
- [ ] Antiplatelet - Indication Noted
- [ ] Antiplatelet - No Indication Noted
- [ ] Hypoglycemic (including Insulin) - Is Taking
- [ ] Hypoglycemic (including Insulin) - Indication Noted
- [ ] Hypoglycemic (including Insulin) - No Indication Noted
- [ ] None of the above

High-Risk Drug Classes: Comments

Cancer and IV Treatments

Cancer Treatments

Chemo Meds: Code any type of chemotherapy medication administered as an antineoplastic for cancer treatment given by any route in this item. Each medication should be evaluated to determine the indication is for Cancer treatment before coding it here.

- [ ] Chemotherapy
- [ ] IV Chemo
- [ ] Oral Chemo
- [ ] Other Chemo
- [x] None of the above

IV Treatments

IV Meds: Code any medication or biological given by intravenous push, epidural pump, or drip through a central or peripheral port in this item.

- [ ] IV Medications
- [ ] Vasoactive IV Medications
- [ ] IV Antibiotics
- [ ] IV Anticoagulation
- [ ] Other type of IV Medication
- [x] None of the above

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Pharmacist Participated in Development of POC

Pharmacist who participated in the Individualized Plan of Care:

campanella, peter

Score: 0.00

Electronically Signed By:

Peter Campanella, PharmD, 03/14/2026 11:50

3/14/2026	14:26	Mara Cleary, CCC-SLP	Note Discipline: Speech-Language Pathologists	Tag: Therapy Evaluation

Form: SLP Initial Evaluation [P17]

Session Time

Session Time for Evaluation

Time In: 03:00 PM

Time Out: 03:30 PM

Total Time of Session: 30

Session Comments:

Late Entry

- [ ] This is a late entry

SLP Patient History

Nutritional Approaches on Admission:

Check all of the following nutritional approaches that apply on admission

- [ ] A. Parenteral/IV Feeding
- [ ] B. Feeding Tube (e.g., nasogastric or abdominal (PEG))
- [ ] C. Mechanically altered diet - require change in texture of food or liquids (e.g., pureed food, thickened liquids)
- [ ] D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)
- [x] Z. None of the above

Precautions:

Fall Precautions Every Shift, Request Type: Now
GERD
aspiration precautions

Therapy Precautions:

Fall Dizziness Light sensitivity Cognitive communication deficits/Word retrieval deficits History of esophageal dysphagia

Disease Specific Programs:

History of Current Illness:

Patient is a 72 year old female with no know PMH initially transfer from Valleywise Maryvale ED to Valleywise Hospital on 3/12 with concerns of persistent dizziness and headaches. Patient states her symptoms have been ongoing for about 7 weeks but have recently increase to the point where she is unable to complete ADLS and the only thing that helps her dizzy spells is laying in a dark room. Vital signs in the ED temp 36.4, heart rate 64, respiratory rate 18, SP02 98% and BP 157/67. CMP bicarb 20 otherwise unremarkable. CTA of head and neck showed no acute no acute intracranial abnormality. CT chest and abdomen and pelvis showed no acute abnormality with no active pneumonia, diverticulosis without diverticulitis, small hiatal hernia and fusiform ectasia of the abdominal aorta measuring 2.6 cm. She was then

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

admitted to OBs with Medicine for further monitoring and evaluation of Vertigo. She was started on Lisinopril. ECHO findings; Left ventricle is normal in size. Sigmoid shape septum with increased basal left ventricular septal wall thickness. Normal. Left ventricular systolic function with an EF of 54-74%. Biplane LVEF is calculated at 74%. PLOF independent with ADLs and mobility. CLOF Min to mod assist with ADLs and Ambulation mod assist for 2 (lateral steps).

Rehab Diagnosis:

Vertigo

Date of Onset

3/12/2026

Prior Level of Function:

Social History:

Patient lives in with son in a SLH with no STE and was previously independent in all ADLs, functional transfers, and ambulation. She is retired and was previously driving herself for leisure activities.

Subjective/Observations:

Patient pleasant and cooperative.

Preferred Language:

- (●) English
- ( ) Spanish
- ( ) American Sign Language
- ( ) Braille
- ( ) Korean
- ( ) German

Other:

Do you need or want an interpreter to communicate with a doctor or healthcare staff?

- ( ) Yes
- (●) No
- ( ) Unable to determine

What form of Interpreter was utilized?

- [ ] By Phone (list Interpreter ID number in comments)
- [ ] By Certified Interpreter (list name or Interpreter ID number in comments)
- [ ] Patient agreed to use family member interpreter (non-medical terminology used)
- [ ] Patient agreed to use bi-lingual staff interpreter (non-medical terminology used)
- [ ] Other (see comments)

Comments:

Hearing Functional:

- ( ) Yes
- (●) No
- ( ) Hearing Aid

Dentures/Partials:

- (●) None
- ( ) Upper
- ( ) Lower
- ( ) Both

Vision Functional:

- ( ) Yes
- ( ) No
- (●) Glasses
- ( ) Reading Glasses Only

Other Assistive Devices:

- [x] None
- [ ] Communication Board
- [ ] Glasses

Other:

Comments:

Patient / Family Goals:

Ongoing

Expression of ideas and wants (consider both verbal and non-verbal expression and excluding language barriers)

3. Exhibits some difficulty with expressing needs and ideas (e.g., some words or finishing thoughts) or speech is not clear

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Understanding verbal and non-verbal content (with hearing aid or device, if used, and excluding language barriers)

4. Understands: Clear comprehension without cues or repetitions

PLOF - Functional Cognition:

3 - Independent

Functional Level of Assistance

Functional Levels of Assistance:

	Level of Assist	Comments
Motor Speech Function		
Respiratory Function		
Hearing		
Articulation		
Social Interaction		
Reading		
Writing		
Attention/Concentration		
Thought Organization		
Functional Math		
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Facial Structure/Function:

WFL

Oral Structure/Function:

Respiratory Function:

WFL

Visual Acuity:

- (●) WNL
- ( ) Glasses
- ( ) Needs Large Print
- ( ) Visually Impaired/Blind

Intelligibility:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Minimal Assitance - 70%

Comprehension:

WFL: Basic
Complex: min assist

Expression:

Word retrieval deficits, self corrections.

Hand Dominance:

- ○ Left Hand Dominant
- ◉ Right Hand Dominant

Voice:

WFL

Fluency:

Orientation:

- ○ Oriented x1
- ○ Oriented x2
- ◉ Oriented x3
- ○ Oriented x4

Other:

Memory:

Mod assist

Reasoning/Problem Solving:

WFL/Basic

Visual Perception:

- ☐ WNL
- ☐ Left Neglect
- ☐ Right Neglect
- ☐ Hemianopsia/Field Cut
- ☐ Diminished Depth Perception/Figure-Ground Deficits

Comments: visual attention deficits noted

Behavior:

WFL, good insight

Results / Comments:

Patient presents with moderate cognitive linguistic deficits and suspected visual deficits.

Swallowing Status: (Check all that apply)

- ☐ Regular Food- Solids and liquids swallowed safely without supervision or modified food or liquid consistency
- ☑ Modified Food Consistency/Supervision - Patient requires modified food or liquid consistency and/or needs supervision during eating for safety
- ☐ Tube/Parenteral Feeding - Tube/Parenteral feeding used wholly or partially as a means of sustenance

Identified Problems/Deficits:

- ☐ None
- ☑ Cognition/Communication
- ☑ Swallowing Disorder

Other:

SLP Assessment

SLP Assessment:

Patient presents with moderate cognitive linguistic/communication deficits and suspected visual deficits, word retrieval deficits.

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

SLP Weekly Progress

SLP Objective for Team Conference

	Admission	Week 1
Attention	Mod	
Cognitive Deficit	Mod	
Communication Deficit	Mod	
Comprehension Deficit	Ind	
Swallowing Impairment	Ind	

SLP Objective for Team Conference (Cont.)

	Week 2	Week 3
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

SLP Objective for Team Conference (Cont.)

	Week 4	Goal
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

Comments for SLP Objective

SLP Special Tests

SLP Special Tests

	Admission Score	Week 1 Score	Week 2 Score	Week 3 Score
Addenbrooke's Cognitive Exam				
Attention Processing Training				
Boston Naming Test 2nd Edition				
Cog Linguistic Quick Test Plus				
Montreal Cognitive Assessment				
NIH Stroke Scale				
Quick Aphasia Battery				
Texas Functional Living Scale				
Western Aphasia Battery-Revise				
Other (Specify in Comments)	14/30			

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

SLP Special Tests (Cont.)

	Week 4 Score	Week 5 Score	Discharge Score
Addenbrooke's Cognitive Exam			
Attention Processing Training			
Boston Naming Test 2nd Edition			
Cog Linguistic Quick Test Plus			
Montreal Cognitive Assessment			
NIH Stroke Scale			
Quick Aphasia Battery			
Texas Functional Living Scale			
Western Aphasia Battery-Revise			
Other (Specify in Comments)			

SLP Test Interpretation

Pt participated in the St. Louis University Mental Status Examination (SLUMS) for standardized assessment of cognitive-linguistic function.
She scored 14/30.

Orientation: 2/3
Calculations: 0/3
Generative naming: 2/3
Short-term recall: 2/5
Digit reversal: 1/2
Clock drawing: 2/4
Shape discrimination: 1/2
Auditory comprehension / Immediate recall: 4/8

Discharge Plans / Education

Discharge Plans:

Home

SLP Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [ ] Decreased Oral Motor Skills
- [x] Dysphagia
  - [ ] Oral
  - [ ] Pharyngeal
- [x] Decreased Cognition
- [x] Decreased Attention
- [x] Impaired Safety Awareness
- [ ] Aphasia
- [ ] Decreased Receptive Language
- [x] Decreased Expressive Language
- [ ] Decreased Vocal Skills
- [ ] Dysarthria
- [ ] Apraxia

Education Provided on:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] Goals for Inpatient Rehab/Plan of Care
- [x] Therapy Schedule
- [ ] Language Disorder
- [x] Safety Awareness
- [ ] Aphasia
- [x] Cognitive Dysfunction
- [ ] Dysarthria
- [x] Dysphagia
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [x] Strategies

Other:

Education Provided to:

- [x] Patient
- [x] Family
- [ ] Caregiver

Other: son

Education Provided by:

- [ ] Demonstration
- [x] Discussion
- [ ] Literature
- [ ] Video

Other:

Response to Education:

- [x] Voiced Understanding
- [x] Demonstrated Understanding
- [x] Learner Requires More Education

Other:

SLP Plan of Care

SLP Plan of Care STG & LTG

Speech Therapy Intensity/Frequency

- [ ] Not required
- [x] 1 hour/day minimum of 5 days per week
- [ ] 0.5 hours/day minimum of 5 days per week
- [ ] 0.5 hour/week, 3-5 days per week

Other Intensity/Frequency:

Total number of days anticipated: 21

Patient/Family Goals

Improve my memory and speech

SLP Participated in Development of Plan of Care

Speech Therapist who participated in the Individualized Plan of Care:

Cleary, Mara MA-CCC-SLP

Score: 0.00

Electronically Signed By:

Mara Cleary, CCC-SLP, 03/14/2026 22:26

3/14/2026	17:16	Mara Cleary, CCC-SLP	Note Discipline: Speech-Language Pathologists	Tag: Therapy Evaluation

Form: SLP Dysphagia Evaluation [P18]

Session Time

Session Time for Evaluation

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Time In: 03:30 PM
Time Out: 04:00 PM
Total Time of Session: 30
Session Comments:

Late Entry

- [ ] This is a late entry

SLP Patient History

Nutritional Approaches on Admission:

Check all of the following nutritional approaches that apply on admission

- [ ] A. Parenteral/IV Feeding
- [ ] B. Feeding Tube (e.g., nasogastric or abdominal (PEG))
- [ ] C. Mechanically altered diet - require change in texture of food or liquids (e.g., pureed food, thickened liquids)
- [ ] D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)
- [x] Z. None of the above

Precautions:

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
History
History of esophageal dysphagia

Disease Specific Programs:

History of Current Illness:

Patient is a 72 year old female with no know PMH initially transfer from Valleywise Maryvale ED to Valleywise Hospital on 3/12 with concerns of persistent dizziness and headaches. Patient states her symptoms have been ongoing for about 7 weeks but have recently increase to the point where she is unable to complete ADLS and the only thing that helps her dizzy spells is laying in a dark room. Vital signs in the ED temp 36.4, heart rate 64, respiratory rate 18, SP02 98% and BP 157/67. CMP bicarb 20 otherwise unremarkable. CTA of head and neck showed no acute no acute intracranial abnormality. CT chest and abdomen and pelvis showed no acute abnormality with no active pneumonia, diverticulosis without diverticulitis, small hiatal hernia and fusiform ectasia of the abdominal aorta measuring 2.6 cm. She was then admitted to OBs with Medicine for further monitoring and evaluation of Vertigo. She was started on Lisinopril. ECHO findings; Left ventricle is normal in size. Sigmoid shape septum with increased basal left ventricular septal wall thickness. Normal. Left ventricular systolic function with an EF of 54-74%. Biplane LVEF is calculated at 74%. PLOF independent with ADLs and mobility. CLOF Min to mod assist with ADLs and Ambulation mod assist for 2 (lateral steps).

Rehab Diagnosis:

Vertigo

Date of Onset

3/12/2026

Prior Level of Function:

Social History:

Patient lives in with son in a SLH with no STE and was previously independent in all ADLs, functional transfers, and ambulation. She is retired and was previously driving herself for leisure activities.

Subjective/Observations:

Patient pleasant and cooperative.

Preferred Language:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- (●) English
- ( ) Spanish
- ( ) American Sign Language
- ( ) Braille
- ( ) Korean
- ( ) German

Other: _____

Do you need or want an interpreter to communicate with a doctor or healthcare staff?

- ( ) Yes
- (●) No
- ( ) Unable to determine

What form of Interpreter was utilized?

- [ ] By Phone (list Interpreter ID number in comments)
- [ ] By Certified Interpreter (list name or Interpreter ID number in comments)
- [ ] Patient agreed to use family member interpreter (non-medical terminology used)
- [ ] Patient agreed to use bi-lingual staff interpreter (non-medical terminology used)
- [ ] Other (see comments)

Comments: _____

Hearing Functional:

- ( ) Yes
- (●) No
- ( ) Hearing Aid

Dentures/Partials:

- (●) None
- ( ) Upper
- ( ) Lower
- ( ) Both

Vision Functional:

- ( ) Yes
- ( ) No
- (●) Glasses
- ( ) Reading Glasses Only

Other Assistive Devices:

- [x] None
- [ ] Communication Board
- [ ] Glasses

Other: _____

Comments:

_____

Dysphagia Evaluation

MBSS:

- [x] N/A

Date: _____ [ ] Initial [ ] Repeat

Location: _____

Results: _____

Behavioral Characteristics:

- [x] Alert
- [x] Awake
- [x] Cooperative
- [ ] Lethargic
- [ ] Uncooperative
- [ ] Non-Responsive

Contraindications:

- [x] No Contraindications
- [ ] Active Respiratory Disease
- [ ] Decreased Cognition
- [ ] Absent Swallow Reflex
- [ ] Decreased Attention Span
- [ ] Impulsive
- [ ] Hyperactive Gag Reflex
- [ ] No Volitional Cough
- [ ] No Volitional Swallow

Liquid Consistencies Presented:

- [x] Thin (0)
- [ ] Slightly Thick (1)
- [ ] Mildly Thick (2)
- [ ] Moderately Thick (3)
- [ ] Extremely Thick (4)
- [ ] Nothing Presented

Comment: _____

Solid Consistencies Presented:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] Regular
- [ ] Easy to Chew (7)
- [ ] Soft & Bite Sized (6)
- [ ] Minced & Moist (5)
- [ ] Pureed (4)
- [ ] Liquidised (3)
- [ ] Nothing Presented

Comment:

Oral Motor:

	Impairment	Comments
Facial		
Labial		
Lingual		
Palate		
Larynx		
Mandible		
Other (Specify in Comment)		
Other (Specify in Comment)		

Dysphagia Symptoms with Liquid Consistencies

	Thin (0)	Slightly Thick (1)
Oral Prep		
Pooling (Rt/Lt/Bilateral)		
Increased Oral Transit Time		
Bolus Formation Difficulty		
Coughing/Choking	No	
Change in Voice Quality	No	
Delayed Swallow	No	
Decreased Laryngeal Elevation	No	
Other (Specify in Comments)		

Dysphagia Symptoms with Liquid Consistencies (Cont.)

	Mildly Thick (2)	Moderately Thick (3)
Oral Prep		
Pooling (Rt/Lt/Bilateral)		
Increased Oral Transit Time		
Bolus Formation Difficulty		
Coughing/Choking		
Change in Voice Quality		
Delayed Swallow		
Decreased Laryngeal Elevation		
Other (Specify in Comments)		

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Dysphagia Symptoms with Liquid Consistencies (Cont.)

	Extremely Thick (4)	Comments
Oral Prep		
Pooling (Rt/Lt/Bilateral)		
Increased Oral Transit Time		
Bolus Formation Difficulty		
Coughing/Choking		
Change in Voice Quality		
Delayed Swallow		
Decreased Laryngeal Elevation		
Other (Specify in Comments)		

Other/Comments:

Dysphagia Symptoms with Solid Consistencies

	Liquidised (3)	Pureed (4)
Oral Prep		
Pooling (Rt/Lt/Bilateral)		
Increased Oral Transit Time		
Bolus Formation Difficulty		
Coughing/Choking		
Change in Voice Quality		
Delayed Swallow		
Decreased Laryngeal Elevation		
Other (Specify in Comments)		

Dysphagia Symptoms with Solid Consistencies (Cont.)

	Minced & Moist (5)	Soft, Bite Sized (6)
Oral Prep		
Pooling (Rt/Lt/Bilateral)		
Increased Oral Transit Time		
Bolus Formation Difficulty		
Coughing/Choking		
Change in Voice Quality		
Delayed Swallow		
Decreased Laryngeal Elevation		
Other (Specify in Comments)		

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Dysphagia Symptoms with Solid Consistencies (Cont.)

	Regular	Easy to Chew (7)
Oral Prep	No	
Pooling (Rt/Lt/Bilateral)		
Increased Oral Transit Time	Yes	
Bolus Formation Difficulty		
Coughing/Choking	No	
Change in Voice Quality	No	
Delayed Swallow	No	
Decreased Laryngeal Elevation	Within Functional Limits	
Other (Specify in Comments)		

Dysphagia Symptoms with Solid Consistencies (Cont.)

	Comments
Oral Prep	
Pooling (Rt/Lt/Bilateral)	
Increased Oral Transit Time	
Bolus Formation Difficulty	
Coughing/Choking	
Change in Voice Quality	
Delayed Swallow	
Decreased Laryngeal Elevation	
Other (Specify in Comments)	

Other/Comments:

History of GERD and esophageal dilatation No s/s of aspiration noted or food reported going down slowly with all consistencies.

Results of Evaluation:

Mild oropharyngeal dysphagia and esophageal dysphagia. Follow up with GI if symptoms persist.

SLP Recommendations

Aspiration Risk:

	Aspiration Risk	Comments
Liquids	Minimal	
Solids	Minimal	

Diet Recommendations:

Foods	Drinks	Medications
Regular	Thin (0)	Regular

Diet Recommendation Comments:

Mild oropharyngeal dysphagia and esophageal dysphagia. Follow up with GI if symptoms persist.

Management:

- [ ] Independent Feeding
- [ ] 1:1 Supervision
- [x] Intermittent Supervision
- [ ] Diner's Club
- [ ] Trial Feed with SLP Only

Compensatory Startegies:

- [ ] None

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Wear Dentures
- [x] Sit Upright at 90 Degrees
- [ ] Chin Down
- [ ] Head Turn Right/Left
- [ ] Place Food on Rt Side
- [ ] Place Food on Lt Side
- [x] Occasional Cough/Throat Clear
- [ ] Repeat Swallows Per Bite
- [x] Small Bites/Sips
- [x] Alternate Solids/Liquids
- [x] Slow Rate
- [ ] Check Vocal Quality
- [ ] Check Mouth for Pocketing
- [ ] Oral Care After Meals
- [x] Upright after Meals for 30 Minutes

Recommendations:

- [ ] No Additional Skilled Services needed at this time
- [ ] MBSS
- [ ] OT Eval
- [x] Dietary Consult
- [ ] Barium Swallow
- [x] Speech/Cognitive Eval
- [ ] FEES
- [ ] Other: _____

Speech Therapy Intensity/Frequency

- [ ] Not required
- [x] 1 hour/day minimum of 5 days per week
- [ ] 0.5 hours/day minimum of 5 days per week
- [ ] 0.5 hour/week, 3-5 days per week

Other Intensity/Frequency: _____

Total number of days anticipated: 21

SLP Education

Education Provided on:

- [x] Goals for Inpatient Rehab/Plan of Care
- [x] Therapy Schedule
- [ ] Language Disorder
- [ ] Safety Awareness
- [ ] Aphasia
- [ ] Cognitive Dysfunction
- [ ] Dysarthria
- [x] Dysphagia
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [x] Strategies

Other: _____

Education Topic(s):

_____

Education Provided to:

- [x] Patient
- [x] Family
- [ ] Caregiver

Other: _____

Education Provided by:

- [ ] Demonstration
- [x] Discussion
- [ ] Literature
- [ ] Video

Other: _____

Response to Education:

- [x] Voiced Understanding
- [x] Demonstrated Understanding
- [x] Learner Requires More Education

Other: _____

Identified Problems/Deficits:

- [ ] None
- [x] Cognition/Communication
- [x] Swallowing Disorder

Other: _____

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

SLP Participated in Development of Plan of Care

Speech Therapist who participated in the Individualized Plan of Care:

Cleary, Mara MA-CCC-SLP

Score: 0.00

Electronically Signed By:

Mara Cleary, CCC-SLP, 03/14/2026 22:32

3/15/2026 9:46 Liviu Vladoianu, NP Note Discipline: Nurse practitioner Tag: Consult Note

Form: Provider Consult Note [39]

Coded Diagnoses

Admitting Diagnosis

#Debility - vertigo
#HTN
#HLD

Review of History

History of Present Illness

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

PMH/SH/FH reviewed

- [ ] No change since admission
- [ ] Changes/updates documented below

Past Medical History

Add comments to any medical history that needs further details

- [x] None
- [ ] AIDS/HIV
- [ ] A-Fib
- [ ] Allergic Rhinitis
- [ ] Alzheimer's
- [ ] Anemia
- [ ] Anxiety
- [ ] Arthritis (Osteo)
- [ ] Arthritis (Rheumatoid)
- [ ] Benign Prostate Hypertrophy
- [ ] CAD
- [ ] Cancer
- [ ] CHF
- [ ] Chronic Back Pain
- [ ] Cirrhosis
- [ ] Compression Fracture
- [ ] Dementia
- [ ] DJD
- [ ] Diabetes
- [ ] Diverticulitis/losis
- [ ] Depression
- [ ] Deep Vein Thrombosis
- [ ] Epilepsy
- [ ] GERD
- [ ] Glaucoma
- [ ] GI Bleed
- [ ] Gout
- [ ] Heart Disease
- [ ] Hiatal Hernia
- [ ] HTN
- [ ] Hyperlipidemia
- [ ] Kidney/Bladder

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Liver Disease
- [ ] Lung Disease
- [ ] Macular Degeneration
- [ ] MI (Not Primary Med Condtn)
- [ ] Obesity
- [ ] Orthopedic
- [ ] Osteoporosis
- [ ] Peptic Ulcer
- [ ] Peripheral Vascular Disease
- [ ] Pulmonary Embolis
- [ ] Seizures
- [ ] Sleep Apnea
- [ ] Stroke
- [ ] TB History
- [ ] Thyroid Disease
- [ ] TIA

Other Medical History: _____

Additional Comments: _____

Past Surgical History

Add comments to any surgical history that needs further details

- [x] None
- [ ] AICD
- [ ] Amputation(s)
- [ ] Anesthesia difficulties
- [ ] Angioplasty
- [ ] Appendectomy
- [ ] Biopsy (site)
- [ ] Bronchoscopy
- [ ] CABG
- [ ] Carotid (CEA)
- [ ] Carpal Tunnel
- [ ] Cataract(s)
- [ ] Cholecystectomy
- [ ] Craniotomy
- [ ] Esophageal Surgery
- [ ] Gastric Surgeries (Type)
- [ ] Heart Valve Surgery
- [ ] Hemorrhoidectomy
- [ ] Hernia Repair (location)
- [ ] Hysterectomy
- [ ] Joint Replacement (location)
- [ ] Lithotripsy
- [ ] Mastectomy (R/L/B)
- [ ] Orthopedic
- [ ] Ostomies (Describe)
- [ ] Pacemaker
- [ ] PEG
- [ ] Pneumonectomy (lobes)
- [ ] Spinal Surgery
- [ ] Stent Placement (location)
- [ ] Tonsillectomy
- [ ] Thyroid Surgery
- [ ] Tracheostomy

Other Surgical History: _____

Additional Comments: _____

Family Medical History

Have any of these conditions occurred on either side of the patient's family? (Check if yes)

- [x] Not applicable
- [ ] Allergies
- [ ] Cardiovascular disease
- [ ] Gastrointestinal disease
- [ ] Pulmonary disease
- [ ] Bleeding tendencies
- [ ] Tuberculosis
- [ ] Psychiatric illness
- [ ] Bone disease
- [ ] Kidney disease
- [ ] Congenital deformities
- [ ] Alcohol/drug dependence
- [ ] Cancer/Tumors
- [ ] Diabetes

Other: _____

Comments: _____

Marital Status

Marital Status: Widowed

Current or past use of alcohol?

- ( ) Yes
- (•) No

Recreational/Illicit Drugs

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] Denies Use
- [ ] Past History of Use
- [ ] Current Use

Type: _____ Frequency: _____

Date last used: _____

Tobacco/Smoking

- [ ] Current Smoker Packs/day: _____ How many years: _____
- [x] Non-Smoker

Previous History of Smoking (indicate last time used and previous PPD): _____

- [ ] Vapes Vapes Frequency: _____
- [ ] Marijuana Marijuana Frequency: _____

Other: _____

Consult ROS

Review of System Status Update

- [ ] N/A (Admission Review of Systems)
- [ ] NO CHANGES in Condition Since Last System Review.
- [ ] CHANGES HAVE Occurred & Amended Below to Reflect an Update to the Patient's Current Condition.
- [ ] Unable to Obtain Review of Systems. Reason: _____

Review of Systems

_____

Complications HEENT:

Check All that Apply.

- [ ] Jaundice
- [ ] Diplopia
- [ ] Photophobia
- [ ] Rhinitis
- [ ] Nose Bleeds
- [ ] Packed Left Nostril
- [ ] Packed Rt. Nostril
- [ ] Tooth ache
- [ ] Ear ache
- [ ] Sore throat
- [ ] Sinus tender
- [ ] Lymphadenopathy

Other: _____

- [ ] None of the Above

Complications of Respiratory System:

Check All that Apply.

- [ ] Intubated
- [ ] SOB at Rest
- [ ] SOB with Exertion
- [ ] Orthopnea
- [ ] Snores
- [ ] Hemoptysis
- [ ] Chills
- [ ] Night Sweats
- [ ] Fatigue
- [ ] Recent Weight Loss
- [ ] Ineffective & Non-Productive Cough
- [ ] Frequent Suctioning
- [ ] Persistent cough less than 2 weeks
- [ ] Persistent cough greater than 2 weeks
- [ ] Tracheostomy
- [ ] Sleep Apnea
- [ ] Oxygen Dependent

Other: _____

- [ ] None of the Above

Complications of Circulatory System:

Check all that Apply.

- [ ] HTN
- [ ] Hypotension
- [ ] Orthostatic Hypotension

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- [ ] Tachycardia
- [ ] Bradycaria
- [ ] Dysrhythmia
- [ ] Edema
- [ ] History of Chest Pains at Rest
- [ ] History of Chest Pains with Activity

Other: _____

- [ ] None of the Above

Complications GI System:

Check all that Apply.

- [ ] Vomiting in Past 72 Hours
- [ ] Diarrhea Within past 72 Hours
- [ ] Chronic Constipation
- [ ] Weight Loss
- [ ] Weight Gain
- [ ] Indigestion
- [ ] Reflux
- [ ] Rectal Bleeding
- [ ] Tarry Stools
- [ ] Rectal Tube
- [ ] Gastric Decompression
- [ ] Jaundice
- [ ] Encephalopathy

Other: _____

- [ ] None of the Above

Complications Endocrine System:

Check all that Apply.

- [ ] Episodes of Hypoglycemia
- [ ] Episodes of Hyperglycemic
- [ ] Frequent Urination
- [ ] Excessive Thirst
- [ ] Fatigue/Malaise
- [ ] Hyperactivity

Other: _____

- [ ] None of the Above

Complications Genitourinary System:

Check all that Apply.

- [ ] Incontinent
- [ ] Neurogenic Bladder
- [ ] Anuria
- [ ] Hematuria
- [ ] Cloudy
- [ ] Odor
- [ ] Urgency
- [ ] Retention
- [ ] Difficulty starting stream
- [ ] Vaginal/Penile Bleeding or Drainage

Other: _____

- [ ] None of the Above

Complications Musculoskeletal/Mobility:

Check all that Apply.

- [ ] Fall Risk
- [ ] Activity Intolerance
- [ ] Poor Balance
- [ ] Weakness/Fatigue
- [ ] Joint Swelling
- [ ] Joint Pain
- [ ] Contractures
- [ ] Dizziness
- [ ] Amputation
- [ ] Hemiplegia
- [ ] Monoplegia
- [ ] Quadrapeligia

Other: _____

- [ ] None of the Above

Complications of Pain:

Check all that Apply.

- [ ] Chronic Pain
- [ ] Intractable Pain
- [ ] Decreased Mental Status
- [ ] Agitation
- [ ] Inability to Concentrate
- [ ] Anxiety
- [ ] Nausea/Vomiting
- [ ] Constipation
- [ ] Itching

Other: _____

- [ ] None of the Above

Pain Management:

- [ ] N/A
- [ ] Heat
- [ ] Ice
- [ ] TENS Unit

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Positioning
- [ ] Biofeedback
- [ ] Medication

Comments:

Complications Skin/Wound:

Check all that Apply.

- [ ] Urinary Incontinence
- [ ] Fecal Incontinence
- [ ] Wound Drainage
- [ ] Erythema
- [ ] Skin Maceration
- [ ] Skin Tear
- [ ] Incision Dehiscense
- [ ] Incision Evisceration
- [ ] Stoma Care
- [ ] Poor Nutrition
- [ ] Immobility

Other:

- [ ] None of the Above

Complication Neuro/Mental Status:

Check all that Apply.

- [ ] Altered Mental Status
- [ ] Dementia Confusion
- [ ] Emotional Liability/Depression
- [ ] Seizures
- [ ] Headache
- [ ] Dizziness/Balance Loss
- [ ] Speech Problem
- [ ] Loss of Gag Reflex
- [ ] Loss of Memory
- [ ] Loss of Safety Awareness
- [ ] Paralysis - Location:

Other:

- [ ] None of the Above

All Recent Investigations Noted

- ○ All Recent Investigations Noted

Consult PE

Blood Pressure:

Systolic (mmHg): 124 Diastolic (mmHg): 59 Position: Lying Side: Left

Comment:

Pulse:

B/min: 68 Position: Lying Site: Monitor Type: Regular

Comment:

Respiration:

Per Minute: 18 Vent: No

Comment:

Weight:

lb: 183.0 Scale: Bed

Comment:

Height:

Feet: 5 Inches: 1

Comment:

Physical Exam

General:

- [ ] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Oriented to person ☐ Oriented to place ☐ Oriented to time ☐ Oriented to situation

Comments: _____

Skin:

☐ Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments: _____

☐ Abnormal

Abnormal findings: _____

HEENT:

☐ Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Neck:

☐ Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Heart:

☐ Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Lungs/Chest:

☐ Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Abdomen:

☐ Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Extremities:

☐ Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Back:

☐ Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Gait and station are normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Romberg test is normal.

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Plantar responses are down going.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- ☐ Normal
- ☐ Abnormal

Other: _____

Sensory: Sensory examination to pinprick and vibration is normal.

- ☐ Normal
- ☐ Abnormal

Other: _____

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed _____

Comment/Non-interfaced results: _____

Diagnostic Imaging Reviewed

_____

Comment/Non-interfaced results: _____

Microbiology Tests Reviewed

Comment: _____

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

_____

Consult Impression/Plan

Action Plan

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ New Action Plan Initiated ☐ Continue Same Course of Care ☐ Updated Action Plan Initiated

Plan:

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

#Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist
#HTN
- lisinopril
#HLD
- on statin
#Skin
- monitor per nursing protocol for decubitus ulcers
- frequent offloading as indicated
#Bowel - constipation
- colace/senna
- lactulose, dulcolax suppository PRN
#Bladder
- monitor for retention
- check PVRs
#DVT ppx
- SCDs

Impression

03/15/2026 seen in the common area, walking with therapy, no acute events, stable

#Debility - vertigo
#HTN
#HLD
99232
Patient is Full Code

Neuro/Psych Assessment and Plan

Respiratory Assessment and Plan

Cardiovascular Assessment and Plan

Renal/Genitourinary Assessment and Plan

Infectious Disease Assessment and Plan

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Gastrointestinal/Stress Ulcer prophylaxis/Nutrition Assessment and Plan

DVT prophylaxis Assessment and Plan

Wounds/Pressure ulcer Assessment and Plan

Medications Reviewed

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

I have Reviewed the Following Patient Data:

- ☐ Prior Medical Records & Test Results
- ☐ Medication Reconciliation
- ☐ Allergies
- ☐ Consultant Report(s)
- ☐ Current/New Medications
- ☐ Current Lab Result & Diagnostic Test
- ☐ I Have Personally Discussed Code Status with this Patient or their Surrogate Decision Maker and Believe they Understand their Medical Condition, Prognosis and the Consequences of their Code Status Decision.
- ☐ All of the Above

Anticipated Post Hospital Service Needs:

- ☐ Home with Help
- ☐ Home with Home Health
- ☐ Assisted Living Facility
- ☐ SNF
- ☐ Acute Inpatient Rehab
- ☐ Home with Outpatient Therapy
- ☐ Hospice
- ☐ Group Home
- ☐ Post Acute Vent Care Unit

Other: _____

Coordinated care with

- ☐ Consultants
- ☐ Nursing
- ☐ Nutrition
- ☐ Therapist
- ☐ Family
- ☐ Attending
- ☐ PA

Other: _____

Plan Discussed in Detail with:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Patient ☐ Family ☐ Nursing Staff ☐ Case Manager

Comment:

Other

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

Physician Addendum

Electronically Signed By:

Liviu Vladoianu, NP, 03/15/2026 17:49

3/15/2026	9:50	Tammy Wright, COTA	Note Discipline: Occupational Therapist Assistant	Tag: Daily Note

Form: OT Note [P13]

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 09:30 AM

Time Out: 10:30 AM

Total Time of Session: 60

Late Entry

☐ This is a late entry

Will this note include a Weekly Team Conference note?

(●) Yes ( ) No

OT Subjective

Subjective/Observations:

Pt supine in bed upon arrival. Agreeable to OT.

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall Dizziness Light sensitivity Cognitive communication deficits/Word retrieval deficits History of esophageal dysphagia

Vitals Taken

Did you take Vitals Pre-Treatment?

○ Yes ○ No

Did you take Vitals During Treatment?

○ Yes ○ No

Did you take Vitals Post-Treatment?

○ Yes ○ No

OT Objective for Daily Note

Therapeutic Intervention

Therapeutic Exercise:

Theraband Color: _____

Weights (lbs): _____

Therapeutic Activity:

Prosthetic/Orthotic Management

Self Care Home Management:

Pt ambulated 200' with FWW, brushed teeth setup, groomed setup, pt educated with proper hand placement for FWW safety. See Conf grid

IADL:

- ☐ Communication (Phones, computers etc)
- ☐ Financial Mgmt
- ☐ Health Mgmt (Scheduling appts)
- ☐ Household Mgmt
- ☐ Medication Mgmt
- ☐ Meal Prep
- ☐ Shopping (Making lists, selecting items, payment method)
- ☐ Transportation Mgmt
- ☐ Other _____

IADL Comments: _____

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Neuromuscular Re-Education:

Cognitive Function:

W/C Mobility/Management Training

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

What additional test(s) were performed for OT?

- [ ] None
- [ ] Amputee Mobility Predictor Assessment Tool
- [ ] Function in Sitting Test (FIST)
- [ ] Katz Index of Independence in Activities of Daily Living
- [ ] Nine Hole Peg Test

Other:

Additional Treatment:

Comments:

OT Assessment

OT Assessment

See Conf Grid. Pt continues to demo dizziness

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

OT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Impaired Safety Awareness
- [ ] Impaired Vision/Perception
- [x] Decreased Strength
- [ ] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [ ] Impaired Communication

DME Needed:

- [ ] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers:

OT Plan of Care

OT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [ ] 1.5 hours/day minimum of 5 days per week

Other Intensity/Frequency: 30-180 mins/ 5-7 days/wk

Total number of days anticipated: 14

Patient/Family Goals:

Not really I think I'm doing okay, I can go home and do whatever I need to do

OT Education

Education Provided On:

- [ ] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [ ] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule
- [ ] Sexual Health
- [ ] Vision Changes

Use of Equipment:

Other:

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other:

Family/Caregiver Training

na

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

OT Special Tests

OT Special Tests

	Admission Score	Week 1 Score	Week 2 Score	Week 3 Score
Actv Measure Post Acute Care				
Catherine Bergego Scale				
Function in Sitting Test				
Functional Reach				
Hand Grip Dynamometer (L)				
Hand Grip Dynamometer (R)				
Katz Index Independence ADLs	3/6			
Lawton IADL Scale				
Modified Barthel				
Nine Hole Peg Test				
Trail Making Test Parts A & B				
Other (Specify in Comments)				

OT Special Tests (Cont.)

	Week 4 Score	Week 5 Score	Discharge Score
Actv Measure Post Acute Care			
Catherine Bergego Scale			
Function in Sitting Test			
Functional Reach			
Hand Grip Dynamometer (L)			
Hand Grip Dynamometer (R)			
Katz Index Independence ADLs			
Lawton IADL Scale			
Modified Barthel			
Nine Hole Peg Test			
Trail Making Test Parts A & B			
Other (Specify in Comments)			

OT Test Interpretation

Katz Index

Katz - Bathing

Need help with bathing more than one part of the body, getting in or out of the tub or shower. Requires total bathing (0)

Bathes self completely or needs help in bathing only a single part of the body such as the back, genital area or disabled extremity. (1)

Katz - Bathing Total Score: 0

Katz - Dressing

Needs help with dressing self or needs to be completely dressed. (0)

Get clothes from closets and drawers and puts on clothes and outer garments complete with fasteners. May have help tying shoes. (1)

Katz - Dressing Total Score: 0

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Katz - Toileting
Needs help transferring to the toilet, cleaning self or uses bedpan or commode. (0)
Goes to toilet, gets on and off, arranges clothes, cleans genital area without help. (1)
Katz - Toileting Total Score: 1

Katz - Transferring
Needs help in moving from bed to chair or requires a complete transfer. (0)
Moves in and out of bed or chair unassisted. Mechanical transfer aids are acceptable (1)
Katz - Transferring Total Score: 0

Katz - Continence
Is partially or totally incontinent of bowel or bladder (0)
Exercises complete self control over urination and defecation. (1)
Katz - Continence Total Score: 1

Katz - Feeding
Needs partial or total help with feeding or requires parenteral feeding. (0)
Gets food from plate into mouth without help. Preparation of food may be done by another person. (1)
Katz - Feeding Total Score: 1

Total Score: 3
Interpretation:
6= High, full functional independence.
4= Moderate impairment.
2 or below= low, severe functional impairment, dependence

OT Objective for Team Conference

Did OT/COTA shower/bathe the patient today?

- (●) No
- ( ) Yes

Activities of Daily Living

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Self Feeding	SBA	SBA				
Oral Hygiene	Min	Setup				
Grooming	CGA	Setup				
Toilet Hygiene	CGA	CGA				
Bathing/Shower	CGA	CGA				
Dressing UE	SBA	SBA				
Dressing LE	CGA	SBA				
Putting On/Taking Off Footwear	SBA	SBA				

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Comments for ADLs for Weekly Team Conference

Current Functional Status:
Feeding- SBA
G/H-SBA/min
Toileting-CGA
Bathing-CGA
UBD -CGA
LBD-SBA
Footwear-SBA
Toilet Transfer-SBA
Shower/tub transfer -SBA
Goals met: 0/3 STG, 0/3 LTG

OT Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Bed		SBA				
Toileting		SBA				
Tub						
Shower						

Comments for OT Mobility for Team Conference

Wheelchair Mobility not assessed

- [ ] On Admission
- [ ] Week 1
- [ ] Week 2
- [ ] Week 3
- [ ] Week 4

Wheelchair Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance						

Type of Wheelchair

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

Level of Assistance with Wheelchair Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						

Comments for Wheelchair Mobility

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

OT Weekly Progress Team Conference

Functional Goals Focused On for Team Conference:

Outcomes/Goals:
1. STG: Patient will complete standing grooming tasks at sink for 5 min with SBA and no LOB to improve endurance for ADL participation
2. LTG: Patient will complete grooming standing at the sink for greater than 5 minutes with no LOB, demonstrating improved activity tolerance for self care.
3. STG: Patient will complete functional reaching outside BOS during standing ADL task with CGA and no LOB to improve dynamic balance for daily activities.
4. LTG: Patient will perform functional mobility during ADL tasks using AD safely and with no LOB to improve safety awareness and independence with ADLs.
5. STG: Patient will complete LB dressing in seated position with SBA and minimal vc for safety awareness to reduce fall risk during ADLs.
6. LTG: Patient will complete UB/LB dressing independently in seated position without LOB or unsafe standing to maximize independence and reduce fall risk.

OT Weekly Progress Towards Functional Goals:

Current Functional Status:
Feeding- SBA
G/H-SBA/min
Toileting-CGA
Bathing-CGA
UBD -CGA
LBD-SBA
Footwear-SBA
Toilet Transfer-SBA
Shower/tub transfer -SBA
Goals met: 0/3 STG, 0/3 LTG

Endurance for Team Conference (eg Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Needs Continued Focus on:

Continue POC

Score: 0.00

Electronically Signed By:

Tammy Wright, COTA, 03/15/2026 10:51

3/16/2026 5:53 Sylvia Valenzuela, RN Note Discipline: Case Manager Tag: Admission

Form: Case Management Initial Assessment [CM1]

Patient Information

How was information obtained/collected for this assessment?

- [x] Patient
- [x] Family
- [x] Review of copied/faxed medical record from referring hospital

Other: Michael son 602 577 6496.

Admission Date:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Admission Date: 3/13/2026

Admission Class:

01.Initial Rehab

Admit From:

02. Short-Term General Hospital

Re-Admission:

- [x] No
- [ ] Yes- (from acute care) Prior Admit Date: _____
- [ ] Yes - (from this facility) Recent DC Date: _____

Rehab Diagnosis:

Vertigo

Date of Onset

3/12/2026

Pre-Hospital Living Setting:

01. Home (private home/apt., board/care, assisted living, group home, transitional living)

Pre-Hospital Living with:

02. Family/Relatives

Advance Directives/Living Will/Power of Attorney:

- [ ] Advance Directives
- [ ] Living Will
- [ ] Copy Requested Conservator: _____
- [ ] No Advance Directives/Living Will - Educational Packet/Counsel Given
- [x] No Advance Directives/Living Will - Educational Packet/Counsel Declined
- [ ] Medical Power of Attorney MPOA Name: _____ [ ] Activated
- [ ] MPOA Copy Obtained

MPOA Copy Requested From: _____

Comments: _____

Do you need or want an interpreter to communicate with a doctor or healthcare staff?

- ( ) Yes
- (•) No
- ( ) Unable to determine

What form of Interpreter was utilized?

- [ ] By Phone (list Interpreter ID number in comments)
- [ ] By Certified Interpreter (list name or Interpreter ID number in comments)
- [ ] Patient agreed to use family member interpreter (non-medical terminology used)
- [ ] Patient agreed to use bi-lingual staff interpreter (non-medical terminology used)
- [ ] Other (see comments)

Comments: _____

Marital Status:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Marital Status: Widowed

What is your preferred language?

Primary Language: english

Education - Highest Level Completed:

- [ ] Elementary
- [x] High School
- [ ] GED
- [ ] Some College
- [ ] Associate Degree
- [ ] Bachelors Degress
- [ ] Masters Degree
- [ ] Doctorate degree

Contact Assisting with Patient Care Needs

Contact Assisting with Patient Care Needs:

Name: Michael
Cell Phone: (602) 577-6496
Home Phone:
Address:
Relationship: son
Contact Email:

- [ ] MPOA
- [ ] Guardian

Contact Assisting with Patient Care Needs:

Name:
Cell Phone:
Home Phone:
Address:
Relationship:
Contact Email:

- [ ] MPOA
- [ ] Guardian

Available to Assist at Discharge?

- [x] Yes
- [ ] No

If Yes, Days/Hours Available: Michael son 602 577 6496 will be involved with discharge plan.

Financial Resources

Primary Insurance:

- [ ] Medicaid
- [ ] Managed Medicaid
- [ ] Workers Comp
- [ ] Veteran
- [ ] Registered with local VA
- [x] Medicare Part A
- [x] Medicare Part B
- [ ] Medicare Part C
- [ ] Medicare Part D
- [ ] Unfunded or Self Pay
- [ ] Commercial
- [ ] Medicare Advantage Plan

Secondary Insurance:

- [ ] Medicaid
- [ ] Managed Medicaid
- [ ] Workers Comp
- [ ] Veteran
- [ ] Registered with local VA
- [ ] Medicare Part A
- [ ] Medicare Part B
- [ ] Medicare Part C
- [ ] Medicare Part D

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- [ ] Unfunded or Self Pay

Medicare Advantage Plan:
Medical Advantage Plan:
Commercial: Aetna
Other:

Financial Resources Available:

- [ ] None
- [x] SSI
- [ ] SSDI
- [ ] Savings
- [ ] Pension
- [ ] Rx Benefits
- [ ] Veteran Services
- [ ] Annuities
- [ ] State Disability Insurance

Other:

Is the patient Employed?

- ( ) Yes
- (•) No

Occupation:

- [x] Retired due to Age
- [ ] Retired due to Disability
- [ ] Homemaker
- [ ] Student
- [ ] Unemployed
- [ ] Employed Full Time
- [ ] Employed Part Time

Type of Occupation: post office
Barriers for Returning to work: none

Would you like Vocational Rehab Resources

- [ ] Yes
- [x] No

Resources Provided:

Living Arrangements/Support Systems Relative to Post Discharge Needs

Living:

- [ ] Alone
- [ ] Significant Other/Spouse
- [x] w/ Family
- [ ] w/ Sitters
- [ ] Domestic Partner
- [ ] Caregivers Hours/Day/Week:
- [ ] Assisted Living Facility Amount of Assistance Received:

Other/Comments: Michael son 602 577 6496

Structure:

- [x] House
- [ ] Apartment
- [ ] Mobile Home
- [ ] RV/Travel Trailer
- [ ] Homeless
- [ ] Ramp/Rail Access
- [ ] Bi-Level Home
- [ ] Single Level
- [ ] Elevator Access

Steps to Enter: 0
Household Barriers: none

Support:

- [x] Family
- [ ] Friends
- [ ] Adult Day Care Services Utilized
- [ ] Home Health Home Health Company:
- [ ] Waiver Waiver Hrs/Days/Week:

Other: Michael son 602 577 6496

Assessment of the patient s and/or support person s ability to provide self-care/care?

Patient admitted from ValleyWise for 3/12 with concerns of persistent dizziness and headache. Patient has the following orders; Aspiration Precautions, Aspirin, Fall Precautions, Antivert, Neurology Consult. Patient lives in a single level home with Michael son 602 577 6496,

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

patient does not own any DME. Discharge plan is to return home with home health.
Follow up appointments: ValleyWise 602 344 1300 at 2601 E. Roosevelt Street Phx

Cultural/Spiritual Preferences:

- [x] N/A

Catholic

Spiritual Concerns:

- [ ] Yes
- [x] No

If Yes, Specify:

Mental/Cognitive/ Communication/Psychosocial

Able to make own decisions:

- (●) Yes
- ( ) No

Mental Status:

- [x] Alert
- [x] Oriented to Person
- [x] Oriented to Time
- [x] Oriented to Place
- [x] Oriented to Situation
- [ ] Confused
- [ ] Forgetful
- [ ] Impulsive
- [ ] Non-Verbal
- [ ] Lethargic
- [ ] Able to Focus on Topic/Task
- [ ] Aware of Diagnosis
- [ ] Memory Problems

Other:

Psychosocial Concerns:

- [x] No psychosocial concerns at this time. Will continue to monitor.
- [ ] Hx of Depression/Anxiety
- [ ] Adjustment to Disability
- [ ] Coping with Illness/Disability
- [ ] Suicidal Behavior/Thoughts
- [ ] Body Image Changes
- [ ] Lifestyle/Role Reversal Changes
- [ ] Recent Loss of Loved One
- [ ] Lack of Support
- [ ] Family Dynamics
- [ ] Substance Abuse Hx
- [ ] Victim of Assault Hx
- [ ] Difficulty with Pain Management
- [ ] Financial Concerns

Other:

History of Prior Psychology Consult/Counseling:

- ( ) Yes
- (●) No

Any history of Drug/ETOH abuse?

- ( ) Yes
- (●) No

Treatment received for Drug/ETOH/Mental Health reason?

- [ ] Yes
- [x] No
- [ ] N/A

If Yes, Treatment Location:

Signs/symptoms/verbalized concern of abuse or neglect?

- [ ] Yes (Notify Appropriate Agencies)
- [x] No

If Yes, Comment:

History of APS involvement?

- [ ] Yes
- [x] No

If Yes, Comment:

Prior Level of Functioning

Transport Resources:

- [ ] Drive Self
- [x] Family
- [ ] Public Transporation

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- [ ] Handicap Community Resources
- [ ] Friend
- [ ] Neighbor

Other: Michael son 602 577 6496

Prior Level of Function:

- [x] Independent
- [ ] Modified Independent
- [ ] Minimal Assistance
- [ ] Moderate Assistance
- [ ] Maximal Assistance
- [ ] Total Care

Comment:

Needed Assistance With:

- [x] No Assistance Needed
- [ ] ADLs
- [ ] Transfer/Mobility
- [ ] Meal Prep/Feeding
- [ ] Toileting
- [ ] Transportation
- [ ] Dressing and/or Bathing
- [ ] Homemaking Services
- [ ] Medication (Administration & Cost)
- [ ] Cost of Living Expenses (Food, Housing, Utilities)

Comment:

Established Providers:

Include Contact Information

Primary Care: Dr Stanley Coombs 602 864 8800
Specialist(s):
Clinic:
Pharmacy: CVS 602) 275-0120
Other:

Current DME in Home:

- [x] No DME in Home
- [ ] BSC
- [ ] Scooter
- [ ] Lifter
- [ ] Glucometer
- [ ] Tub Bench
- [ ] Nebulizer
- [ ] CPAP
- [ ] BiPAP
- [ ] Oxygen

Oxygen Amt:

- [ ] Manual Walker
- [ ] Rolling Walker
- [ ] Walker w/ Seat
- [ ] Straight Cane
- [ ] Quad Cane
- [ ] Wheelchair
- [ ] Shower Chair
- [ ] Grab Bars
- [ ] Hospital Bed/Specialty Mattress:

Other/Comments:

DME Providers:

- [x] No DME Providers

Currently Scheduled Appointment(s) Comments:

Follow up appointments: ValleyWise 602 344 1300 at 2601 E. Roosevelt Street Phx

Anticipated Discharge Needs Dependent on Response to Treatments(s)

Anticipated Discharge Needs:

- [ ] Skilled Placement
- [ ] Long Term Care
- [ ] Acute Rehab
- [ ] Home - Self Care
- [ ] Home Care w/Private Duty/Sitter
- [x] Home, Home Health Assist
- [ ] Hospice
- [ ] ALF
- [ ] Outpatient Therapy
- [ ] LTAC
- [ ] APS
- [ ] Vocational Rehab
- [ ] DMV Letter
- [ ] Meals on Wheels
- [ ] Disabled Parking Application
- [ ] Veteran's Services
- [ ] Financial Resources
- [ ] Support Group
- [ ] Social Services
- [ ] Transportation Assistance

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- [ ] Infusion Services
- [ ] Wound Vac

Outpatient Clinic: _____

Other: _____

Anticipated Discharge Location

Home with Home Health

Dialysis:

- [x] N/A
- [ ] New Placement
- [ ] Hemodialysis
- [ ] Peritoneal
- [ ] HD Established
- [ ] MWF
- [ ] TTS
- Vendor/Location: _____
- [ ] PD Establshied
- [ ] Daily
- [ ] Nightly
- Vendor/Location: _____

Reason for Admission/Patient Goals

Case Management Discharge Barriers Identified:

- [ ] N/A
- [ ] Safety
- [ ] Social
- [ ] Medical
- [ ] Functional
- [ ] Financial
- [ ] Lack of Understanding
- [ ] Lack of Family Support
- [ ] Cognitive Defecits
- [ ] Communication Defecits
- [ ] Behavioral Issues
- [ ] Emotional Issues
- [x] Decreased Strength
- [x] Decreased ROM
- [ ] Environmental
- [ ] Architechtural
- [ ] Transportation

Stated Patient Goals and Treatment Preferences:

feel better

Additional Information:

none

Case Management Education Record

Case Management Areas Reviewed:

- [x] Patient Information Folder
- [x] Discharge Planning
- [x] Plan of Care, Treatment & Services
- [x] Community Resources
- [x] Case Manager's Role
- [x] Patient Rights and Responsibilities
- [x] Orientation & Education
- [x] Advanced Directives & Power of Attorney
- [ ] LTAC/SNF/Swing Bed/Rehab Program

Current Knowledge Assessment:

- (●) Understands basic information & has necessary skills for self-management
- ( ) Some understanding of basic information; needs reinforcement
- ( ) Does not understand basic information
- ( ) Not applicable

Factors Affecting Learning:

- [x] None
- [ ] Visual
- [ ] Hearing
- [ ] Inability to read
- [ ] Cultural/Spiritual
- [ ] Language
- [ ] Cognitive impairment
- [ ] Emotional barriers
- [ ] Unable to comprhend

Other: _____

Learner:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- ○ Patient
- ○ Family/Significant Other
- ◉ Both

Readniess to Learn:

- ◉ Motivated
- ○ Denying
- ○ No Family Available
- ○ No Interest
- ○ Refuses

Preferred Teaching Method:

- ☑ Verbal Discussion
- ☐ Demonstration
- ☐ Handout/Print Material

Teaching Method:

- ☑ Verbal
- ☐ Audio/Video
- ☐ Telephone
- ☐ Demonstration
- ☐ Handouts
- ☐ Other

Outcomes:

- ☑ Verbalizes Understanding/Proficiency
- ☐ Correct Return Demonstration
- ☐ Needs Practice/Reinforcement
- ☐ Needs Additional Instruction/Supervision

Additional Information:

none

CM Participated in Development of POC

Case Manager who participated in the Individualized Plan of Care:

Valenzuela, Sylvia RN CM

Score: 0.00

Electronically Signed By:

Sylvia Valenzuela, RN, 03/16/2026 18:53

3/16/2026 7:26 Margaret M Demer, PT, DPT Note Discipline: Physical Therapist Tag: Daily Note

Form: PT Note (P33)

Session Information

Amount of the session patient attended

- ◉ Entire Session
- ○ Partial (missed some of the session)
- ○ None (missed entire visit)

Session Time

Time In: 01:58 PM
Time Out: 02:58 PM
Total Time of Session: 60

Late Entry

- ☐ This is a late entry

Will this note include a Weekly Team Conference note?

- ◉ Yes
- ○ No

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT Subjective

Subjective/Observations:

Pt received in bed in room with son present, agreeable to PT session at this time. Pt denies dizziness, endorses HA. Pt's son present throughout this session.

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vitals Taken

Did you take Vitals Pre-Treatment?

○ Yes ○ No

Did you take Vitals During Treatment?

○ Yes ○ No

Did you take Vitals Post-Treatment?

○ Yes ○ No

PT Objective for Daily Note

Gait Deviations:

Therapeutic Intervention

Therapeutic Exercise:

Therapeutic Activity:

supine>sitting EOB SBA
pt performed several STSs this session CGA-SBA
pt performed car transfer with FWW SBA, cueing for safety
pt ascended and descended 12 stairs with BHR CGA-SBA

Prosthetic/Orthotic Management

Exercise Equipment (Sci Fit / Nu Step, etc.):

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Neuromuscular Re-Education:

to improve balance, decrease future fall risk:
-3x30" AE pad stance with eyes open/closed, fwd reach, narrow BOS, and other challenges. CGA-MinA for steadying throughout
-3x30" modified SLS with 2in step CGA-MinA for steadying

Gait:

pt ambulated with FWW ~150 ft x4, ~250 ft x2 CGA pt ambulated with no AD ~60 ft with CGA-MinA for steadying

W/C Mobility/Management Training

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Additional LE Measurements taken?

What special tests were performed for PT

- [ ] None
- [ ] 2 Minute Walk Test
- [ ] 6 Minute Walk Test
- [ ] 10 Meter Walk Test
- [ ] Timed Up and Go (TUG)
- [ ] Berg
- [ ] Tinetti Assessment
- [ ] Activities-Specific Balance Test
- [ ] Function in Sitting Test (FIST) Results
- [ ] 5 Time Sit to Stand Test
- [ ] Amputee Mobility Predictor Assessment Tool
- [ ] Functional Gait Assessment
- [ ] Lower and Thoracic Evaluation

Other:

Additional Treatment:

PT Assessment

PT Assessment

Pt tolerated session this PM well. Pt demonstrates some difficulty with balance challenges performed this date, would benefit from continued balance training in future sessions. This session concluded with pt seated in room with son present, all needs met.

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

PT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Decreased Strength
- [x] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [x] Impaired Safety Awareness
- [ ] Impaired Communication

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Impaired Vision/Perception

DME Needed:

- [ ] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers:

PT Plan of Care

PT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency:

Total number of days anticipated: 14

Patient/Family Goals:

Return to PLOF of Indep c ADLs, amb and grooming

PT Education

Education Provided On:

- [ ] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [ ] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Weight Bearing Precautions
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule

Use of Equipment:

Other:

Education Provided To:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Patient ☐ Family ☐ Caregiver

Other:

Response to Education:

☐ Voiced Understanding ☐ Demonstrated Understanding ☐ Learner Requires More Education

Other:

Education Provided By:

☐ Demonstration ☐ Discussion ☐ Literature ☐ Video

Other:

Identified Problems/Deficits:

☐ Knowledge Deficit ☐ Safety Awareness ☐ Motor Function Impairment

Other:

Family/Caregiver Training

initiated with son

PT Special Tests

PT Special Tests

	Admission Score	Week 1 Score	Week 2 Score	Week 3 Score
2 Minute Walk Test				
5 Times Sit to Stand				
6 Minute Walk Test				
10 Meter Walk Test				
Activities-Specific Balance				
Actv Measure Post Acute Care				
Amputee Predictor Assess Tool				
Berg Balance Test				
Elderly Mobility Scale				
Function in Sitting Test				
Functional Gait Assessment				
Timed up and Go Test				
Tinetti Test				
Other (Specify in Comments)	Dix Hallpike			

PT Special Tests (Cont.)

	Week 4 Score	Week 5 Score	Discharge Score
2 Minute Walk Test			
5 Times Sit to Stand			
6 Minute Walk Test			
10 Meter Walk Test			
Activities-Specific Balance			
Actv Measure Post Acute Care			
Amputee Predictor Assess Tool			
Berg Balance Test			
Elderly Mobility Scale			
Function in Sitting Test			

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Functional Gait Assessment			
Timed up and Go Test			
Tinetti Test			
Other (Specify in Comments)			

PT Test Interpretation

+ Dix on Right for BPPV

Modified Rankin Score

Is the patient in the Rehab Stroke Program?

If yes, the Modified Rankin Scale (mRS) assesses the patient's level of disability or dependence in daily activities following a stroke or other neurological condition. Inform the patient (or caregiver, if the patient is unable to respond) that you will be asking questions about their usual daily functioning since the stroke or illness.

○ Yes ◉ No

Modified Rankin Score Total Score: .00

PT Objective for Team Conference

Functional Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Rolling Left		SBA				
Rolling Right		SBA				
Supine to Sit	CGA	SBA				
Sit to Supine	CGA	SBA				
Bed to Chair		SBA				
Sit to Stand	Min	SBA				
Toilet Transfer						

Comments for Functional Mobility

Ambulation Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance		250 ft				

Ambulation Device

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device		Rolling Walker				

Level of Assistance with Ambulation

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance		CGA				
Rolling Right						
Supine to Sit						
Sit to Supine						
Bed to Chair						
Sit to Stand						

Gait Deviations:

Comments for Ambulation

Stairs not assessed

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ On Admission ☐ Week 1 ☐ Week 2
☐ Week 3 ☐ Week 4

Number of Steps

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
# of Steps		12				

Device for Stairs

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device		Handrail Bilateral				

Level of Assistance with Stairs

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance		CGA				

Comments for Stairs

Wheelchair Mobility not assessed

☐ On Admission ☑ Week 1 ☐ Week 2
☐ Week 3 ☐ Week 4

Wheelchair Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance		N/A				

Type of Wheelchair

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

Level of Assistance with Wheelchair Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						

Comments for Wheelchair Mobility

PT Weekly Progress Team Conference

Functional Goals Focused On for Team Conference:

Outcomes/Goals:
1. STG: She will demonstrate Indep with Amb 250' on uneven terrain c FWW within 5 days free of VC and MI.
2. LTG: She will demonstrate ability to amb 350' free o AD, no VC and even and uneven surfaces in order to return home to live Indep
3. STG: She will participate in all transfers free of dizziness, nausea and room spinning in order to return home free of challenges
4. LTG: She will demonstrate all bed mobility Indep and free of dizziness in order to return home Indep

PT Weekly Progress Towards Functional Goals:

Barriers: dizziness, fall risk, decreased activity tolerance

functional mob:
bed mob: SBA
transfers: SBA_CGA
gait: CGA with FWW 250 ft
stairs: 12 CGA with BHR

DME owned: none
DME rec: FWW (ordered 3/16)

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

DC rec:

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.)for Team Conference:

Needs Continued Focus on:

endurance training
functional strengthening

Score: 0.00

Electronically Signed By:

Margaret M Demer, PT, DPT, 03/16/2026 15:05

3/16/2026 7:38 Amy Grogan, RN Note Discipline: Case Manager Tag: Interdisciplinary

Form: Interdisciplinary Plan of Care [IPOC]

Interdisciplinary Plan of Care

Anticipated Length of Stay

10-12 days

Anticipated Discharge Location

Home with Home Health

Medical Prognosis:

Medicare requires documentation beyond just a statement of "Good/Fair/Poor". Provide detailed explanation below as to "why" a patient's potential is "Good/Fair/Poor".

- ○ Good (anticipate managing medical conditions without interruption to intensive therapy plan)
- ◉ Fair (special prophylaxis required to mitigate elevated risks)
- ○ Poor (High risk of unavoidable complications during this stay)

Rehab Prognosis:

Medicare requires documentation beyond just a statement of "Good/Fair/Poor". Provide detailed explanation below as to "why" a patient's potential is "Good/Fair/Poor".

- ◉ Good (anticipate managing medical conditions without interruption to intensive therapy plan)
- ○ Fair (special prophylaxis required to mitigate elevated risks)
- ○ Poor (High risk of unavoidable complications during this stay)

Rehab/Medical Prognosis - Explain rationale for admission despite risks:

Anticipate managing medical risks

PT Intensity/Frequency

- ☐ Not required
- ☐ 1 hour/day minimum of 5 days per week
- ☑ 1.5 hours/day minimum of 5 days per week

Other intensity/frequency: _____

Total number of days anticipated: 14

OT Intensity/Frequency

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [ ] 1.5 hours/day minimum of 5 days per week

Other Intensity/Frequency: 30-180 mins/ 5-7 days/wk

Total number of days anticipated: 14

Speech Therapy Intensity/Frequency

- [ ] Not required
- [x] 1 hour/day minimum of 5 days per week
- [ ] 0.5 hours/day minimum of 5 days per week
- [ ] 0.5 hour/week, 3-5 days per week

Other Intensity/Frequency:

Total number of days anticipated: 21

Reason for Individualized Therapy Plan:

Anticipated Functional Outcomes: Self Care/Mobility

Self Care	Mobility
Independent	Modified Independent

Self Care/Mobility Comments

Additional Comments:

IPOC Agreement:

- [x] I agree with this IPOC and it supports the need for rehabilitation admission and continued stay based on the above medical issues, interventions, functional impairments, and intensity of therapies to reach the above functional outcomes within a reasonable period of time.

IPOC Signatures

Nurse who participated in the development of the Individualized Plan of Care:

foster, timika

Physical Therapist who participated in the development of the Individualized Plan of Care:

Perry, Michelle

Occupational Therapist who participated in the development of the Individualized Plan of Care:

Olson, Chelsea

Case Manager who participated in the Individualized Plan of Care:

Valenzuela, Sylvia RN CM

Speech Therapist who participated in the Individualized Plan of Care:

Cleary, Mara MA-CCC-SLP

IPOC Physician Agreement & Signature

I agree with this IPOC and it supports the need for rehabilitation admission and continued stay based on the medical issues, interventions, functional impairments, and intensity of therapies in this document, to reach the functional outcomes within a reasonable period of time.

The Interdisciplinary Plan of Care was Reviewed, Approved, and Electronically Signed by the Physician.

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Electronically Signed By:
Amy Grogan, RN, 03/16/2026 07:39
Joseph Wong, MD, 03/16/2026 18:18

3/16/2026	8:46	Kenneth Pasternak, PTA	Note Discipline: Physical Therapist Assistant	Tag: Daily Note

Form: PT Note (P33)

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 08:30 AM
Time Out: 09:30 AM
Total Time of Session: 60

Late Entry

- [ ] This is a late entry

Will this note include a Weekly Team Conference note?

( ) Yes (●) No

PT Subjective

Subjective/Observations:

pt seated in conversion chair upon arrival and agreeable to therapy

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall Dizziness Light sensitivity Cognitive communication deficits/Word retrieval deficits History of esophageal dysphagia

Vitals Taken

Did you take Vitals Pre-Treatment?

( ) Yes ( ) No

Did you take Vitals During Treatment?

( ) Yes ( ) No

Did you take Vitals Post-Treatment?

( ) Yes ( ) No

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT Objective for Daily Note

PT Mobility for Daily Note

	Assistance Level	Comment
Rolling Right		
Rolling Left		
Supine to Sit		
Sit to Supine		
Bed to Chair		
Sit to Stand		

Comments for Functional Mobility

PT Ambulation for Daily Note

	Distance	Device
Results		
Comments		
Gait Deviations		
Level of Assistance		

PT Ambulation for Daily Note (Cont.)

	Gait Deviations	Level of Assistance
Results		
Comments		
Gait Deviations		
Level of Assistance		

Gait Deviations:

Comments for Ambulation

PT Stairs for Daily Note

	Number of Steps	Device
Results		
Comments		

PT Stairs for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments for Stairs

Wheelchair Mobility for Daily Note

	Distance	Type
Results		
Comments		

Wheelchair Mobility for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Therapeutic Intervention

Therapeutic Exercise:

seated therex in multiple planes.

Therapeutic Activity:

transfer training to improve functional task performance including sts spt and toilet transfers with sba

Prosthetic/Orthotic Management

Exercise Equipment (Sci Fit / Nu Step, etc.):

Sci fit Step One at level 1 x 15 min

Neuromuscular Re-Education:

Gait:

therapist facilitated gait training to normalize gait pattern including ambulation in hallways x 250 feet three times using a fww with sba

W/C Mobility/Management Training

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Additional LE Measurements taken?

What special tests were performed for PT

- [ ] None
- [ ] 2 Minute Walk Test
- [ ] 6 Minute Walk Test
- [ ] 10 Meter Walk Test
- [ ] Timed Up and Go (TUG)
- [ ] Berg
- [ ] Tinetti Assessment
- [ ] Activities-Specific Balance Test
- [ ] Function in Sitting Test (FIST) Results
- [ ] 5 Time Sit to Stand Test
- [ ] Amputee Mobility Predictor Assessment Tool
- [ ] Functional Gait Assessment
- [ ] Lower and Thoracic Evaluation

Other:

Additional Treatment:

PT Assessment

PT Assessment

pt progressing toward functional goals. Skilled inpatient PT services remain medically necessary to facilitate improvement in patient autonomy/IND in all aspects of care, to increase IND for functional transfers and ambulation, to improve activity tolerance necessary for participation with environment, to reduce fall risk, and to decrease CG burden. Continue PT POC as outlined.

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

PT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Decreased Strength

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [x] Impaired Safety Awareness
- [ ] Impaired Communication
- [ ] Impaired Vision/Perception

DME Needed:

- [ ] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers:

PT Response to Treatment

Functional Goals Focused On:

PT Functional Progress

Needs Continued Focus On:

PT Plan of Care

PT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency:

Total number of days anticipated: 14

Patient/Family Goals:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Return to PLOF of Indep c ADLs, amb and grooming

PT Education

Education Provided On:

- [ ] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [ ] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Weight Bearing Precautions
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule

Use of Equipment: _____

Other: _____

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other: _____

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other: _____

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other: _____

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other: _____

Family/Caregiver Training

TBD

PT Objective for Team Conference

Gait Deviations:

_____

Score: 0.00

Electronically Signed By:

Kenneth Pasternak, PTA, 03/16/2026 09:32

3/16/2026	13:27	Terri Sandoval, RRT	Note Discipline: Respiratory Therapists	Tag: Admission

Form: RT Initial Pulmonary Evaluation [P81]

RT Patient History

History of Cardiopulmonary Disease:

- [ ] Asthma
- [ ] Emphysema
- [ ] Chronic bronchitis
- [ ] IPF

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Lobectomy
- [ ] Chest surgery
- [ ] TB
- [ ] COPD
- [ ] Asbestosis
- [ ] Sarcoidosis
- [ ] CF
- [ ] Lung Transplant
- [ ] Pneumonia

Other: _____

How do you feel?

- [ ] SOB at rest
- [ ] SOB with ADLs
- [ ] SOB with climbing stairs
- [ ] SOB with eating
- [ ] Wheezing
- [ ] Coughing
- [ ] Swelling
- [ ] Sleeping problem
- [ ] Chest pain
- [ ] Racing heart rate

Comments: _____

Have you been exposed to second hand smoke?

- (●) Yes
- ( ) No

Do you currently smoke?

- ( ) Yes
- (●) No

Did you smoke in the past?

- (●) Yes
- ( ) No

How long did you smoke?

- [ ] Never smoked
- [ ] 1-5 years
- [ ] 6-10 years
- [ ] 11-20 years
- [ ] 21-30 years
- [x] 31-40 years
- [ ] 41-50 years
- [ ] 51-60 years
- [ ] 60+ years

How much did you smoke per day?

- ( ) Less than half a pack
- (●) Half a pack
- ( ) 1 pack
- ( ) 2 packs
- ( ) 3 packs
- ( ) 4 packs
- ( ) 5+ packs

When did you quit smoking?

- ( ) Less than a month
- ( ) Less than 6 months
- ( ) Less than a year
- ( ) 1-5 years
- ( ) 6-10 years
- ( ) 11-20 years
- (●) 21-30 years
- ( ) 31-40 years
- ( ) 41-50 years
- ( ) 61+ years

What are you taking for a nicotine replacement?

- [ ] Gum
- [ ] Patch
- [ ] Medication
- [ ] N/A

Smoking Cessation information given?

- ( ) Yes
- ( ) No
- ( ) N/A

Have you ever particiapted in a Pulmonary Rehab?

- (●) Never participated in pulmonary rehab
- ( ) Less than a month ago
- ( ) Less than a year ago
- ( ) Less than 5 years ago
- ( ) Less than 10 years ago

Other: _____

What is you work history?

- [ ] Employed
- [ ] Plan to return to work
- [ ] Retired
- [ ] On disability

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Work environment inside
- [ ] Work environment outside

Length of Work History:

- ○ Never worked
- ○ Worked 1-5 years
- ○ Worked 6-10 years
- ○ Worked 11-20 years
- ○ Worked 21-30 years
- ○ Worked 30+ years

Environmental Exposure:

- [x] None
- [ ] Asbestosis
- [ ] Chemicals
- [ ] Dust
- [ ] Smoke
- [ ] Silica

Other: _____

What home DME do you have?

- [x] No home DME
- [ ] Oxygen Continuous
- [ ] Nocturnal Oxygen
- [ ] Nebulizer
- [ ] MDI-Spacer
- [ ] CPAP
- [ ] BIPAP
- [ ] Suction
- [ ] Aerosol Mask
- [ ] Peak Flow Meter

Other: _____

DME Provider: _____

How much and what kind of oxygen do you use?

- [x] Patient does not use Oxygen

Liters O2: _____

- [ ] Continuous
- [ ] PRN
- [ ] HS
- [ ] Humidification
- [ ] Nasal Cannula
- [ ] Aerosol Mask-Trach
- [ ] CPAP
- [ ] BIPAP
- [ ] Concentrator
- [ ] Gas
- [ ] Liquid

RT Medications: What medications do you take and how often do you use them?

_____

Have you ever had a sleep study for Sleep Apnea?

- [ ] Yes
- [x] No

If Yes, When?: _____

What interfaces do you use with your CPAP/BIPAP?

- [x] Not applicable
- [ ] Oxygen
- [ ] Cool humidifier
- [ ] Cool-Headed humidifier
- [ ] Full face mask
- [ ] Nasal mask
- [ ] Nasal pillows
- [ ] Chin strap

Other: _____

CPAP/BIPAP Pressure:

- [x] Not applicable

CPAP: _____

BIPAP: _____

RT Assessment

Breathing Pattern:

Normal Pattern: Respirations unlabored & symmetrical: regular rhythm & depth. No abnormal breath sounds. Nailbeds & mucous membranes pink.

- [x] Within Normal Limits
- [ ] Nasal flaring
- [ ] Irregular
- [ ] Dyspneic
- [ ] Tachypnea
- [ ] Labored
- [ ] SOB at rest
- [ ] SOB with exertion
- [ ] Use of accessory muscles
- [ ] Apneic

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Orthopnea
- [ ] Agonal
- [ ] Snores
- [ ] Pursed lips

Comments:

Breath Sounds

Left Upper Lobe	Right Upper Lobe	Right Middle Lobe
Clear	Clear	Clear

Breath Sounds (Cont.)

Left Lower Lobe	Right Lower Lobe
Clear	Clear

Pulse:

B/min: 95 Position: Sitting Site: Monitor Type: Regular

Comment:

Respirations:

- [ ] Spontaneous

Per Minute: 16 Vent: No

Comment:

Cough/Secretions Assessment

	Cough	Secretion Amt.	Color
Secretions	No Cough		

Cough/Secretions Assessment (Cont.)

	Consistency	Suction Frequency	Suction Route
Secretions			

Cough/Secretions Assessment (Cont.)

	Duration
Secretions	

Visual Chest Assessment

- (●) Rises Symetrically
- ( ) Asymmetrical
- ( ) Paradoxical motion

Other/Comments:

Oxygen Therapy:

- [x] None
- [ ] O2 via Nasal Cannula
- [ ] O2 via Facial Mask
- [ ] O2 via Oxymask
- [ ] O2 via Partial Rebreather
- [ ] O2 via Non Rebreather
- [ ] BiPAP
- [ ] CPAP
- [ ] Tracheostomy
- [ ] Ventilator

Delivery Rate:

- [ ] O2 is intermittent
- [ ] O2 is continuous
- [ ] O2 is high concentration

Does Patient have a Trach:

- (●) No
- ( ) Yes

RT Goals/Interventions

Goals of Treatment:

- [ ] Improve Endurance
- [ ] Prevent Infection
- [ ] Wean Off O2
- [ ] Improve Oxygenation

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- [ ] Clear Secretions
- [ ] Decannulate
- [ ] Downsize Trach

Other/Comments: _____

Interventions:

- [ ] Assess Respiratory Status
- [ ] Auscultate Breath Sounds
- [ ] Monitor ABG
- [ ] Observe S/S of Respiratory Distress
- [ ] Oral Hygiene
- [ ] Monitor O2
- [ ] Trach Care
- [ ] Administer Medications
- [ ] Monitor Effects of Medications
- [ ] Respiratory Treatment
- [ ] Education (New)

Other/Comments: _____

Planned Treatments:

- [x] No Treatment
- [ ] Aerosol/Nebulizer
- [ ] Trach Care
- [ ] Cough Assist
- [ ] MDI
- [ ] Trach Care
- [ ] IS
- [ ] Speaking Valve
- [ ] Metaneb
- [ ] Cap
- [ ] Chest PT Manual
- [ ] Chest PT Vest Percussion
- [ ] Flutter
- [ ] IPPB

Other (Specify): _____

Oxygen Therapy:

- [ ] None
- [ ] O2 via Nasal Cannula
- [ ] O2 via Facial Mask
- [ ] O2 via Oxymask
- [ ] O2 via Partial Rebreather
- [ ] O2 via Non Rebreather
- [ ] BiPAP
- [ ] CPAP
- [ ] Tracheostomy

Delivery Rate: _____

RT Problem

PT Problem: _____

RT Goals and Interventions

Goal and Intervention(s): _____

Goal date: _____

- [ ] Initiated
- [ ] Continue
- [ ] Met
- [ ] Not Met
- [ ] Discontinued
- [ ] Medical Hold

Comment: _____

RT Problem

PT Problem: _____

RT Goals and Interventions

Goal and Intervention(s): _____

Goal date: _____

- [ ] Initiated
- [ ] Continue
- [ ] Met
- [ ] Not Met
- [ ] Discontinued
- [ ] Medical Hold

Comment: _____

RT Discharge

Respiratory Barriers to Discharge:

_____

Respiratory Equipment Needs for D/C

- [ ] No Equipment
- [ ] Nasal Canula
- [ ] Peak Flow Meter
- [ ] Venti-Mask
- [ ] Non-Rebreather

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- [ ] Tracheostomy Supplies
- [ ] Nebulizer
- [ ] Suction Setup
- [ ] Ventilator
- [ ] BIPAP
- [ ] CPAP
- [ ] O2 Concentrator
- [ ] Simple Mask
- [ ] Portable Oxygen

Other:

RT Participated in Development of POC

Respiratory Therapist who participated in the Individualized Plan of Care:

Sandoval, Terri

Electronically Signed By:

Terri Sandoval, RRT, 03/16/2026 13:30

3/16/2026 15:47 Joseph Wong, MD Note Discipline: Physician Tag: Progress Note

Form: Rehab Progress Note [PPN]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

E785 - Hyperlipidemia, unspecified

I77811 - Abdominal aortic ectasia

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

Z6834 - Body mass index [BMI] 34.0-34.9, adult

Subjective

Subjective:

No acute events overnight. slept ok. denies headache, blurry vision at present. no n/v.

Denies fever/chills, chest pain, SOB, cough, nausea, vomiting, diarrhea, constipation, dysuria, new weakness or changes in sensation.
Complete ROS negative unless mentioned above.

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Physical Exam

Blood Pressure:

Systolic (mmHg): 116 Diastolic (mmHg): 60 Position: Sitting Side: Right

Comment:

Pulse:

B/min: 95 Position: Sitting Site: Monitor Type: Regular

Comment:

Respiration:

Per Minute: 16 Vent: No

Comment:

Weight:

lb: 183.0 Scale: Bed

Comment:

Height:

Feet: 5 Inches: 1

Comment:

Physical Exam

General:

- [x] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments:

Skin:

- [x] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments:

- [ ] Abnormal

Abnormal findings:

HEENT:

- [x] Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: mm dry

- [ ] Abnormal

Abnormal findings:

Neck:

- [x] Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments:

- [ ] Abnormal

Abnormal findings:

Heart:

- [x] Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Abnormal

Abnormal findings:

Lungs/Chest:

☑ Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments:

☐ Abnormal

Abnormal findings:

Abdomen:

☐ Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments:

☑ Abnormal

Abnormal findings: obese
soft ntnd

Extremities:

☑ Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments:

☐ Abnormal

Abnormal findings:

Back:

☑ Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments:

☐ Abnormal

Abnormal findings:

Cranial Nerves: ii. Optic

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

☐ Normal

☐ Abnormal

Other:

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

☐ Normal

☐ Abnormal

Other:

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

☐ Normal

☐ Abnormal

Other:

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

☐ Normal

☐ Abnormal

Other:

Cranial Nerves: vii. Auditor

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: xii. Hypoglossal

Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Gait and station are normal.

- [ ] Normal
- [x] Abnormal

Other:

fluent speech, follows commands
bilateral UE 5/5
bilateral LE 5-/5

Motor Functions: Romberg test is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Muscle tone is normal. No abnormal movements.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Plantar responses are down going.

- [ ] Normal

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- [ ] Abnormal

Other:

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Sensory: Sensory examination to pinprick and vibration is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed

Comment/Non-interfaced results:

Diagnostic Imaging Reviewed

Comment/Non-interfaced results:

Microbiology Tests Reviewed

Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Medical Conditions and Risks

Medical Conditions requiring close physician supervision:

- [x] Bowel & Bladder
- [x] Management of Pain
- [x] Medications
- [ ] Family Education/Counseling
- [x] Respiratory Complications
- [ ] Communication with Outside MDs

Other:

Risks being managed:

- [x] Infection
- [ ] Electrolyte Imbalance
- [x] Dehydration/Decrease Nutrition
- [x] DVT, PT/INR
- [x] Medication Management/Interactions

Other:

Functional Risks/Deficits

Functional Risks/Deficits needing close physician supervision:

- [x] Safety Awareness
- [x] Endurance for Activity
- [x] Transfers
- [ ] Bowel & Bladder Incontinence
- [x] ADL Performance
- [ ] Cognition/Language

Other:

Therapy and Nursing Focus

Continue PT/OT functional focus on:

- [x] Transfer Training
- [x] ADL Retraining
- [x] Ambulation Training

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] Safety Training
- [x] Balance Training
- [x] Improving ROM
- [x] Strengthening Exercises
- [x] Positioning
- [x] Fall Prevention
- [ ] Orthopedic Issues
- [x] Adaptive Equipment Training
- [ ] Caregiver Education

Other:

Continue ST functional focus on:

- [ ] NA
- [ ] Memory Strategies
- [ ] Swallowing Techniques
- [ ] Express/Comprehension
- [ ] Problem Solving
- [ ] Cognitive Retraining
- [ ] Safety Judgment
- [ ] Linguistics
- [ ] Caregiver education

Continue Rehab Nursing focus on:

- [ ] Bowel & Bladder Retraining
- [x] Pressure Relief/Skin Integrity/ Positioning
- [x] Fall Prevention
- [x] Carryover of Self-Care
- [x] Carryover of Cognitive Techniques
- [x] Med Management/Education
- [x] Carryover of Mobility Skills
- [ ] Carryover of Swallowing techniques
- [ ] Caregiver education

Other:

Barriers to Participating in Therapy

Barriers to Participating in Therapy

- [ ] None at this time
- [x] fatigue/decrease tolerance for activity
- [ ] orthostatic hypotension
- [ ] noncompliant behavior
- [x] pain
- [ ] agitation
- [ ] nausea/vomiting
- [ ] change in mental status
- [ ] altered sleep/wake cycle

Other:

Functional and Medical Progress Since Last Note:

PT Assessment

Pt tolerated session this PM well. Pt demonstrates some difficulty with balance challenges performed this date, would benefit from continued balance training in future sessions. This session concluded with pt seated in room with son present, all needs met.

OT Assessment

See Conf Grid. Pt continues to demo dizziness

SLP Assessment

Medical Progress Since Last Note:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Current Medications

Medications:

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

Medication Changes

Medications Changes Listed and Described: _____

Medical Assessment & Plan (progress & goals):

Is the patient discharging within 24 hours?

○ Yes ○ No

Medical Assessment and Plan (progress & goals):

Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist

HTN
- lisinopril

HLD
- on statin

Skin
- monitor per nursing protocol for decubitus ulcers
- frequent offloading as indicated

Bowel - constipation

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- colace/senna
- lactulose, dulcolax suppository PRN

Bladder
- monitor for retention
- check PVRs

DVT ppx
- SCDs

Functional Assessment & Plan (progress & goals):

Functional Assessment and Plan (progress & goals):

Patient is making progress towards functional goals. Continue rehab plan as above.

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

Physician Addendum

Electronically Signed By:

Joseph Wong, MD, 03/16/2026 15:48

3/16/2026 17:59 Samer Hasson, MD Note Discipline: Physician Tag: Consult Note

Form: Provider Consult Note [39]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

E785 - Hyperlipidemia, unspecified

I77811 - Abdominal aortic ectasia

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Z6834 - Body mass index [BMI] 34.0-34.9, adult

Review of History

History of Present Illness

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

PMH/SH/FH reviewed

- [ ] No change since admission
- [ ] Changes/updates documented below

Past Medical History

Add comments to any medical history that needs further details

- [x] None
- [ ] AIDS/HIV
- [ ] A-Fib
- [ ] Allergic Rhinitis
- [ ] Alzheimer's
- [ ] Anemia
- [ ] Anxiety
- [ ] Arthritis (Osteo)
- [ ] Arthritis (Rheumatoid)
- [ ] Benign Prostate Hypertrophy
- [ ] CAD
- [ ] Cancer
- [ ] CHF
- [ ] Chronic Back Pain
- [ ] Cirrhosis
- [ ] Compression Fracture
- [ ] Dementia
- [ ] DJD
- [ ] Diabetes
- [ ] Diverticulitis/losis
- [ ] Depression
- [ ] Deep Vein Thrombosis
- [ ] Epilepsy
- [ ] GERD
- [ ] Glaucoma
- [ ] GI Bleed
- [ ] Gout
- [ ] Heart Disease
- [ ] Hiatal Hernia
- [ ] HTN
- [ ] Hyperlipidemia
- [ ] Kidney/Bladder
- [ ] Liver Disease
- [ ] Lung Disease
- [ ] Macular Degeneration
- [ ] MI (Not Primary Med Condtn)
- [ ] Obesity
- [ ] Orthopedic
- [ ] Osteoporosis
- [ ] Peptic Ulcer
- [ ] Peripheral Vascular Disease
- [ ] Pulmonary Embolis
- [ ] Seizures
- [ ] Sleep Apnea
- [ ] Stroke
- [ ] TB History
- [ ] Thyroid Disease
- [ ] TIA

Other Medical History: _____

Additional Comments: _____

Past Surgical History

Add comments to any surgical history that needs further details

- [x] None
- [ ] AICD
- [ ] Amputation(s)
- [ ] Anesthesia difficulties
- [ ] Angioplasty
- [ ] Appendectomy

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Biopsy (site)
- [ ] Bronchoscopy
- [ ] CABG
- [ ] Carotid (CEA)
- [ ] Carpal Tunnel
- [ ] Cataract(s)
- [ ] Cholecystectomy
- [ ] Craniotomy
- [ ] Esophageal Surgery
- [ ] Gastric Surgeries (Type)
- [ ] Heart Valve Surgery
- [ ] Hemorrhoidectomy
- [ ] Hernia Repair (location)
- [ ] Hysterectomy
- [ ] Joint Replacement (location)
- [ ] Lithotripsy
- [ ] Mastectomy (R/L/B)
- [ ] Orthopedic
- [ ] Ostomies (Describe)
- [ ] Pacemaker
- [ ] PEG
- [ ] Pneumonectomy (lobes)
- [ ] Spinal Surgery
- [ ] Stent Placement (location)
- [ ] Tonsillectomy
- [ ] Thyroid Surgery
- [ ] Tracheostomy

Other Surgical History: _____

Additional Comments: _____

Family Medical History

Have any of these conditions occurred on either side of the patient's family? (Check if yes)

- [x] Not applicable
- [ ] Allergies
- [ ] Cardiovascular disease
- [ ] Gastrointestinal disease
- [ ] Pulmonary disease
- [ ] Bleeding tendencies
- [ ] Tuberculosis
- [ ] Psychiatric illness
- [ ] Bone disease
- [ ] Kidney disease
- [ ] Congenital deformities
- [ ] Alcohol/drug dependence
- [ ] Cancer/Tumors
- [ ] Diabetes

Other: _____

Comments: _____

Marital Status

Marital Status: Widowed

Current or past use of alcohol?

- ( ) Yes
- (•) No

Recreational/Illicit Drugs

- [x] Denies Use
- [ ] Past History of Use
- [ ] Current Use

Type: _____ Frequency: _____

Date last used: _____

Tobacco/Smoking

- [ ] Current Smoker Packs/day: _____ How many years: _____
- [x] Non-Smoker

Previous History of Smoking (indicate last time used and previous PPD): _____

- [ ] Vapes Vapes Frequency: _____
- [ ] Marijuana Marijuana Frequency: _____

Other: _____

Consult ROS

Review of System Status Update

- [ ] N/A (Admission Review of Systems)
- [x] NO CHANGES in Condition Since Last System Review.

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- [ ] CHANGES HAVE Occurred & Amended Below to Reflect an Update to the Patient's Current Condition.
- [ ] Unable to Obtain Review of Systems. Reason: _____

Review of Systems

Complications HEENT:

Check All that Apply.

- [ ] Jaundice
- [ ] Diplopia
- [ ] Photophobia
- [ ] Rhinitis
- [ ] Nose Bleeds
- [ ] Packed Left Nostril
- [ ] Packed Rt. Nostril
- [ ] Tooth ache
- [ ] Ear ache
- [ ] Sore throat
- [ ] Sinus tender
- [ ] Lymphadenopathy

Other: _____

- [ ] None of the Above

Complications of Respiratory System:

Check All that Apply.

- [ ] Intubated
- [ ] SOB at Rest
- [ ] SOB with Exertion
- [ ] Orthopnea
- [ ] Snores
- [ ] Hemoptysis
- [ ] Chills
- [ ] Night Sweats
- [ ] Fatigue
- [ ] Recent Weight Loss
- [ ] Ineffective & Non-Productive Cough
- [ ] Frequent Suctioning
- [ ] Persistent cough less than 2 weeks
- [ ] Persistent cough greater than 2 weeks
- [ ] Tracheostomy
- [ ] Sleep Apnea
- [ ] Oxygen Dependent

Other: _____

- [ ] None of the Above

Complications of Circulatory System:

Check all that Apply.

- [ ] HTN
- [ ] Hypotension
- [ ] Orthostatic Hypotension
- [ ] Tachycardia
- [ ] Bradycaria
- [ ] Dysrhythmia
- [ ] Edema
- [ ] History of Chest Pains at Rest
- [ ] History of Chest Pains with Activity

Other: _____

- [ ] None of the Above

Complications GI System:

Check all that Apply.

- [ ] Vomiting in Past 72 Hours
- [ ] Diarrhea Within past 72 Hours
- [ ] Chronic Constipation
- [ ] Weight Loss
- [ ] Weight Gain
- [ ] Indigestion
- [ ] Reflux
- [ ] Rectal Bleeding
- [ ] Tarry Stools
- [ ] Rectal Tube
- [ ] Gastric Decompression
- [ ] Jaundice
- [ ] Encephalopathy

Other: _____

- [ ] None of the Above

Complications Endocrine System:

Check all that Apply.

- [ ] Episodes of Hypoglycemia
- [ ] Episodes of Hyperglycemic
- [ ] Frequent Urination

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Excessive Thirst
- [ ] Fatigue/Malaise
- [ ] Hyperactivity

Other: _____

- [ ] None of the Above

Complications Genitourinary System:

Check all that Apply.

- [ ] Incontinent
- [ ] Neurogenic Bladder
- [ ] Anuria
- [ ] Hematuria
- [ ] Cloudy
- [ ] Odor
- [ ] Urgency
- [ ] Retention
- [ ] Difficulty starting stream
- [ ] Vaginal/Penile Bleeding or Drainage

Other: _____

- [ ] None of the Above

Complications Musculoskeletal/Mobility:

Check all that Apply.

- [ ] Fall Risk
- [ ] Activity Intolerance
- [ ] Poor Balance
- [ ] Weakness/Fatigue
- [ ] Joint Swelling
- [ ] Joint Pain
- [ ] Contractures
- [ ] Dizziness
- [ ] Amputation
- [ ] Hemiplegia
- [ ] Monoplegia
- [ ] Quadrapeligia

Other: _____

- [ ] None of the Above

Complications of Pain:

Check all that Apply.

- [ ] Chronic Pain
- [ ] Intractable Pain
- [ ] Decreased Mental Status
- [ ] Agitation
- [ ] Inability to Concentrate
- [ ] Anxiety
- [ ] Nausea/Vomiting
- [ ] Constipation
- [ ] Itching

Other: _____

- [ ] None of the Above

Pain Management:

- [ ] N/A
- [ ] Heat
- [ ] Ice
- [ ] TENS Unit
- [ ] Positioning
- [ ] Biofeedback
- [ ] Medication

Comments: _____

Complications Skin/Wound:

Check all that Apply.

- [ ] Urinary Incontinence
- [ ] Fecal Incontinence
- [ ] Wound Drainage
- [ ] Erythema
- [ ] Skin Maceration
- [ ] Skin Tear
- [ ] Incision Dehiscense
- [ ] Incision Evisceration
- [ ] Stoma Care
- [ ] Poor Nutrition
- [ ] Immobility

Other: _____

- [ ] None of the Above

Complication Neuro/Mental Status:

Check all that Apply.

- [ ] Altered Mental Status
- [ ] Dementia Confusion
- [ ] Emotional Liability/Depression
- [ ] Seizures
- [ ] Headache
- [ ] Dizziness/Balance Loss

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Speech Problem
- [ ] Loss of Gag Reflex
- [ ] Loss of Memory
- [ ] Loss of Safety Awareness
- [ ] Paralysis - Location: _____

Other: _____

- [ ] None of the Above

All Recent Investigations Noted

- ( ) All Recent Investigations Noted

Consult PE

Blood Pressure:

Systolic (mmHg): 116 Diastolic (mmHg): 60 Position: Sitting Side: Right

Comment: _____

Pulse:

B/min: 95 Position: Sitting Site: Monitor Type: Regular

Comment: _____

Respiration:

Per Minute: 16 Vent: No

Comment: _____

Weight:

lb: 183.0 Scale: Bed

Comment: _____

Height:

Feet: 5 Inches: 1

Comment: _____

Physical Exam

HEENT: Normocephalic, atraumatic, moist oral mucosa.
NECK: Supple, No JVD, No carotid bruit, No thyromegaly.
Cardiovascular system: S1 S2 normal. Regular heart rate and rhythm. No murmurs
Respiratory system: Good air entry bilaterally in chest without any crackles. No chest wall tenderness. No use of accessory muscles.
Gastrointestinal system: Abdomen full soft, moves with respiration. No organomegaly, No tenderness. Bowel sounds present.
Musculoskeletal system: No deformities. No swelling.
Psychiatry: Good affect. Oriented to place, time and person.
Skin: No lesions, warm and dry. Wounds deferred per wound team

General:

- [ ] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments: _____

Skin:

- [ ] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

HEENT:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Neck:

☐ Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Heart:

☐ Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Lungs/Chest:

☐ Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Abdomen:

☐ Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Extremities:

☐ Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Back:

☐ Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

☐ Normal

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Gait and station are normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Romberg test is normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

- [ ] Normal

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- ☐ Abnormal

Other:

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- ☐ Normal
- ☐ Abnormal

Other:

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- ☐ Normal
- ☐ Abnormal

Other:

Motor Functions: Plantar responses are down going.

- ☐ Normal
- ☐ Abnormal

Other:

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- ☐ Normal
- ☐ Abnormal

Other:

Sensory: Sensory examination to pinprick and vibration is normal.

- ☐ Normal
- ☐ Abnormal

Other:

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed

Comment/Non-interfaced results:

Diagnostic Imaging Reviewed

Comment/Non-interfaced results:

Microbiology Tests Reviewed

Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Consult Impression/Plan

Action Plan

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ New Action Plan Initiated ☐ Continue Same Course of Care ☐ Updated Action Plan Initiated

Plan:

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

#Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist
#HTN
- lisinopril
#HLD
- on statin

#DVT ppx
- SCDs

Impression

03/15/2026 seen in the common area, walking with therapy, no acute events, stable
03/16/2026: no acute issues she is resting in her room, no acute issues , no new concerns per patient or staff , vitals are stable

Neuro/Psych Assessment and Plan

Respiratory Assessment and Plan

Cardiovascular Assessment and Plan

Renal/Genitourinary Assessment and Plan

Infectious Disease Assessment and Plan

Gastrointestinal/Stress Ulcer prophylaxis/Nutrition Assessment and Plan

DVT prophylaxis Assessment and Plan

Wounds/Pressure ulcer Assessment and Plan

Medications Reviewed

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

I have Reviewed the Following Patient Data:

- ☐ Prior Medical Records & Test Results
- ☐ Medication Reconciliation
- ☐ Allergies
- ☐ Consultant Report(s)
- ☐ Current/New Medications
- ☐ Current Lab Result & Diagnostic Test
- ☐ I Have Personally Discussed Code Status with this Patient or their Surrogate Decision Maker and Believe they Understand their Medical Condition, Prognosis and the Consequences of their Code Status Decision.
- ☐ All of the Above

Anticipated Post Hospital Service Needs:

- ☐ Home with Help
- ☐ Home with Home Health
- ☐ Assisted Living Facility
- ☐ SNF
- ☐ Acute Inpatient Rehab
- ☐ Home with Outpatient Therapy
- ☐ Hospice
- ☐ Group Home
- ☐ Post Acute Vent Care Unit

Other: _____

Coordinated care with

- ☐ Consultants
- ☐ Nursing
- ☐ Nutrition
- ☐ Therapist
- ☐ Family
- ☐ Attending
- ☐ PA

Other: _____

Plan Discussed in Detail with:

- ☐ Patient
- ☐ Family
- ☐ Nursing Staff
- ☐ Case Manager

Comment: _____

Other

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

#Debility - vertigo
#HTN
#HLD
99232
Patient is Full Code

Physician Addendum

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Electronically Signed By:
Samer Hasson, MD, 03/16/2026 18:02

3/16/2026 21:50 Anthony Populo, FNP-C Note Discipline: Nurse practitioner Tag: Consult Note

Form: APP Consult Note [PCC]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

E785 - Hyperlipidemia, unspecified

I77811 - Abdominal aortic ectasia

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

Z6834 - Body mass index [BMI] 34.0-34.9, adult

Review of History

History of Present Illness

Patient is seen resting in bed at time of visit, she states she went to the hospital due to constant dizziness and headaches for weeks, she states she experienced the dizziness constantly while laying, sitting and standing, she states she had also been experiencing a headache during this time and no medication would relieve the headache, while in the hospital she states she received a migaine cocktail which did seem to help, denies experiencing ear ringing, she states she is experiencing a slight headache at time of visit but feels it is going to build to a stronger headache, she states she is also experiencing some dizziness while sitting up in bed, she states her dizziness has improved since her hospitalization. She states her dizziness is triggered when standing or trying to speak. She is AOx2, she has word finding difficulties, able to spell WORLD forwards but not backwards. Could not spell GAME backwards.

ROR showed:
""72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026."

PMH/SH/FH reviewed

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] No change since admission
- [ ] Changes/updates documented below

Past Medical History

Add comments to any medical history that needs further details

- [x] None
- [ ] AIDS/HIV
- [ ] A-Fib
- [ ] Allergic Rhinitis
- [ ] Alzheimer's
- [ ] Anemia
- [ ] Anxiety
- [ ] Arthritis (Osteo)
- [ ] Arthritis (Rheumatoid)
- [ ] Benign Prostate Hypertrophy
- [ ] CAD
- [ ] Cancer
- [ ] CHF
- [ ] Chronic Back Pain
- [ ] Cirrhosis
- [ ] Compression Fracture
- [ ] Dementia
- [ ] DJD
- [ ] Diabetes
- [ ] Diverticulitis/losis
- [ ] Depression
- [ ] Deep Vein Thrombosis
- [ ] Epilepsy
- [ ] GERD
- [ ] Glaucoma
- [ ] GI Bleed
- [ ] Gout
- [ ] Heart Disease
- [ ] Hiatal Hernia
- [ ] HTN
- [ ] Hyperlipidemia
- [ ] Kidney/Bladder
- [ ] Liver Disease
- [ ] Lung Disease
- [ ] Macular Degeneration
- [ ] MI (Not Primary Med Condtn)
- [ ] Obesity
- [ ] Orthopedic
- [ ] Osteoporosis
- [ ] Peptic Ulcer
- [ ] Peripheral Vascular Disease
- [ ] Pulmonary Embolis
- [ ] Seizures
- [ ] Sleep Apnea
- [ ] Stroke
- [ ] TB History
- [ ] Thyroid Disease
- [ ] TIA

Other Medical History: _____

Additional Comments: _____

Past Surgical History

Add comments to any surgical history that needs further details

- [x] None
- [ ] AICD
- [ ] Amputation(s)
- [ ] Anesthesia difficulties
- [ ] Angioplasty
- [ ] Appendectomy
- [ ] Biopsy (site)
- [ ] Bronchoscopy
- [ ] CABG
- [ ] Carotid (CEA)
- [ ] Carpal Tunnel
- [ ] Cataract(s)
- [ ] Cholecystectomy
- [ ] Craniotomy
- [ ] Esophageal Surgery
- [ ] Gastric Surgeries (Type)
- [ ] Heart Valve Surgery
- [ ] Hemorrhoidectomy
- [ ] Hernia Repair (location)
- [ ] Hysterectomy
- [ ] Joint Replacement (location)
- [ ] Lithotripsy
- [ ] Mastectomy (R/L/B)
- [ ] Orthopedic
- [ ] Ostomies (Describe)
- [ ] Pacemaker
- [ ] PEG
- [ ] Pneumonectomy (lobes)
- [ ] Spinal Surgery
- [ ] Stent Placement (location)

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Tonsillectomy
- [ ] Thyroid Surgery
- [ ] Tracheostomy

Other Surgical History: _____

Additional Comments: _____

Family Medical History

Have any of these conditions occurred on either side of the patient's family? (Check if yes)

- [x] Not applicable
- [ ] Allergies
- [ ] Cardiovascular disease
- [ ] Gastrointestinal disease
- [ ] Pulmonary disease
- [ ] Bleeding tendencies
- [ ] Tuberculosis
- [ ] Psychiatric illness
- [ ] Bone disease
- [ ] Kidney disease
- [ ] Congenital deformities
- [ ] Alcohol/drug dependence
- [ ] Cancer/Tumors
- [ ] Diabetes

Other: _____

Comments: _____

Marital Status

Marital Status: Widowed

Current or past use of alcohol?

- ( ) Yes
- (•) No

Recreational/Illicit Drugs

- [x] Denies Use
- [ ] Past History of Use
- [ ] Current Use

Type: _____ Frequency: _____

Date last used: _____

Tobacco/Smoking

- [ ] Current Smoker Packs/day: _____ How many years: _____
- [x] Non-Smoker

Previous History of Smoking (indicate last time used and previous PPD): _____

- [ ] Vapes Vapes Frequency: _____
- [ ] Marijuana Marijuana Frequency: _____

Other: _____

Consult ROS

Review of System Status Update

- [ ] N/A (Admission Review of Systems)
- [ ] NO CHANGES in Condition Since Last System Review.
- [ ] CHANGES HAVE Occurred & Amended Below to Reflect an Update to the Patient's Current Condition.
- [ ] Unable to Obtain Review of Systems. Reason: _____

Review of Systems

General: Appetite is satisfactory. No significant weight change.
Eyes: Pt denies vision changes, pain, discharge or blurry vision.
Ears/Nose/Mouth/Throat: Pt denies ear pain, change in hearing. Pt denies nasal congestion/discharge or sinus pain. Pt denies sore throat, difficulty swallowing or new dental problems.
Respiratory: Pt denies cough, dyspnea, wheezing or chest pain
Cardiovascular: Pt denies chest pain or tightness, palpitations or edema.
Gastrointestinal: Pt denies abdominal pain, N/V/D, constipation

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Musculoskeletal: Pt denies new joint pain or stiffness, redness or swelling.
Neurological: dizziness (+)
No loss of consciousness, transient ischemic symptoms or seizures

Complications HEENT:

Check All that Apply.

- [ ] Jaundice
- [ ] Diplopia
- [ ] Photophobia
- [ ] Rhinitis
- [ ] Nose Bleeds
- [ ] Packed Left Nostril
- [ ] Packed Rt. Nostril
- [ ] Tooth ache
- [ ] Ear ache
- [ ] Sore throat
- [ ] Sinus tender
- [ ] Lymphadenopathy

Other: _____

- [ ] None of the Above

Complications of Respiratory System:

Check All that Apply.

- [ ] Intubated
- [ ] SOB at Rest
- [ ] SOB with Exertion
- [ ] Orthopnea
- [ ] Snores
- [ ] Hemoptysis
- [ ] Chills
- [ ] Night Sweats
- [ ] Fatigue
- [ ] Recent Weight Loss
- [ ] Ineffective & Non-Productive Cough
- [ ] Frequent Suctioning
- [ ] Persistent cough less than 2 weeks
- [ ] Persistent cough greater than 2 weeks
- [ ] Tracheostomy
- [ ] Sleep Apnea
- [ ] Oxygen Dependent

Other: _____

- [ ] None of the Above

Complications of Circulatory System:

Check all that Apply.

- [ ] HTN
- [ ] Hypotension
- [ ] Orthostatic Hypotension
- [ ] Tachycardia
- [ ] Bradycaria
- [ ] Dysrhythmia
- [ ] Edema
- [ ] History of Chest Pains at Rest
- [ ] History of Chest Pains with Activity

Other: _____

- [ ] None of the Above

Complications GI System:

Check all that Apply.

- [ ] Vomiting in Past 72 Hours
- [ ] Diarrhea Within past 72 Hours
- [ ] Chronic Constipation
- [ ] Weight Loss
- [ ] Weight Gain
- [ ] Indigestion
- [ ] Reflux
- [ ] Rectal Bleeding
- [ ] Tarry Stools
- [ ] Rectal Tube
- [ ] Gastric Decompression
- [ ] Jaundice
- [ ] Encephalopathy

Other: _____

- [ ] None of the Above

Complications Endocrine System:

Check all that Apply.

- [ ] Episodes of Hypoglycemia
- [ ] Episodes of Hyperglycemic
- [ ] Frequent Urination
- [ ] Excessive Thirst
- [ ] Fatigue/Malaise
- [ ] Hyperactivity

Other: _____

- [ ] None of the Above

Complications Genitourinary System:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Check all that Apply.

- [ ] Incontinent
- [ ] Neurogenic Bladder
- [ ] Anuria
- [ ] Hematuria
- [ ] Cloudy
- [ ] Odor
- [ ] Urgency
- [ ] Retention
- [ ] Difficulty starting stream
- [ ] Vaginal/Penile Bleeding or Drainage

Other: _____

- [ ] None of the Above

Complications Musculoskeletal/Mobility:

Check all that Apply.

- [ ] Fall Risk
- [ ] Activity Intolerance
- [ ] Poor Balance
- [ ] Weakness/Fatigue
- [ ] Joint Swelling
- [ ] Joint Pain
- [ ] Contractures
- [ ] Dizziness
- [ ] Amputation
- [ ] Hemiplegia
- [ ] Monoplegia
- [ ] Quadrapeligia

Other: _____

- [ ] None of the Above

Complications of Pain:

Check all that Apply.

- [ ] Chronic Pain
- [ ] Intractable Pain
- [ ] Decreased Mental Status
- [ ] Agitation
- [ ] Inability to Concentrate
- [ ] Anxiety
- [ ] Nausea/Vomiting
- [ ] Constipation
- [ ] Itching

Other: _____

- [ ] None of the Above

Pain Management:

- [ ] N/A
- [ ] Heat
- [ ] Ice
- [ ] TENS Unit
- [ ] Positioning
- [ ] Biofeedback
- [ ] Medication

Comments: _____

Complications Skin/Wound:

Check all that Apply.

- [ ] Urinary Incontinence
- [ ] Fecal Incontinence
- [ ] Wound Drainage
- [ ] Erythema
- [ ] Skin Maceration
- [ ] Skin Tear
- [ ] Incision Dehiscense
- [ ] Incision Evisceration
- [ ] Stoma Care
- [ ] Poor Nutrition
- [ ] Immobility

Other: _____

- [ ] None of the Above

Complication Neuro/Mental Status:

Check all that Apply.

- [ ] Altered Mental Status
- [ ] Dementia Confusion
- [ ] Emotional Liability/Depression
- [ ] Seizures
- [ ] Headache
- [ ] Dizziness/Balance Loss
- [ ] Speech Problem
- [ ] Loss of Gag Reflex
- [ ] Loss of Memory
- [ ] Loss of Safety Awareness
- [ ] Paralysis - Location: _____

Other: _____

- [ ] None of the Above

All Recent Investigations Noted

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

○ All Recent Investigations Noted

Consult PE

Blood Pressure:

Systolic (mmHg): 116 Diastolic (mmHg): 60 Position: Lying Side: Left

Comment:

Pulse:

B/min: 95 Position: Lying Site: Monitor Type: Regular

Comment:

Respiration:

Per Minute: 18 Vent: No

Comment:

Weight:

lb: 183.0 Scale: Bed

Comment:

Height:

Feet: 5 Inches: 1

Comment:

Physical Exam

General: This patient is well developed and in no acute distress.
Neck: Supple without thyromegaly, lymphadenopathy, or masses.
Respiratory: Normal respiratory effort.
Cardiovascular: Regular rhythm; no murmurs, rubs, or gallops.
Chest: Chest normal to inspection.
Gastrointestinal: Abdomen: Soft, nontender, and without masses. Bowel sounds active.
Musculoskeletal: Digits/nails: No clubbing, cyanosis, inflammation or ischemia.
Neurological:
MENTAL STATUS: The patient is Awake, alert and oriented x 2. Speech is normal. Attention span intact and can follow 3 step commands.

CRANIAL NERVES:
CN 2: intact.
CN3, 4, 6: Pupils are equal and round, 3mm. Reactive to direct and consensual, EOMI - conjugate. No INO, APD. No Horners
CN 5: normal facial sensation
CN 7: Symmetric facial movement
CN 8: Hearing intact
CN 9, 10: Symmetric palate movement and normal gag reflex
CN 11: Shoulder shrug strength normal
CN 12: Tongue is midline.

MOTOR:
RUE 5/5 proximal 5/5 distal
LUE: 5/5 proximal, 5/5 distal
RLE 5/5 proximal, 5/5 distal
LLE 5/5 proximal, 5/5 distal.

MOVEMENT: no tremor

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

ABNORMAL MOVEMENTS: No dyskinesias

DTR'S: R/L
Biceps 1+/1+
Triceps 1+/1+
Knees 1+/1+
Ankles 0/0

PLANTAR RESPONSE: B/L downgoing

COORDINATION: No ataxia / dysmetria

HINTS exam (-)

General:

- [ ] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments: _____

Skin:

- [ ] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

HEENT:

- [ ] Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Neck:

- [ ] Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Heart:

- [ ] Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Lungs/Chest:

- [ ] Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Abdomen:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Extremities:

☐ Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Back:

☐ Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: Vii. Facial

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: vii. Auditor

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: xi. Accessory

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Gait and station are normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Romberg test is normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Plantar responses are down going.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Sensory: Sensory examination to pinprick and vibration is normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed _____

Comment/Non-interfaced results: _____

Diagnostic Imaging Reviewed

_____

Comment/Non-interfaced results: _____

Microbiology Tests Reviewed

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Consult Impression/Plan

Action Plan

☐ New Action Plan Initiated ☐ Continue Same Course of Care ☐ Updated Action Plan Initiated

Plan:

#spontaneous vestibular syndrome
- differential dx include: vestibular neuritis vs. vestibular migraine vs. functional
- HINTS (-)
- consider starting triptan if headache worsens
- appreciate PT recommendations on Dix-Hallpike and Epley maneuvers.

Impression

Neuro/Psych Assessment and Plan

Respiratory Assessment and Plan

Cardiovascular Assessment and Plan

Renal/Genitourinary Assessment and Plan

Infectious Disease Assessment and Plan

Gastrointestinal/Stress Ulcer prophylaxis/Nutrition Assessment and Plan

DVT prophylaxis Assessment and Plan

Wounds/Pressure ulcer Assessment and Plan

Medications Reviewed

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

I have Reviewed the Following Patient Data:

- [ ] Prior Medical Records & Test Results
- [ ] Medication Reconciliation
- [ ] Allergies
- [ ] Consultant Report(s)
- [ ] Current/New Medications
- [ ] Current Lab Result & Diagnostic Test
- [ ] I Have Personally Discussed Code Status with this Patient or their Surrogate Decision Maker and Believe they Understand their Medical Condition, Prognosis and the Consequences of their Code Status Decision.
- [x] All of the Above

Anticipated Post Hospital Service Needs:

- [ ] Home with Help
- [ ] Home with Home Health
- [ ] Assisted Living Facility
- [ ] SNF
- [ ] Acute Inpatient Rehab
- [ ] Home with Outpatient Therapy
- [ ] Hospice
- [ ] Group Home
- [ ] Post Acute Vent Care Unit

Other: _____

Coordinated care with

- [ ] Consultants
- [ ] Nursing
- [ ] Nutrition
- [ ] Therapist
- [ ] Family
- [ ] Attending
- [ ] PA

Other: _____

Plan Discussed in Detail with:

- [ ] Patient
- [ ] Family
- [ ] Nursing Staff
- [ ] Case Manager

Comment: _____

Other

Approximately 19 minutes were spent discussing Advanced Care Planning with family/POA/patient/Staff including the explanation and discussion of advanced directives and their voluntary nature due to their stay at the current facility and their medical condition. As well as chart review.

75 minutes were spent reviewing history, labs, imaging and plan of care. This includes direct bedside attendance, interpretation of investigative studies, review of prior records, involvement of consultants.

This encounter was coordinated with supervising physician Dr. Eric Taylor. Half of the time (at least) was spent collaborating with supervising physician, Dr. Taylor.

Thank you for allowing me to participate in the care of this patient. Feel free to call me with any questions.

Anthony Populo, DNP, AGACNP - BC

Electronically Signed By:

Anthony Populo, FNP-C, 03/17/2026 17:06

3/17/2026	6:58	Sylvia Valenzuela, RN	Note Discipline: Case Manager	Tag: Interdisciplinary

Form: Interdisciplinary Team Conference [E57]

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Attendees

Case Manager
(First Name Last Name, Credentials): Sylvia Valenzuela, RN CM

Nurse
(First Name Last Name, Credentials): Jacor Calvin, RN

Occupational Therapist
(First Name Last Name, Credentials): Nancy Hansmeier, OT

Physical Therapist
(First Name Last Name, Credentials): Margaret Demer, PT

Speech Language Pathologist
(First Name Last Name, Credentials): Mara Cleary, SLP

Respiratory Therapist
(First Name Last Name, Credentials): Terri Sandoval, RT

Psychologist
(First Name Last Name, Credentials):

Physician
(First Name Last Name, Credentials): Dr. Joseph Wong

Social Worker
(First Name Last Name, Credentials):

Dietitian
(First Name Last Name, Credentials): Katrina Moore, RD

Pharmacist
(First Name Last Name, Credentials): Diana Ganem , PharmD

Patient
(First Name Last Name):

Family
(First Name Last Name):

Family
(First Name Last Name):

Additional Attendee
(First Name Last Name): Amy Grogan, DPO

Additional Attendee
(First Name Last Name):

Additional Attendee
(First Name Last Name):

Additional Attendee
(First Name Last Name):

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Nursing

Bladder Continence

Bowel Continence

Elimination device

- ☐ N/A
- ☐ Foley
- ☐ External/condom catheter
- ☐ Suprapubic
- ☐ Intermittent cath
- ☐ Self cath
- ☐ Urostomy

Other: _____

Indwelling Catheter Necessity:

- ☐ Known or suspected urinary tract obstruction
- ☐ Neurogenic bladder dysfunction or urinary retention
- ☐ Recent urologic surgery, bladder injury, pelvic surgery, or recent surgery involving structures continguous with the bladder or urinary tract, after pelvic surgery
- ☐ Other surgical patient postoperative day 0 or 1
- ☐ Urinary incontinence in a patient with Stage 3 or Stage 4 pressure injury on the trunk, perineal wounds, necrotizing infections
- ☐ Need for accurate measurement of urinary output
- ☐ Gross hematuria in patients with potential clots
- ☐ Epidural catheter still in place
- ☐ Pallative care for terminally ill
- ☐ Immobility (complete bedrest) with inability to empty bladder without mechanical means
- ☐ Patient with indwelling urinary catheter in place on admission
- ☐ None apply - request order for catheter removal

Gastric Tube

	Type	Suction
Gastric Tube		

Gastric Tube (Cont.)

	Drainage
Gastric Tube	

Pain Goal

Pain Goal: 0

☐ Non-Verbal patient unable to state goal

Comment: _____

Has the patient had any falls during stay?

◉ None

Yes, Number of Falls: _____

Did the patient have a Blood Transfusion?

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Yes
- [x] No

If Yes, Comment:

DVT Mechanical Prophylaxis Orders:

- [ ] DVT Mechanical Prophylaxis Not Indicated

Was Diabetic Management Education provided to patient?

- [x] Diabetic Management Education Not Indicated

Nursing Plan of Care STG & LTG:

Problem: Priscila is at risk for falls
related to Vertigo/dizziness, Unsteady gait

Problem: Alteration in Communication
related to word finding difficulty

Problem: Alteration in ADL

Problem: Mobility Impairment

Problem: Alteration in Cognition/communication
related to Vertigo as evidenced by Vertigo

Problem: Alteration in Swallowing function related to Dysphagia/GERD as evidenced by dysphagia

Nursing Weekly Progress Towards Goals:

Nursing Barriers to Discharge:

- [ ] None
- [x] Medical condition
- [x] Decreased mobility
- [ ] Functional status
- [ ] Social/cultural barriers
- [ ] Poor activity tolerance
- [ ] Architectural barriers in home
- [ ] Limited family/caregiver support
- [ ] Skin/Wounds
- [ ] Poor pain tolerance
- [ ] History of falls
- [ ] Bowel/Bladder

Other:

Nursing Equipment Needs on Discharge:

PT Objective for Team Conference

Functional Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Rolling Left		SBA				
Rolling Right		SBA				
Supine to Sit	CGA	SBA				
Sit to Supine	CGA	SBA				

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Bed to Chair		SBA				
Sit to Stand	Min	SBA				
Toilet Transfer						

Comments for Functional Mobility

No bed rolling assessed since upon subjective report, this writer is confident Pt presents with vertigo. LOB recovered upon standing

Ambulation Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance		250 ft				

Ambulation Device

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device		Rolling Walker				

Level of Assistance with Ambulation

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance		CGA				
Rolling Right						
Supine to Sit						
Sit to Supine						
Bed to Chair						
Sit to Stand						

Gait Deviations:

Comments for Ambulation

Stairs not assessed

- [ ] On Admission
- [ ] Week 1
- [ ] Week 2
- [ ] Week 3
- [ ] Week 4

Number of Steps

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
# of Steps		12				

Device for Stairs

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device		Handrail Bilateral				

Level of Assistance with Stairs

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance		CGA				

Comments for Stairs

Wheelchair Mobility not assessed

- [ ] On Admission
- [x] Week 1
- [ ] Week 2
- [ ] Week 3
- [ ] Week 4

Wheelchair Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance		N/A				

Type of Wheelchair

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Level of Assistance with Wheelchair Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						

Comments for Wheelchair Mobility

PT Plan of Care

PT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency:

Total number of days anticipated: 14

Patient/Family Goals:

Return to PLOF of Indep c ADLs, amb and grooming

Modified Rankin Score

Is the patient in the Rehab Stroke Program?

If yes, the Modified Rankin Scale (mRS) assesses the patient's level of disability or dependence in daily activities following a stroke or other neurological condition. Inform the patient (or caregiver, if the patient is unable to respond) that you will be asking questions about their usual daily functioning since the stroke or illness.

- ○ Yes
- ◉ No

Modified Rankin Score Total Score: .00

PT Weekly Progress Team Conference

Functional Goals Focused On for Team Conference:

Outcomes/Goals:
1. STG: She will demonstrate Indep with Amb 250' on uneven terrain c FWW within 5 days free of VC and MI.
2. LTG: She will demonstrate ability to amb 350' free o AD, no VC and even and uneven surfaces in order to return home to live Indep
3. STG: She will participate in all transfers free of dizziness, nausea and room spinning in order to return home free of challenges
4. LTG: She will demonstrate all bed mobility Indep and free of dizziness in order to return home Indep

PT Weekly Progress Towards Functional Goals:

Barriers: dizziness, fall risk, decreased activity tolerance
functional mob:
bed mob: SBA
transfers: SBA_CGA
gait: CGA with FWW 250 ft
stairs: 12 CGA with BHR
DME owned: none
DME rec: FWW (ordered 3/16)
DC rec:

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.)for Team Conference:

Needs Continued Focus on:

endurance training
functional strengthening

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

OT Objective for Team Conference

Activities of Daily Living

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Self Feeding	SBA	SBA				
Oral Hygiene	Min	Setup				
Grooming	CGA	Setup				
Toilet Hygiene	CGA	CGA				
Bathing/Shower	CGA	CGA				
Dressing UE	SBA	SBA				
Dressing LE	CGA	SBA				
Putting On/Taking Off Footwear	SBA	SBA				

Comments for ADLs for Weekly Team Conference

Current Functional Status:
Feeding- SBA
G/H-SBA/min
Toileting-CGA
Bathing-CGA
UBD -CGA
LBD-SBA
Footwear-SBA
Toilet Transfer-SBA
Shower/tub transfer -SBA
Goals met: 0/3 STG, 0/3 LTG

OT Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Bed		SBA				
Toileting		SBA				
Tub						
Shower						

Comments for OT Mobility for Team Conference

Wheelchair Mobility not assessed

- ☐ On Admission
- ☐ Week 1
- ☐ Week 2
- ☐ Week 3
- ☐ Week 4

Wheelchair Distance

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Distance						

Type of Wheelchair

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Device						

Level of Assistance with Wheelchair Mobility

	Admission	Week 1	Week 2	Week 3	Week 4	Week 5
Level of Assistance						

Comments for Wheelchair Mobility

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

OT Plan of Care

OT Intensity/Frequency

- ☐ Not required
- ☐ 1 hour/day minimum of 5 days per week
- ☐ 1.5 hours/day minimum of 5 days per week

Other Intensity/Frequency: 30-180 mins/ 5-7 days/wk

Total number of days anticipated: 14

Patient/Family Goals:

Not really I think I'm doing okay, I can go home and do whatever I need to do

OT Weekly Progress Team Conference

Functional Goals Focused On for Team Conference:

Outcomes/Goals:
1. STG: Patient will complete standing grooming tasks at sink for 5 min with SBA and no LOB to improve endurance for ADL participation
2. LTG: Patient will complete grooming standing at the sink for greater than 5 minutes with no LOB, demonstrating improved activity tolerance for self care.
3. STG: Patient will complete functional reaching outside BOS during standing ADL task with CGA and no LOB to improve dynamic balance for daily activities.
4. LTG: Patient will perform functional mobility during ADL tasks using AD safely and with no LOB to improve safety awareness and independence with ADLs.
5. STG: Patient will complete LB dressing in seated position with SBA and minimal vc for safety awareness to reduce fall risk during ADLs.
6. LTG: Patient will complete UB/LB dressing independently in seated position without LOB or unsafe standing to maximize independence and reduce fall risk.

OT Weekly Progress Towards Functional Goals:

Current Functional Status:
Feeding- SBA
G/H-SBA/min
Toileting-CGA
Bathing-CGA
UBD -CGA
LBD-SBA
Footwear-SBA
Toilet Transfer-SBA
Shower/tub transfer -SBA
Goals met: 0/3 STG, 0/3 LTG

Endurance for Team Conference (eg Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Needs Continued Focus on:

Continue POC

SLP Plan of Care

Speech Therapy Intensity/Frequency

- ☐ Not required

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] 1 hour/day minimum of 5 days per week
- [ ] 0.5 hours/day minimum of 5 days per week
- [ ] 0.5 hour/week, 3-5 days per week

Other Intensity/Frequency: _____

Total number of days anticipated: 21

Patient/Family Goals

Improve my memory and speech

SLP Weekly Progress Team Conference

SLP Objective for Team Conference

	Admission	Week 1
Attention	Mod	
Cognitive Deficit	Mod	
Communication Deficit	Mod	
Comprehension Deficit	Ind	
Swallowing Impairment	Ind	

SLP Objective for Team Conference (Cont.)

	Week 2	Week 3
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

SLP Objective for Team Conference (Cont.)

	Week 4	Goal
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

Comments for SLP Objective

Functional Goals Focused on:

SLP Weekly Progress Towards Functional Goals:

Patient's Response and/or Demonstration for Team Conference:

Dysphagia

Was Dysphagia Addressed?

- ○ Yes
- ○ No

Oral Motor Exercises:

- [ ] Not Done

Swallowing Exercises:

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Not Done

Neck Isometric Exercises:
☐ Not Done

Oral Motor Stimulation:
☐ Not Done

Safe Swallowing Technique:
☐ Not Done

Compensatory Strategies:
☐ Not Done

Therapeutic Trials:
☐ Not Done

NMES:
☐ Not Done

Other:

Speech Production

Was Speech Production Addressed?
○ Yes ○ No

Length of Utterance:
☐ Not Done

Volume:
☐ Not Done

Initiation:
☐ Not Done

Rate:
☐ Not Done

Other:

Cognition / Language

Was Cognition/Language Addressed?
○ Yes ○ No

Cognition:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Orientation
- [ ] Memory
- [ ] Attention
- [ ] Sequencing
- [ ] Problem Solving
- [ ] Deductive Reasoning
- [ ] Visual Scanning
- [ ] Flexibility
- [ ] Insight/Awareness

Expressive Language:

- [ ] Verbal/Nonverbal
- [ ] Automatic Speech
- [ ] Imitation
- [ ] Naming
- [ ] Sentence Completion
- [ ] Sentence Formulation
- [ ] Picture Description
- [ ] Word Retrival
- [ ] Narrative Conversation
- [ ] Alernative Communication

Receptive Language:

- [ ] Y/N Questions: Simple/Complex/Egocentric
- [ ] Where Questions: Simple/Complex/Egocentric
- [ ] When Questions: Simple/Complex/Egocentric
- [ ] Lt/Rt Discrimination
- [ ] Conversational Exchange
- [ ] Content
- [ ] Picture ID Field: _____
- [ ] Follows Directions ___ Steps: _____

Other:

Respiratory Therapy

Is RT on Service?

- ○ Yes
- ○ No

Wounds

Skin Breakdown

Skin integrity issues will need to be documented in the Wound Care module.

- [ ] Pressure injuries present
- [ ] Skin tears present
- [ ] Bruises present
- [ ] Cuts, scrapes, bruises present

Comments: _____

Risk Level for Skin Breakdown:

Braden Score: _____

- [ ] Severe risk (less than/= 9)
- [ ] High Risk (10-12)
- [ ] Moderate Risk (13-14)
- [ ] Mild Risk (15-18)
- [ ] No Risk (>/= 19)

Interventions for Skin Health

- [ ] Positioning device/wedge/pillows to keep pressure off boney prominences
- [ ] Toileting q 2 hours
- [x] OOB to chair
- [x] Assist with ambulation
- [ ] Elevate HOB no more than 30 degrees (except during and for 30 minutes after eating)
- [ ] Avoid positioning on greater trochanter
- [ ] Provide supplements for protein/ calories
- [x] Elevate legs in chair
- [ ] Use lift sheet to move patient

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Apply moisture barrier/skin protectant/protective incontinence care
- [x] Pillow under calves to elevate heels
- [ ] Heel protection boots
- [ ] Trapeze

Specialty bed: _____

Other: _____

Dietary

Dietary Orders:

Diet Order Every Shift, Request Type: Routine, Diet Consistency: Regular, Liquid Consistency: Thin (0), Therapeutic Diets: General Healthful, Comments: GERD precautions
History of dilatation
Cut meds in half as needed
One at a time
Intermittent supervision
NOTHING TOO HARD OR DRY

Nutritional Risk Level

- ○ High
- ○ Moderate
- ○ Low

Malnutrition Risk

- ◉ None
- ○ Non-Severe Malnutrition
- ○ Severe Malnutrition

Goals:

- [ ] PO Intake > 50% of Needs
- [ ] PO Intake > 75% of Needs
- [ ] Tolerate TF @ Goal
- [ ] TPN Tolerance
- [ ] Maintain Weight without Significant Change
- [ ] Gradual Weight Loss
- [ ] Gradual Weight Gain
- [ ] Glycemic Control
- [ ] < 4 Loose BMs Daily
- [ ] Wound Healing
- [ ] Demonstrated Understanding of Education Provided

Other: _____

Monitor:

- [ ] Education Needs
- [ ] PO Intake
- [ ] Monitor Labs
- [ ] Monitor Tube Feeding Rate/ Tolerance
- [ ] Monitor Weight
- [ ] Monitor BM
- [ ] Monitor Skin Integrity
- [ ] Monitor Respiratory Status

Other: _____

Comments:

No risk. dx: vertigo

Education Provided On:

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Nutrition ☐ Plan of Care ☐ Therapeutic Diet Interventions ☐ Diet Medication Interaction

Other:
- meal schedule
- selective menus
- nourishments available
- hydration

Response to Education:

☑ Voiced Understanding ☐ Demonstrated Understanding ☐ Learner Requires More Education

Other:

Identified Problems/Deficits:

☑ Knowledge Deficit ☐ Safety Awareness ☐ Self Feeding ☐ Cognitive Deficit

Other:

Dietary Additional Comments:

Pharmacy

Medications:

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

Antibiotics/Diagnosis for Antibiotics:

☐ N/A

Anticoagulation:

☐ N/A

Neuro/Psych:

☐ N/A

Cardiovascular:

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☐ N/A

Pain Control:
☐ N/A

Electrolyte/Nutrition Replacement:
☐ N/A

Other (List Significant Medication and Reason for Taking):
☐ N/A

Pharmacy Interventions:
☐ N/A

Case Management

Anticipated Discharge Date:
3/23/2026

Patient/Caregiver Anticipated Discharge Plans:
Patient will discharge home with home health

Alternate Discharge Plan(s) per Patient/Caregiver:
family

Have Patient/Caregiver Training & Education needs been addressed?
☑ Yes ☐ No ☐ N/A

Comments:

Case Management Discharge Barriers Identified:

☐ N/A	☐ Safety	☐ Social	☐ Medical
☐ Functional	☐ Financial		
☐ Lack of Understanding	☐ Lack of Family Support	☐ Cognitive Defecits	☐ Communication Defecits
☐ Behavioral Issues	☐ Emotional Issues		
☑ Decreased Strength	☑ Decreased ROM	☐ Environmental	☐ Architechtural
☐ Transportation			

Current Needs:
This include all problems that are impeding progress towards treatment goals.

☑ Mobility	☑ Self-Care	☐ Cognition	☐ Communication
☐ Discharge Planning	☑ Daily Life Skills	☐ Sphincter Control	☐ Endocrine
☐ Integumentary	☐ HEENT	☐ Risk of Infection	☐ Respiratory
☐ Nutrition	☐ Pain	☐ Safety	☐ Psychosocial
☐ Swallow	☐ Elopement Risk		

Other:

Comorbidities

☐ Cellulitis	☐ C. Difficile	☐ Pseudomonas	☐ UTI

Other Infection:

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- [ ] Diabetic Neuropathy
- [ ] Diabetic Myopathy
- [ ] Acute Kidney Failure
- [ ] Chronic Kidney Failure
- [ ] Aspiration Pneumonia
- [ ] Pneumonitis
- [ ] Pneumonia
- [ ] BMI >40
- [ ] Dysphagia
- [ ] Hemiparesis
- [ ] Diastolic Heart Failure
- [ ] Systolic Heart Failure

Other: Vertigo

DME Needed:

- [x] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers:

Areas of Progress from Prior Week

- [ ] Continence
- [ ] Nutrition
- [ ] Hydration
- [ ] Pain Management
- [ ] Sleep
- [x] Skin Integrity
- [ ] Medication Management
- [ ] Disease Management
- [ ] Strength Training
- [ ] Balance Training
- [ ] Sensation
- [ ] Speech
- [ ] Activity Tolerance
- [ ] Skill Acquisition
- [ ] Coordination
- [ ] Cognition
- [ ] Language
- [ ] Swallowing

Other:

Facilitating Factors Allowing Patient to Progress

- [x] Patient Compliance
- [ ] Level of Understanding
- [ ] Motivation
- [ ] Family/Social Support
- [ ] Psychological Stability
- [ ] Pain Management
- [x] Improved Motor Function
- [ ] Improved Sensory Function
- [ ] Proficiency with Equipment
- [ ] Improved Endurance
- [ ] Use of Compensatory Strategies

Other:

Utilization Management

- [x] Patient meets continued stay criteria
- [ ] Patient has met all goals and is safe to be discharged — Date: 3/23/2026
- [ ] Updates/changes to the plan of care have been reviewed with the patient/and or patient representative?

Case Management Additional Comments:

Initial IDT meeting: 03/17/26
Patient updated on progress and recommendation for discharge.
Nursing reports : Aspiration Precautions , Aspirin, Fall Precautions, Neurology Consult,

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT reports: Barriers: dizziness, fall risk, decreased activity tolerance
functional mob:
bed mob: SBA
transfers: SBA_CGA
gait: CGA with FWW 250 ft
stairs: 12 CGA with BHR
DME owned: none
DME rec: FWW (ordered 3/16)
DC rec:
RT reports:
Physician reports patient has: Vertigo
Discharge plan is home with HHC and Yes family support

Patient is participating in a specialty program:

- [ ] Stroke Program
- [ ] Traumatic Brain Injury (TBI) Program
- [ ] Spinal Cord Injury (SCI) Program
- [ ] Amputee Program
- [x] None

MD

I have led and participated in the Interdisciplinary Team Conference and I concur with the established team goals and plan.

Medical Prognosis:

Medicare requires documentation beyond just a statement of "Good/Fair/Poor".
Provide detailed explanation below as to "why" a patient's potential is "Good/Fair/Poor".

- ( ) Good (anticipate managing medical conditions without interruption to intensive therapy plan)
- (•) Fair (special prophylaxis required to mitigate elevated risks)
- ( ) Poor (High risk of unavoidable complications during this stay)

Rehab Prognosis:

Medicare requires documentation beyond just a statement of "Good/Fair/Poor".
Provide detailed explanation below as to "why" a patient's potential is "Good/Fair/Poor".

- (•) Good (anticipate managing medical conditions without interruption to intensive therapy plan)
- ( ) Fair (special prophylaxis required to mitigate elevated risks)
- ( ) Poor (High risk of unavoidable complications during this stay)

Comments on Changes:

Electronically Signed By:

Sylvia Valenzuela, RN, 03/17/2026 18:31
Amy Grogan, RN, 03/17/2026 08:33
Joseph Wong, MD, 03/18/2026 18:17

3/17/2026	6:59	Sylvia Valenzuela, RN	Note Discipline: Case Manager	Tag: Progress Note

Form: Case Management Note [CM2]

Case Management Note

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

CM Spoke with:

- [x] Patient
- [ ] Family
- [ ] Caregiver
- [ ] Significant Other/Spouse
- [ ] Domestic Partner
- [x] Other:

Do you need or want an interpreter to communicate with a doctor or healthcare staff?

- ( ) Yes
- (•) No
- ( ) Unable to determine

What form of Interpreter was utilized?

- [ ] By Phone (list Interpreter ID number in comments)
- [ ] By Certified Interpreter (list name or Interpreter ID number in comments)
- [ ] Patient agreed to use family member interpreter (non-medical terminology used)
- [ ] Patient agreed to use bi-lingual staff interpreter (non-medical terminology used)
- [ ] Other (see comments)

Comments:

Case Management Discharge Barriers Identified:

- [ ] N/A
- [ ] Safety
- [ ] Social
- [ ] Medical
- [ ] Functional
- [ ] Financial
- [ ] Lack of Understanding
- [ ] Lack of Family Support
- [ ] Cognitive Defecits
- [ ] Communication Defecits
- [ ] Behavioral Issues
- [ ] Emotional Issues
- [x] Decreased Strength
- [x] Decreased ROM
- [ ] Environmental
- [ ] Architechtural
- [ ] Transportation

Established Providers:

Include Contact Information

Primary Care: Dr Stanley Coombs 602 864 8800
Specialist(s):
Clinic:
Pharmacy: CVS 602) 275-0120
Other:

POC Review:

- (•) Yes
- ( ) No
- ( ) N/A

Patient/Caregiver Anticipated Discharge Plans:

Patient will discharge home with home health

Alternate Discharge Plan(s) per Patient/Caregiver:

family

Provided Medicare.gov lists with data directly from website based on Discharge Plan:

- [x] Home Health Agency
- [ ] Skilled Nursing Facility
- [ ] Long-Term Acute Care Hospital
- [ ] Inpatient Rehabilitation Facility
- [ ] Patient Refused List
- [ ] NA

Discharge Disposition:

06. Home under care of organized home health services organization

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Currently Scheduled Appointments(s):

Specialty	Name	Date	Time	Address

Currently Scheduled Appointments(s): (Cont.)

Phone	Comments

Currently Scheduled Appointment(s) Comments:

Follow up appointments: ValleyWise 602 344 1300 at 2601 E. Roosevelt Street Phx

Have Patient/Caregiver Training & Education needs been addressed?

- [ ] Yes
- [ ] No
- [ ] N/A

Comments:

DME Needed:

- [x] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers: .

Anticipated Discharge Date:

3/23/2026

Case Management Narrative:

Patient will discharge home with home health

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Score: 0.00

Electronically Signed By:
Sylvia Valenzuela, RN, 03/17/2026 18:36

3/17/2026 7:01 Margaret M Demer, PT, DPT Note Discipline: Physical Therapist Tag: Daily Note

Form: PT Note (P33)

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 07:37 AM
Time Out: 08:37 AM
Total Time of Session: 60

Late Entry

- [ ] This is a late entry

Will this note include a Weekly Team Conference note?

( ) Yes (●) No

PT Subjective

Subjective/Observations:

Pt received seated in recliner in room, agreeable to PT session at this time. Pt denies HA/pain at this time.

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vitals Taken

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Did you take Vitals Pre-Treatment?
- ○ Yes
- ○ No

Did you take Vitals During Treatment?
- ○ Yes
- ○ No

Did you take Vitals Post-Treatment?
- ○ Yes
- ○ No

PT Objective for Daily Note

PT Mobility for Daily Note

	Assistance Level	Comment
Rolling Right		
Rolling Left		
Supine to Sit		
Sit to Supine		
Bed to Chair		
Sit to Stand		

Comments for Functional Mobility

PT Ambulation for Daily Note

	Distance	Device
Results		
Comments		
Gait Deviations		
Level of Assistance		

PT Ambulation for Daily Note (Cont.)

	Gait Deviations	Level of Assistance
Results		
Comments		
Gait Deviations		
Level of Assistance		

Gait Deviations:

Comments for Ambulation

PT Stairs for Daily Note

	Number of Steps	Device
Results		
Comments		

PT Stairs for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments for Stairs

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Wheelchair Mobility for Daily Note

	Distance	Type
Results		
Comments		

Wheelchair Mobility for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments

Therapeutic Intervention

Therapeutic Exercise:

SCI fit to improve endurance, promote reciprocal motion: L3 x12 min

Therapeutic Activity:

STS from multiple surfaces throughout session SBA
3x10 unsupported STS from elevated EOM SBA
standing reaching OBOS pt played connect 4 ~8 min SBA, no LOBs

Prosthetic/Orthotic Management

Exercise Equipment (Sci Fit / Nu Step, etc.):

Neuromuscular Re-Education:

Gait:

pt ambulated ~250 ft x5 with FWW SBA
fwd and retro ambulation with FWW ~30 ft x3 ea CGA-SBA

W/C Mobility/Management Training

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Additional LE Measurements taken?

What special tests were performed for PT

- ☐ None
- ☐ 2 Minute Walk Test
- ☐ 6 Minute Walk Test
- ☐ 10 Meter Walk Test
- ☐ Timed Up and Go (TUG)
- ☐ Berg
- ☐ Tinetti Assessment
- ☐ Activities-Specific Balance Test
- ☐ Function in Sitting Test (FIST) Results
- ☐ 5 Time Sit to Stand Test
- ☐ Amputee Mobility Predictor Assessment Tool
- ☐ Functional Gait Assessment
- ☐ Lower and Thoracic Evaluation

Other:

Additional Treatment:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT Assessment

PT Assessment

Pt tolerated session this AM well. Demonstrates improved stability and safety awareness during ambulation this session. This session concluded with pt seated in recliner in room with breakfast present, call light in reach, all needs met.

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

PT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Decreased Strength
- [x] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [x] Impaired Safety Awareness
- [ ] Impaired Communication
- [ ] Impaired Vision/Perception

DME Needed:

- [ ] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
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- [ ] Arm Trough
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- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers:

PT Response to Treatment

Functional Goals Focused On:

PT Functional Progress

Needs Continued Focus On:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT Plan of Care

PT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency:

Total number of days anticipated: 14

Patient/Family Goals:

Return to PLOF of Indep c ADLs, amb and grooming

PT Education

Education Provided On:

- [ ] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [ ] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Weight Bearing Precautions
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule

Use of Equipment:

Other:

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other:

Family/Caregiver Training

none

PT Objective for Team Conference

Gait Deviations:

Score: 0.00

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Electronically Signed By:
Margaret M Demer, PT, DPT, 03/17/2026 08:59

3/17/2026	8:45	Mara Cleary, CCC-SLP	Note Discipline: Speech-Language Pathologists	Tag: Daily Note

Form: SLP Note [P19]

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: _____
Time Out: _____
Total Time of Session: _____

Late Entry

- [ ] This is a late entry

Will this note include a Weekly Team Conference note?

(●) Yes ( ) No

SLP Subjective

Subjective/Observations:

_____

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vital Signs

Did you take Vitals Pre-Treatment?

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

○ Yes ○ No

Did you take Vitals Post-Treatment?

○ Yes ○ No

SLP Objective for Daily Note

SLP Objective

	Level of Assistance	Comments
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

Dysphagia

Was Dysphagia Addressed?

○ Yes ○ No

Speech Production

Was Speech Production Addressed?

○ Yes ○ No

Cognition / Language

Was Cognition/Language Addressed?

○ Yes ○ No

SLP Special Tests

SLP Special Tests

	Admission Score	Week 1 Score	Week 2 Score	Week 3 Score
Addenbrooke's Cognitive Exam				
Attention Processing Training				
Boston Naming Test 2nd Edition				
Cog Linguistic Quick Test Plus				
Montreal Cognitive Assessment				
NIH Stroke Scale				
Quick Aphasia Battery				
Texas Functional Living Scale				
Western Aphasia Battery-Revise				
Other (Specify in Comments)	14/30			

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

SLP Special Tests (Cont.)

	Week 4 Score	Week 5 Score	Discharge Score
Addenbrooke's Cognitive Exam			
Attention Processing Training			
Boston Naming Test 2nd Edition			
Cog Linguistic Quick Test Plus			
Montreal Cognitive Assessment			
NIH Stroke Scale			
Quick Aphasia Battery			
Texas Functional Living Scale			
Western Aphasia Battery-Revise			
Other (Specify in Comments)			

SLP Test Interpretation

Pt participated in the St. Louis University Mental Status Examination (SLUMS) for standardized assessment of cognitive-linguistic function.
She scored 14/30.

Orientation: 2/3
Calculations: 0/3
Generative naming: 2/3
Short-term recall: 2/5
Digit reversal: 1/2
Clock drawing: 2/4
Shape discrimination: 1/2
Auditory comprehension / Immediate recall: 4/8

SLP Education

Education Provided on:

- ☐ Goals for Inpatient Rehab/Plan of Care
- ☐ Therapy Schedule
- ☐ Language Disorder
- ☐ Safety Awareness
- ☐ Aphasia
- ☐ Cognitive Dysfunction
- ☐ Dysarthria
- ☐ Dysphagia
- ☐ Amputation Program Handbook
- ☐ SCI Program Handbook
- ☐ Stroke Program Handbook
- ☐ TBI Program Handbook
- ☐ Strategies _____

Other: _____

Education Topic(s):

_____

Education Provided to:

- ☐ Patient
- ☐ Family
- ☐ Caregiver

Other: _____

Education Provided by:

- ☐ Demonstration
- ☐ Discussion
- ☐ Literature
- ☐ Video

Other: _____

Response to Education:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Identified Problems/Deficits:

- [ ] None
- [ ] Cognition/Communication
- [ ] Swallowing Disorder

Other:

Family/Caregiver Training

SLP Assessment

SLP Assessment

Is patient being discharged in next 24 hours?

- ○ No
- ○ Yes

SLP Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [ ] Decreased Oral Motor Skills
- [x] Dysphagia
  - [ ] Oral
  - [ ] Pharyngeal
- [x] Decreased Cognition
- [x] Decreased Attention
- [x] Impaired Safety Awareness
- [ ] Aphasia
- [ ] Decreased Receptive Language
- [x] Decreased Expressive Language
- [ ] Decreased Vocal Skills
- [ ] Dysarthria
- [ ] Apraxia

SLP Plan of Care

Speech Therapy Intensity/Frequency

- [ ] Not required
- [x] 1 hour/day minimum of 5 days per week
- [ ] 0.5 hours/day minimum of 5 days per week
- [ ] 0.5 hour/week, 3-5 days per week

Other Intensity/Frequency:

Total number of days anticipated: 21

Patient/Family Goals

Improve my memory and speech

SLP Response to Treatment

Education Provided:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

SLP Weekly Progress Team Conference

SLP Objective for Team Conference

	Admission	Week 1
Attention	Mod	
Cognitive Deficit	Mod	
Communication Deficit	Mod	
Comprehension Deficit	Ind	
Swallowing Impairment	Ind	

SLP Objective for Team Conference (Cont.)

	Week 2	Week 3
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

SLP Objective for Team Conference (Cont.)

	Week 4	Goal
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

Comments for SLP Objective

Functional Goals Focused on:

SLP Weekly Progress Towards Functional Goals:

Patient's Response and/or Demonstration for Team Conference:

Score: 0.00

Electronically Signed By:
Mara Cleary, CCC-SLP, 03/17/2026 15:35

3/17/2026 12:00 Diana Ganem, PharmD Note Discipline: Pharmacist Tag: RX - Team Conference

Form: Pharmacy Weekly Team Conference Note [E72]

Pharmacy

Medications:

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

Antibiotics/Diagnosis for Antibiotics:

☐ N/A

Anticoagulation:

☐ N/A

Neuro/Psych:

☐ N/A

Cardiovascular:

☐ N/A

Pain Control:

☐ N/A

Electrolyte/Nutrition Replacement:

☐ N/A

Other (List Significant Medication and Reason for Taking):

☐ N/A

Pharmacy Interventions:

☐ N/A

No Interventions

Score: 0.00

Electronically Signed By:

Diana Ganem, PharmD, 03/17/2026 12:01

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

3/17/2026 13:04 Joseph Wong, MD Note Discipline: Physician Tag: Progress Note

Form: Rehab Progress Note [PPN]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

E785 - Hyperlipidemia, unspecified

I77811 - Abdominal aortic ectasia

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

Z6834 - Body mass index [BMI] 34.0-34.9, adult

Subjective

Subjective:

No acute events overnight. participating in therapies. denies pain at present.

Denies fever/chills, chest pain, SOB, cough, nausea, vomiting, diarrhea, constipation, dysuria, new weakness or changes in sensation. Complete ROS negative unless mentioned above.

The patient was discussed in interdisciplinary team conference. Results and recommendations were communicated to the patient and/or family, who is in agreement with plan. All questions were answered.

I spent greater than 50 minutes on the unit today, in which greater than 50% of this time was spent face to face with the patient and coordinating care with therapy disciplines, nursing, and case manager. The care of the patient was extensively discussed with all care providers in weekly case conference today. An appropriate rehabilitation plan was formulated for this patient. Barriers to progression of therapy were identified and appropriate interventions discussed with each discipline.

Physical Exam

Blood Pressure:

Systolic (mmHg): 157 Diastolic (mmHg): 83 Position: Lying Side: Left

Comment:

Pulse:

B/min: 105 Position: Lying Site: Monitor Type: Regular

Comment:

Respiration:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Per Minute: 20 Vent: No
Comment:

Weight:

lb: 183.0 Scale: Bed
Comment:

Height:

Feet: 5 Inches: 1
Comment:

Physical Exam

General:

- [x] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments:

Skin:

- [x] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments:
- [ ] Abnormal

Abnormal findings:

HEENT:

- [x] Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: mm dry
- [ ] Abnormal

Abnormal findings:

Neck:

- [x] Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments:
- [ ] Abnormal

Abnormal findings:

Heart:

- [x] Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments:
- [ ] Abnormal

Abnormal findings:

Lungs/Chest:

- [x] Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments:
- [ ] Abnormal

Abnormal findings:

Abdomen:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments:

- [x] Abnormal

Abnormal findings:
obese
soft ntnd

Extremities:

- [x] Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments:

- [ ] Abnormal

Abnormal findings:

Back:

- [x] Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments:

- [ ] Abnormal

Abnormal findings:

Cranial Nerves: ii. Optic

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Abnormal

Other:

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: xii. Hypoglossal

Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Gait and station are normal.

- [ ] Normal
- [x] Abnormal

Other:

fluent speech, follows commands
bilateral UE 5/5
bilateral LE 5-/5

Motor Functions: Romberg test is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Muscle tone is normal. No abnormal movements.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Plantar responses are down going.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Sensory: Sensory examination to pinprick and vibration is normal.

- [ ] Normal

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Abnormal

Other:

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed

Comment/Non-interfaced results:

Diagnostic Imaging Reviewed

Comment/Non-interfaced results:

Microbiology Tests Reviewed

Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Medical Conditions and Risks

Medical Conditions requiring close physician supervision:

- [x] Bowel & Bladder
- [x] Management of Pain
- [x] Medications
- [ ] Family Education/Counseling
- [x] Respiratory Complications
- [ ] Communication with Outside MDs

Other:

Risks being managed:

- [x] Infection
- [ ] Electrolyte Imbalance
- [x] Dehydration/Decrease Nutrition
- [x] DVT, PT/INR
- [x] Medication Management/Interactions

Other:

Functional Risks/Deficits

Functional Risks/Deficits needing close physician supervision:

- [x] Safety Awareness
- [x] Endurance for Activity
- [x] Transfers
- [ ] Bowel & Bladder Incontinence
- [x] ADL Performance
- [ ] Cognition/Language

Other:

Therapy and Nursing Focus

Continue PT/OT functional focus on:

- [x] Transfer Training
- [x] ADL Retraining
- [x] Ambulation Training
- [x] Safety Training
- [x] Balance Training
- [x] Improving ROM
- [x] Strengthening Exercises
- [x] Positioning
- [x] Fall Prevention
- [ ] Orthopedic Issues
- [x] Adaptive Equipment Training
- [ ] Caregiver Education

Other:

Continue ST functional focus on:

- [ ] NA
- [ ] Memory Strategies
- [ ] Swallowing Techniques
- [ ] Express/Comprehension
- [ ] Problem Solving
- [ ] Cognitive Retraining

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- [ ] Safety Judgment
- [ ] Linguistics
- [ ] Caregiver education

Continue Rehab Nursing focus on:

- [ ] Bowel & Bladder Retraining
- [x] Pressure Relief/Skin Integrity/ Positioning
- [x] Fall Prevention
- [x] Carryover of Self-Care
- [x] Carryover of Cognitive Techniques
- [x] Med Management/Education
- [x] Carryover of Mobility Skills
- [ ] Carryover of Swallowing techniques
- [ ] Caregiver education

Other:

Barriers to Participating in Therapy

Barriers to Participating in Therapy

- [ ] None at this time
- [x] fatigue/decrease tolerance for activity
- [ ] orthostatic hypotension
- [ ] noncompliant behavior
- [x] pain
- [ ] agitation
- [ ] nausea/vomiting
- [ ] change in mental status
- [ ] altered sleep/wake cycle

Other:

Functional and Medical Progress Since Last Note:

PT Assessment

Pt tolerated session this PM well. Pt demonstrates some difficulty with balance challenges performed this date, would benefit from continued balance training in future sessions. This session concluded with pt seated in room with son present, all needs met.

OT Assessment

See Conf Grid. Pt continues to demo dizziness

SLP Assessment

Medical Progress Since Last Note:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Current Medications

Medications:

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

Medication Changes

Medications Changes Listed and Described: _____

Medical Assessment & Plan (progress & goals):

Is the patient discharging within 24 hours?

○ Yes ○ No

Medical Assessment and Plan (progress & goals):

Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist

HTN
- lisinopril

HLD
- on statin

Skin
- monitor per nursing protocol for decubitus ulcers
- frequent offloading as indicated

Bowel - constipation

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- colace/senna
- lactulose, dulcolax suppository PRN

Bladder
- monitor for retention
- check PVRs

DVT ppx
- SCDs

Functional Assessment & Plan (progress & goals):

Functional Assessment and Plan (progress & goals):

Patient is making progress towards functional goals. Continue rehab plan as above. Discharge date: 3/23

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

Physician Addendum

Electronically Signed By:

Joseph Wong, MD, 03/17/2026 13:04

3/17/2026	13:18	Kennedy Kelly, OT	Note Discipline: Occupational Therapist	Tag: Daily Note

Form: OT Note [P13]

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 01:00 AM
Time Out: 02:00 AM
Total Time of Session: 60

Late Entry

☐ This is a late entry

Will this note include a Weekly Team Conference note?

( ) Yes (●) No

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

OT Subjective

Subjective/Observations:

Pt was seated in bed upon therapist arrival. Pt noted wanting to shower this session.

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vitals Taken

Did you take Vitals Pre-Treatment?

○ Yes ○ No

Did you take Vitals During Treatment?

○ Yes ○ No

Did you take Vitals Post-Treatment?

○ Yes ○ No

OT Objective for Daily Note

Did OT/COTA shower/bathe the patient today?

◉ Yes ○ No

Activities of Daily Living

	Assistance Level	Comments
Eating		
Grooming	Mod I	
Oral Hygiene		
Toilet Hygiene		
Shower/Bathe	Mod I	
Dressing UE	Min	to don/doff the ties on gowns
Dressing LE		
Putting On/Taking Off Footwear	Mod I	
Bed Transfer	Mod I	
Toilet Transfer		
Tub Transfer		

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Shower Transfer	Mod I	

Activities of Daily Living (Cont.)

	Comments
Eating	
Grooming	
Oral Hygiene	
Toilet Hygiene	
Shower/Bathe	
Dressing UE	
Dressing LE	
Putting On/Taking Off Footwear	
Bed Transfer	
Toilet Transfer	
Tub Transfer	
Shower Transfer	

Comments for ADLs

Wheelchair Mobility for Daily Note

	Distance	Type
Results		
Comments		

Wheelchair Mobility for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments for Wheelchair Mobility

Therapeutic Intervention

Therapeutic Exercise:

Theraband Color: _____

Weights (lbs): _____

Therapeutic Activity:

Prosthetic/Orthotic Management

Self Care Home Management:

IADL:

- ☐ Communication (Phones, computers etc)
- ☐ Financial Mgmt
- ☐ Health Mgmt (Scheduling appts)
- ☐ Household Mgmt
- ☐ Medication Mgmt
- ☐ Meal Prep
- ☐ Shopping (Making lists, selecting items, payment method)
- ☐ Transportation Mgmt
- ☐ Other _____

IADL Comments: _____

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Neuromuscular Re-Education:

Cognitive Function:

W/C Mobility/Management Training

What additional test(s) were performed for OT?

- [ ] None
- [ ] Amputee Mobility Predictor Assessment Tool
- [ ] Function in Sitting Test (FIST)
- [ ] Katz Index of Independence in Activities of Daily Living
- [ ] Nine Hole Peg Test

Other:

Additional Treatment:

See ADL section for assistance. Pt required Min VC to increase safety awareness when participating in showering task as seen by putting one foot onto shower bench to dry BLE and standing throughout shower session. Pt demo'd increased difficulty manipulating shower head.
Pt walked from room to gym and back w/ FWW requiring MOD VC to not run into objects on the left side while on the way to the gym w/ Max verbal/physical cueing on the way back to room.
Bike 1.5 resistance for 15 minutes. Pt demo'd poor short-term memory during this exercise as seen by frequently reasking duration of exercise every few minutes.

Session ended w/ pt supine in bed w/ call button within reach.

Comments:

OT Assessment

OT Assessment

Pt was motivated to participate in tx session. Patient making improvements in overall endurance and strength. Continue POC to further improve independence.

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

OT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Impaired Safety Awareness
- [ ] Impaired Vision/Perception
- [x] Decreased Strength
- [ ] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [ ] Impaired Communication

DME Needed:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers:

OT Response to Treatment

Functional Goals Focused On:

OT Functional Progress:

Needs Continued Focus On:

OT Plan of Care

OT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [ ] 1.5 hours/day minimum of 5 days per week

Other Intensity/Frequency: 30-180 mins/ 5-7 days/wk

Total number of days anticipated: 14

Patient/Family Goals:

Not really I think I'm doing okay, I can go home and do whatever I need to do

OT Education

Education Provided On:

- [x] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [x] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Home Exercise Program

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule
- [ ] Sexual Health
- [ ] Vision Changes

Use of Equipment:

Other:

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other:

Family/Caregiver Training

N/A

Score: 0.00

Electronically Signed By:

Kennedy Kelly, OT, 03/17/2026 13:56

3/17/2026	15:34	Mara Cleary, CCC-SLP	Note Discipline: Speech-Language Pathologists	Tag: Daily Note

Form: SLP Note [P19]

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 01:30 PM

Time Out: 02:30 PM

Total Time of Session: 60

Late Entry

- [ ] This is a late entry

Will this note include a Weekly Team Conference note?

- ( ) Yes
- (●) No

SLP Subjective

Subjective/Observations:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vital Signs

Did you take Vitals Pre-Treatment?

○ Yes ○ No

Did you take Vitals Post-Treatment?

○ Yes ○ No

SLP Objective for Daily Note

SLP Objective

	Level of Assistance	Comments
Attention		
Cognitive Deficit		
Communication Deficit		
Comprehension Deficit		
Swallowing Impairment		

Dysphagia

Was Dysphagia Addressed?

○ Yes ○ No

Speech Production

Was Speech Production Addressed?

○ Yes ○ No

Cognition / Language

Was Cognition/Language Addressed?

○ Yes ○ No

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

SLP Education

Education Provided on:

- [ ] Goals for Inpatient Rehab/Plan of Care
- [ ] Therapy Schedule
- [ ] Language Disorder
- [ ] Safety Awareness
- [ ] Aphasia
- [ ] Cognitive Dysfunction
- [ ] Dysarthria
- [ ] Dysphagia
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Strategies

Other:

Education Topic(s):

Education Provided to:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Education Provided by:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Identified Problems/Deficits:

- [ ] None
- [ ] Cognition/Communication
- [ ] Swallowing Disorder

Other:

Family/Caregiver Training

SLP Assessment

SLP Assessment

Is patient being discharged in next 24 hours?

- ○ No
- ○ Yes

SLP Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [ ] Decreased Oral Motor Skills
- [x] Dysphagia
  - [ ] Oral
  - [ ] Pharyngeal
- [x] Decreased Cognition
- [x] Decreased Attention
- [x] Impaired Safety Awareness
- [ ] Aphasia
- [ ] Decreased Receptive Language
- [x] Decreased Expressive Language
- [ ] Decreased Vocal Skills
- [ ] Dysarthria

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Apraxia

SLP Plan of Care

Speech Therapy Intensity/Frequency

- [ ] Not required
- [x] 1 hour/day minimum of 5 days per week
- [ ] 0.5 hours/day minimum of 5 days per week
- [ ] 0.5 hour/week, 3-5 days per week

Other Intensity/Frequency: _____

Total number of days anticipated: 21

Patient/Family Goals

Improve my memory and speech

SLP Response to Treatment

Functional Goals Focused on:

SLP Functional Progress:

Needs Continued Focus on:

Education Provided:

Patient's Response and/or Demonstration:

Score: 0.00

Electronically Signed By:

Mara Cleary, CCC-SLP, 03/17/2026 15:34

3/17/2026 21:40 Samer Hasson, MD Note Discipline: Physician Tag: Consult Note

Form: Provider Consult Note [39]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

E785 - Hyperlipidemia, unspecified

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

I77811 - Abdominal aortic ectasia

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

Z6834 - Body mass index [BMI] 34.0-34.9, adult

Review of History

History of Present Illness

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

PMH/SH/FH reviewed

- [ ] No change since admission
- [ ] Changes/updates documented below

Past Medical History

Add comments to any medical history that needs further details

- [x] None
- [ ] AIDS/HIV
- [ ] A-Fib
- [ ] Allergic Rhinitis
- [ ] Alzheimer's
- [ ] Anemia
- [ ] Anxiety
- [ ] Arthritis (Osteo)
- [ ] Arthritis (Rheumatoid)
- [ ] Benign Prostate Hypertrophy
- [ ] CAD
- [ ] Cancer
- [ ] CHF
- [ ] Chronic Back Pain
- [ ] Cirrhosis
- [ ] Compression Fracture
- [ ] Dementia
- [ ] DJD
- [ ] Diabetes
- [ ] Diverticulitis/losis
- [ ] Depression
- [ ] Deep Vein Thrombosis
- [ ] Epilepsy
- [ ] GERD
- [ ] Glaucoma
- [ ] GI Bleed
- [ ] Gout
- [ ] Heart Disease
- [ ] Hiatal Hernia
- [ ] HTN
- [ ] Hyperlipidemia
- [ ] Kidney/Bladder
- [ ] Liver Disease
- [ ] Lung Disease
- [ ] Macular Degeneration
- [ ] MI (Not Primary Med Condtn)
- [ ] Obesity
- [ ] Orthopedic
- [ ] Osteoporosis
- [ ] Peptic Ulcer
- [ ] Peripheral Vascular Disease
- [ ] Pulmonary Embolis
- [ ] Seizures
- [ ] Sleep Apnea
- [ ] Stroke
- [ ] TB History
- [ ] Thyroid Disease

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] TIA

Other Medical History: _____

Additional Comments: _____

Past Surgical History

Add comments to any surgical history that needs further details

- [x] None
- [ ] AICD
- [ ] Amputation(s)
- [ ] Anesthesia difficulties
- [ ] Angioplasty
- [ ] Appendectomy
- [ ] Biopsy (site)
- [ ] Bronchoscopy
- [ ] CABG
- [ ] Carotid (CEA)
- [ ] Carpal Tunnel
- [ ] Cataract(s)
- [ ] Cholecystectomy
- [ ] Craniotomy
- [ ] Esophageal Surgery
- [ ] Gastric Surgeries (Type)
- [ ] Heart Valve Surgery
- [ ] Hemorrhoidectomy
- [ ] Hernia Repair (location)
- [ ] Hysterectomy
- [ ] Joint Replacement (location)
- [ ] Lithotripsy
- [ ] Mastectomy (R/L/B)
- [ ] Orthopedic
- [ ] Ostomies (Describe)
- [ ] Pacemaker
- [ ] PEG
- [ ] Pneumonectomy (lobes)
- [ ] Spinal Surgery
- [ ] Stent Placement (location)
- [ ] Tonsillectomy
- [ ] Thyroid Surgery
- [ ] Tracheostomy

Other Surgical History: _____

Additional Comments: _____

Family Medical History

Have any of these conditions occurred on either side of the patient's family? (Check if yes)

- [x] Not applicable
- [ ] Allergies
- [ ] Cardiovascular disease
- [ ] Gastrointestinal disease
- [ ] Pulmonary disease
- [ ] Bleeding tendencies
- [ ] Tuberculosis
- [ ] Psychiatric illness
- [ ] Bone disease
- [ ] Kidney disease
- [ ] Congenital deformities
- [ ] Alcohol/drug dependence
- [ ] Cancer/Tumors
- [ ] Diabetes

Other: _____

Comments: _____

Marital Status

Marital Status: Widowed

Current or past use of alcohol?

- ( ) Yes
- (•) No

Recreational/Illicit Drugs

- [x] Denies Use
- [ ] Past History of Use
- [ ] Current Use

Type: _____ Frequency: _____

Date last used: _____

Tobacco/Smoking

- [ ] Current Smoker Packs/day: _____ How many years: _____
- [x] Non-Smoker

Previous History of Smoking (indicate last time used and previous PPD): _____

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- [ ] Vapes Vapes Frequency: _____
- [ ] Marijuana Marijuana Frequency: _____

Other: _____

Consult ROS

Review of System Status Update

- [ ] N/A (Admission Review of Systems)
- [x] NO CHANGES in Condition Since Last System Review.
- [ ] CHANGES HAVE Occurred & Amended Below to Reflect an Update to the Patient's Current Condition.
- [ ] Unable to Obtain Review of Systems. Reason: _____

Review of Systems

_____

Complications HEENT:

Check All that Apply.

- [ ] Jaundice
- [ ] Diplopia
- [ ] Photophobia
- [ ] Rhinitis
- [ ] Nose Bleeds
- [ ] Packed Left Nostril
- [ ] Packed Rt. Nostril
- [ ] Tooth ache
- [ ] Ear ache
- [ ] Sore throat
- [ ] Sinus tender
- [ ] Lymphadenopathy

Other: _____

- [ ] None of the Above

Complications of Respiratory System:

Check All that Apply.

- [ ] Intubated
- [ ] SOB at Rest
- [ ] SOB with Exertion
- [ ] Orthopnea
- [ ] Snores
- [ ] Hemoptysis
- [ ] Chills
- [ ] Night Sweats
- [ ] Fatigue
- [ ] Recent Weight Loss
- [ ] Ineffective & Non-Productive Cough
- [ ] Frequent Suctioning
- [ ] Persistent cough less than 2 weeks
- [ ] Persistent cough greater than 2 weeks
- [ ] Tracheostomy
- [ ] Sleep Apnea
- [ ] Oxygen Dependent

Other: _____

- [ ] None of the Above

Complications of Circulatory System:

Check all that Apply.

- [ ] HTN
- [ ] Hypotension
- [ ] Orthostatic Hypotension
- [ ] Tachycardia
- [ ] Bradycaria
- [ ] Dysrhythmia
- [ ] Edema
- [ ] History of Chest Pains at Rest
- [ ] History of Chest Pains with Activity

Other: _____

- [ ] None of the Above

Complications GI System:

Check all that Apply.

- [ ] Vomiting in Past 72 Hours
- [ ] Diarrhea Within past 72 Hours
- [ ] Chronic Constipation
- [ ] Weight Loss
- [ ] Weight Gain
- [ ] Indigestion
- [ ] Reflux

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- [ ] Rectal Bleeding
- [ ] Tarry Stools
- [ ] Rectal Tube
- [ ] Gastric Decompression
- [ ] Jaundice
- [ ] Encephalopathy

Other: _____

- [ ] None of the Above

Complications Endocrine System:

Check all that Apply.

- [ ] Episodes of Hypoglycemia
- [ ] Episodes of Hyperglycemic
- [ ] Frequent Urination
- [ ] Excessive Thirst
- [ ] Fatigue/Malaise
- [ ] Hyperactivity

Other: _____

- [ ] None of the Above

Complications Genitourinary System:

Check all that Apply.

- [ ] Incontinent
- [ ] Neurogenic Bladder
- [ ] Anuria
- [ ] Hematuria
- [ ] Cloudy
- [ ] Odor
- [ ] Urgency
- [ ] Retention
- [ ] Difficulty starting stream
- [ ] Vaginal/Penile Bleeding or Drainage

Other: _____

- [ ] None of the Above

Complications Musculoskeletal/Mobility:

Check all that Apply.

- [ ] Fall Risk
- [ ] Activity Intolerance
- [ ] Poor Balance
- [ ] Weakness/Fatigue
- [ ] Joint Swelling
- [ ] Joint Pain
- [ ] Contractures
- [ ] Dizziness
- [ ] Amputation
- [ ] Hemiplegia
- [ ] Monoplegia
- [ ] Quadrapeligia

Other: _____

- [ ] None of the Above

Complications of Pain:

Check all that Apply.

- [ ] Chronic Pain
- [ ] Intractable Pain
- [ ] Decreased Mental Status
- [ ] Agitation
- [ ] Inability to Concentrate
- [ ] Anxiety
- [ ] Nausea/Vomiting
- [ ] Constipation
- [ ] Itching

Other: _____

- [ ] None of the Above

Pain Management:

- [ ] N/A
- [ ] Heat
- [ ] Ice
- [ ] TENS Unit
- [ ] Positioning
- [ ] Biofeedback
- [ ] Medication

Comments: _____

Complications Skin/Wound:

Check all that Apply.

- [ ] Urinary Incontinence
- [ ] Fecal Incontinence
- [ ] Wound Drainage
- [ ] Erythema
- [ ] Skin Maceration
- [ ] Skin Tear
- [ ] Incision Dehiscense
- [ ] Incision Evisceration
- [ ] Stoma Care

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Poor Nutrition
- [ ] Immobility

Other: _____

- [ ] None of the Above

Complication Neuro/Mental Status:

Check all that Apply.

- [ ] Altered Mental Status
- [ ] Dementia Confusion
- [ ] Emotional Liability/Depression
- [ ] Seizures
- [ ] Headache
- [ ] Dizziness/Balance Loss
- [ ] Speech Problem
- [ ] Loss of Gag Reflex
- [ ] Loss of Memory
- [ ] Loss of Safety Awareness
- [ ] Paralysis - Location: _____

Other: _____

- [ ] None of the Above

All Recent Investigations Noted

- ○ All Recent Investigations Noted

Consult PE

Blood Pressure:

Systolic (mmHg): _____ Diastolic (mmHg): _____ Position: _____ Side: _____

Comment: _____

Pulse:

B/min: _____ Position: _____ Site: _____ Type: _____

Comment: _____

Respiration:

Per Minute: _____ Vent: _____

Comment: _____

Weight:

lb: 183.0 Scale: Bed

Comment: _____

Height:

Feet: 5 Inches: 1

Comment: _____

Physical Exam

HEENT: Normocephalic, atraumatic, moist oral mucosa.
NECK: Supple, No JVD, No carotid bruit, No thyromegaly.
Cardiovascular system: S1 S2 normal. Regular heart rate and rhythm. No murmurs
Respiratory system: Good air entry bilaterally in chest without any crackles. No chest wall tenderness. No use of accessory muscles.
Gastrointestinal system: Abdomen full soft, moves with respiration. No organomegaly, No tenderness. Bowel sounds present.
Musculoskeletal system: No deformities. No swelling.
Psychiatry: Good affect. Oriented to place, time and person.
Skin: No lesions, warm and dry. Wounds deferred per wound team

General:

- [ ] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious

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☐ Oriented to person ☐ Oriented to place ☐ Oriented to time ☐ Oriented to situation

Comments: _____

Skin:

☐ Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments: _____

☐ Abnormal

Abnormal findings: _____

HEENT:

☐ Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Neck:

☐ Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Heart:

☐ Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Lungs/Chest:

☐ Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Abdomen:

☐ Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Extremities:

☐ Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Back:

☐ Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Gait and station are normal.

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Romberg test is normal.

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Plantar responses are down going.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- ☐ Normal
- ☐ Abnormal

Other: _____

Sensory: Sensory examination to pinprick and vibration is normal.

- ☐ Normal
- ☐ Abnormal

Other: _____

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed _____

Comment/Non-interfaced results: _____

Diagnostic Imaging Reviewed

_____

Comment/Non-interfaced results: _____

Microbiology Tests Reviewed

Comment: _____

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

_____

Consult Impression/Plan

Action Plan

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ New Action Plan Initiated ☐ Continue Same Course of Care ☐ Updated Action Plan Initiated

Plan:

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

#Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist
#HTN
- lisinopril
#HLD
- on statin

#DVT ppx
- SCDs

Impression

03/15/2026 seen in the common area, walking with therapy, no acute events, stable
03/16/2026: no acute issues she is resting in her room, no acute issues , no new concerns per patient or staff , vitals are stable
03/17/2026: no acute issues , she is stating she is doing well , no issues per staff , vitals are stable , all questions were addressed and answered

Neuro/Psych Assessment and Plan

Respiratory Assessment and Plan

Cardiovascular Assessment and Plan

Renal/Genitourinary Assessment and Plan

Infectious Disease Assessment and Plan

Gastrointestinal/Stress Ulcer prophylaxis/Nutrition Assessment and Plan

DVT prophylaxis Assessment and Plan

Wounds/Pressure ulcer Assessment and Plan

Medications Reviewed

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

I have Reviewed the Following Patient Data:

- ☐ Prior Medical Records & Test Results
- ☐ Medication Reconciliation
- ☐ Allergies
- ☐ Consultant Report(s)
- ☐ Current/New Medications
- ☐ Current Lab Result & Diagnostic Test
- ☐ I Have Personally Discussed Code Status with this Patient or their Surrogate Decision Maker and Believe they Understand their Medical Condition, Prognosis and the Consequences of their Code Status Decision.
- ☐ All of the Above

Anticipated Post Hospital Service Needs:

- ☐ Home with Help
- ☐ Home with Home Health
- ☐ Assisted Living Facility
- ☐ SNF
- ☐ Acute Inpatient Rehab
- ☐ Home with Outpatient Therapy
- ☐ Hospice
- ☐ Group Home
- ☐ Post Acute Vent Care Unit

Other: _____

Coordinated care with

- ☐ Consultants
- ☐ Nursing
- ☐ Nutrition
- ☐ Therapist
- ☐ Family
- ☐ Attending
- ☐ PA

Other: _____

Plan Discussed in Detail with:

- ☐ Patient
- ☐ Family
- ☐ Nursing Staff
- ☐ Case Manager

Comment: _____

Other

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

#Debility - vertigo
#HTN
#HLD
99232
Patient is Full Code

Physician Addendum

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Electronically Signed By:
Samer Hasson, MD, 03/17/2026 21:41

3/18/2026 7:02 Margaret M Demer, PT, DPT Note Discipline: Physical Therapist Tag: Daily Note

Form: PT Note (P33)

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 08:26 AM
Time Out: 08:56 AM
Total Time of Session: 30

Late Entry

- [ ] This is a late entry

Will this note include a Weekly Team Conference note?

( ) Yes (●) No

PT Subjective

Subjective/Observations:

Pt received in recliner in room, agreeable to brief PT session at this time.

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vitals Taken

Did you take Vitals Pre-Treatment?

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

○ Yes ○ No

Did you take Vitals During Treatment?

○ Yes ○ No

Did you take Vitals Post-Treatment?

○ Yes ○ No

PT Objective for Daily Note

PT Mobility for Daily Note

	Assistance Level	Comment
Rolling Right		
Rolling Left		
Supine to Sit		
Sit to Supine		
Bed to Chair		
Sit to Stand		

Comments for Functional Mobility

PT Ambulation for Daily Note

	Distance	Device
Results		
Comments		
Gait Deviations		
Level of Assistance		

PT Ambulation for Daily Note (Cont.)

	Gait Deviations	Level of Assistance
Results		
Comments		
Gait Deviations		
Level of Assistance		

Gait Deviations:

Comments for Ambulation

PT Stairs for Daily Note

	Number of Steps	Device
Results		
Comments		

PT Stairs for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments for Stairs

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Wheelchair Mobility for Daily Note

	Distance	Type
Results		
Comments		

Wheelchair Mobility for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments

Therapeutic Intervention

Therapeutic Exercise:

3x10 ea in // bars: hip abd, hip ext, marches, heel raises

Therapeutic Activity:

pt performed several STSs this session IND

Prosthetic/Orthotic Management

Exercise Equipment (Sci Fit / Nu Step, etc.):

Neuromuscular Re-Education:

Gait:

pt ambulated with FWW ~250 ft x2 SBA

W/C Mobility/Management Training

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Additional LE Measurements taken?

What special tests were performed for PT

- ☐ None
- ☐ 2 Minute Walk Test
- ☐ 6 Minute Walk Test
- ☐ 10 Meter Walk Test
- ☐ Timed Up and Go (TUG)
- ☐ Berg
- ☐ Tinetti Assessment
- ☐ Activities-Specific Balance Test
- ☐ Function in Sitting Test (FIST) Results
- ☐ 5 Time Sit to Stand Test
- ☐ Amputee Mobility Predictor Assessment Tool
- ☐ Functional Gait Assessment
- ☐ Lower and Thoracic Evaluation

Other:

Additional Treatment:

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PT Assessment

PT Assessment

Pt tolerated brief session this AM well. Progressing towards ModI level at this time. This session concluded with pt seated in recliner in room with call light in reach, all needs met.

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

PT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Decreased Strength
- [x] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [x] Impaired Safety Awareness
- [ ] Impaired Communication
- [ ] Impaired Vision/Perception

DME Needed:

- [x] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers: .

PT Response to Treatment

Functional Goals Focused On:

PT Functional Progress

Needs Continued Focus On:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT Plan of Care

PT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency:

Total number of days anticipated: 14

Patient/Family Goals:

Return to PLOF of Indep c ADLs, amb and grooming

PT Education

Education Provided On:

- [ ] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [ ] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Weight Bearing Precautions
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule

Use of Equipment:

Other:

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other:

Family/Caregiver Training

none

PT Objective for Team Conference

Gait Deviations:

Score: 0.00

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Electronically Signed By:
Margaret M Demer, PT, DPT, 03/18/2026 08:53

3/18/2026 8:58 Anthony Populo, FNP-C Note Discipline: Nurse practitioner Tag: Progress Note

Form: Medicine APP Progress Note [PGC]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

E785 - Hyperlipidemia, unspecified

I77811 - Abdominal aortic ectasia

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

Z6834 - Body mass index [BMI] 34.0-34.9, adult

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Subjective

Subjective:

Patient is seen resting in bed at time of visit, she states she went to the hospital due to constant dizziness and headaches for weeks, she states she experienced the dizziness constantly while laying, sitting and standing, she states she had also been experiencing a headache during this time and no medication would relieve the headache, while in the hospital she states she received a migaine cocktail which did seem to help, denies experiencing ear ringing, she states she is experiencing a slight headache at time of visit but feels it is going to build to a stronger headache, she states she is also experiencing some dizziness while sitting up in bed, she states her dizziness has improved since her hospitalization. She states her dizziness is triggered when standing or trying to speak. She is AOx2, she has word finding difficulties, able to spell WORLD forwards but not backwards. Could not spell GAME backwards.

3/18/26 patient is seen resting in bedside chair at time of visit, she states she participated in physical therapy this morning and felt it went good, she states when she finished PT she felt dizzy with a lightheaded feeling and is still experiencing it at time of visit but it is resolving slowly. She is AOx2 - self and place, she is unable to recall year or month after being told correct answer after distraction, can recall city and state. She states she has been sleeping well.

ROR showed:
""72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026."

Active Diagnoses

Indicate the patient's primary medical condition category.

- ○ 1.Stroke
- ○ 2.Spinal cord injury
- ○ 3.Congenital deformity
- ○ 4.Amputation
- ○ 5.Major multiple trauma
- ○ 6.Fracture of femur(hip fracture)
- ○ 7.Brain injury
- ○ 8.Neurological disorders including MS, motor neuron diseases, MD or Parkinson's disease
- ○ 9.Burns
- ○ 10.Active, polyarticular RA, psoriatic arthritis, resulting in significant functional impairment
- ○ 11.Severe or advanced osteoarthritis involving 2 or more major weight bearing joints
- ○ 12.Knee or hip joint replacement, or both during acute hospitalization

Other Medical Condition- enter ICD code.: _____

Metastatic Cancer
- ○ yes

Severe Cancer
- ○ yes

Severe Left Systolic/Ventricular Dysfunction (known ejection fraction less than or equal to 30%)
- ○ yes

Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease
- ○ yes

Chronic Kidney Disease, Stage 5
- ○ yes

Acute Renal Failure
- ○ yes

Septicemia, Sepsis, Systemic Inflammatory Response Syndrome/Shock

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- yes

Central Nervous System Infections, Opportunistic Infections, Bone/Joint/ Muscle Infections/Necrosis
- yes

Diabetes Mellitus (DM)
- yes

Major Lower Limb Amputation (e.g., above knee, below knee)
- yes

Stroke
- yes

Dementia
- yes

Hemiplegia or Hemiparesis
- yes

Paraplegia
- yes

Complete Tetraplegia
- yes

Incomplete Tetraplegia
- yes

Other Spinal Cord Disorder/Injury (e.g., myelitis, cauda equina syndrome
- yes

Multiple Sclerosis (MS)
- yes

Huntington's Disease
- yes

Parkinson's Disease
- yes

Amyotrophic Lateral Sclerosis (ALS)
- yes

Other Progressive Neuromuscular Disease
- yes

Locked-In State
- yes

Severe Anoxic Brain Damage, Cerebral Edema, or Compression of Brain
- yes

Other Severe Neurological Injury, Disease, or Dysfunction
- yes

Malnutrition (protein or calorie)
- yes

At Risk for Malnutrition
- yes

Lung Transplant

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- ○ yes

Heart Transplant
- ○ yes

Liver Transplant
- ○ yes

Kidney Transplant
- ○ yes

Bone Marrow Transplant
- ○ yes

None of the above
- ○ yes

Review of Systems (ROS)

Review of System Status Update

- ☐ N/A (Admission Review of Systems)
- ☐ NO CHANGES in Condition Since Last System Review.
- ☐ CHANGES HAVE Occurred & Amended Below to Reflect an Update to the Patient's Current Condition.
- ☐ Unable to Obtain Review of Systems. Reason: _____

Review of Systems

General: Appetite is satisfactory. No significant weight change.
Eyes: Pt denies vision changes, pain, discharge or blurry vision.
Ears/Nose/Mouth/Throat: Pt denies ear pain, change in hearing. Pt denies nasal congestion/discharge or sinus pain. Pt denies sore throat, difficulty swallowing or new dental problems.
Respiratory: Pt denies cough, dyspnea, wheezing or chest pain
Cardiovascular: Pt denies chest pain or tightness, palpitations or edema.
Gastrointestinal: Pt denies abdominal pain, N/V/D, constipation
Musculoskeletal: Pt denies new joint pain or stiffness, redness or swelling.
Neurological: dizziness (+)
No loss of consciousness, transient ischemic symptoms or seizures

Complications HEENT:

- ☐ Jaundice
- ☐ Diplopia
- ☐ Photophobia
- ☐ Rhinitis
- ☐ Nose Bleeds
- ☐ Packed Left Nostril
- ☐ Packed Rt. Nostril
- ☐ Tooth ache
- ☐ Ear ache
- ☐ Sore throat
- ☐ Sinus tender
- ☐ Lymphadenopathy

Other: _____

- ☐ None of the Above

Complications of Respiratory System:

- ☐ Intubated
- ☐ SOB at Rest
- ☐ SOB with Exertion
- ☐ Orthopnea
- ☐ Snores
- ☐ Hemoptysis
- ☐ Chills
- ☐ Night Sweats
- ☐ Fatigue
- ☐ Recent Weight Loss
- ☐ Ineffective & Non-Productive Cough
- ☐ Frequent Suctioning
- ☐ Persistent cough less than 2 weeks
- ☐ Persistent cough greater than 2 weeks
- ☐ Tracheostomy

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- [ ] Sleep Apnea
- [ ] Oxygen Dependent

Other: _____

- [ ] None of the Above

Complications of Circulatory System:

- [ ] HTN
- [ ] Hypotension
- [ ] Orthostatic Hypotension
- [ ] Tachycardia
- [ ] Bradycaria
- [ ] Dysrhythmia
- [ ] Edema
- [ ] History of Chest Pains at Rest
- [ ] History of Chest Pains with Activity

Other: _____

- [ ] None of the Above

Complications GI System:

- [ ] Vomiting in Past 72 Hours
- [ ] Diarrhea Within past 72 Hours
- [ ] Chronic Constipation
- [ ] Weight Loss
- [ ] Weight Gain
- [ ] Indigestion
- [ ] Reflux
- [ ] Rectal Bleeding
- [ ] Tarry Stools
- [ ] Rectal Tube
- [ ] Gastric Decompression
- [ ] Jaundice
- [ ] Encephalopathy

Other: _____

- [ ] None of the Above

Complications Endocrine System:

- [ ] Episodes of Hypoglycemia
- [ ] Episodes of Hyperglycemic
- [ ] Frequent Urination
- [ ] Excessive Thirst
- [ ] Fatigue/Malaise
- [ ] Hyperactivity

Other: _____

- [ ] None of the Above

Complications Genitourinary System:

- [ ] Incontinent
- [ ] Neurogenic Bladder
- [ ] Anuria
- [ ] Hematuria
- [ ] Cloudy
- [ ] Odor
- [ ] Urgency
- [ ] Retention
- [ ] Difficulty starting stream
- [ ] Vaginal/Penile Bleeding or Drainage

Other: _____

- [ ] None of the Above

Indwelling Foley Medical Justification

- [ ] Order for foley is on patient chart and current
- [ ] Documented urinary retention that cannot be corrected and alternatives are not feasible
- [ ] Documented post void residual over 200mL
- [ ] Inability to manage retention/ incontinence with intermittent cath
- [ ] Contamination of Stage III or IV pressure injury with urine that has impeded healing
- [ ] Terminal illness or severe impairment
- [ ] Untreatable blockage
- [ ] Need for exact measurement of urine output
- [ ] History of being unable to void after having Foley removed
- [ ] Quad/paraplegia who failed a past attempt to remove Foley catheter
- [ ] Bladder tumors
- [ ] Spinal cord disease
- [ ] Nursing has notified that protocol is being followed for patient who can void routinely

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Nursing has notified that protocol is being followed for patient who can NOT void routinely

Complications Musculoskeletal/Mobility:

- [ ] Fall Risk
- [ ] Activity Intolerance
- [ ] Poor Balance
- [ ] Weakness/Fatigue
- [ ] Joint Swelling
- [ ] Joint Pain
- [ ] Contractures
- [ ] Dizziness
- [ ] Amputation
- [ ] Hemiplegia
- [ ] Monoplegia
- [ ] Quadrapeligia

Other: _____

- [ ] None of the Above

Complications of Pain:

- [ ] Chronic Pain
- [ ] Intractable Pain
- [ ] Decreased Mental Status
- [ ] Agitation
- [ ] Inability to Concentrate
- [ ] Anxiety
- [ ] Nausea/Vomiting
- [ ] Constipation
- [ ] Itching

Other: _____

- [ ] None of the Above

Pain Management:

- [ ] N/A
- [ ] Heat
- [ ] Ice
- [ ] TENS Unit
- [ ] Positioning
- [ ] Biofeedback
- [ ] Medication

Comments: _____

Complications Skin/Wound:

- [ ] Urinary Incontinence
- [ ] Fecal Incontinence
- [ ] Wound Drainage
- [ ] Erythema
- [ ] Skin Maceration
- [ ] Skin Tear
- [ ] Incision Dehiscense
- [ ] Incision Evisceration
- [ ] Stoma Care
- [ ] Poor Nutrition
- [ ] Immobility

Other: _____

- [ ] None of the Above

Complication Neuro/Mental Status:

- [ ] Altered Mental Status
- [ ] Dementia Confusion
- [ ] Emotional Liability/Depression
- [ ] Seizures
- [ ] Headache
- [ ] Dizziness/Balance Loss
- [ ] Speech Problem
- [ ] Loss of Gag Reflex
- [ ] Loss of Memory
- [ ] Loss of Safety Awareness
- [ ] Paralysis - Location: _____

Other: _____

- [ ] None of the Above

All Recent Investigations Noted

- ○ All Recent Investigations Noted

Physical Exam

Blood Pressure:

Systolic (mmHg): 153 Diastolic (mmHg): 63 Position: Sitting Side: Left

Comment: _____

Pulse:

B/min: 82 Position: Sitting Site: Monitor Type: Regular

Comment: _____

Respiration:

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Per Minute: 18 Vent: No
Comment:

Weight:

lb: 183.0 Scale: Bed
Comment:

Height:

Feet: 5 Inches: 1
Comment:

Physical Exam

General: This patient is well developed and in no acute distress.
Neck: Supple without thyromegaly, lymphadenopathy, or masses.
Respiratory: Normal respiratory effort.
Cardiovascular: Regular rhythm; no murmurs, rubs, or gallops.
Chest: Chest normal to inspection.
Gastrointestinal: Abdomen: Soft, nontender, and without masses. Bowel sounds active.
Musculoskeletal: Digits/nails: No clubbing, cyanosis, inflammation or ischemia.
Neurological:
MENTAL STATUS: The patient is Awake, alert and oriented x 2. Speech is normal. Attention span intact and can follow 3 step commands.

CRANIAL NERVES:
CN 2: intact.
CN3, 4, 6: Pupils are equal and round, 3mm. Reactive to direct and consensual, EOMI - conjugate. No INO, APD. No Horners
CN 5: normal facial sensation
CN 7: Symmetric facial movement
CN 8: Hearing intact
CN 9, 10: Symmetric palate movement and normal gag reflex
CN 11: Shoulder shrug strength normal
CN 12: Tongue is midline.

MOTOR:
RUE 5/5 proximal 5/5 distal
LUE: 5/5 proximal, 5/5 distal
RLE 5/5 proximal, 5/5 distal
LLE 5/5 proximal, 5/5 distal.

MOVEMENT: no tremor

ABNORMAL MOVEMENTS: No dyskinesias

DTR'S: R/L
Biceps 1+/1+
Triceps 1+/1+
Knees 1+/1+
Ankles 0/0

PLANTAR RESPONSE: B/L downgoing

COORDINATION: No ataxia / dysmetria

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HINTS exam (-)

General:

- [ ] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments: _____

Skin:

- [ ] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

HEENT:

- [ ] Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Neck:

- [ ] Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Heart:

- [ ] Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Lungs/Chest:

- [ ] Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Abdomen:

- [ ] Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Extremities:

- [ ] Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Back:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

- [ ] Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: Vii. Facial

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: vii. Auditor

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xi. Accessory

- [ ] Normal
- [ ] Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

- [ ] Normal
- [ ] Abnormal

Other: _____

Motor Functions: Gait and station are normal.

- [ ] Normal

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☐ Abnormal

Other: _____

Motor Functions: Romberg test is normal.

☐ Normal

☐ Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

☐ Normal

☐ Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

☐ Normal

☐ Abnormal

Other: _____

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

☐ Normal

☐ Abnormal

Other: _____

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

☐ Normal

☐ Abnormal

Other: _____

Motor Functions: Plantar responses are down going.

☐ Normal

☐ Abnormal

Other: _____

Motor Functions: Finger-to-nose and heel-to-shin is normal.

☐ Normal

☐ Abnormal

Other: _____

Sensory: Sensory examination to pinprick and vibration is normal.

☐ Normal

☐ Abnormal

Other: _____

CXR:

_____

EKG:

_____

Labs /Diagnostic Imaging Reviewed

Lab Results

_____

Comment/Non-interfaced results: _____

Diagnostic Imaging Reviewed

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Comment/Non-interfaced results:

Microbiology Tests Reviewed

Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Impression and Plan

Impression

#spontaneous vestibular syndrome - improved.
- differential dx include: vestibular neuritis vs. vestibular migraine vs. functional
- HINTS (-)
- currently on Meclizine
- consider starting triptan if headache worsens
- appreciate PT recommendations on Dix-Hallpike and Epley maneuvers.

Neuro/Psych Assessment and Plan

Respiratory Assessment and Plan

Cardiovascular Assessment and Plan

Renal/Genitourinary Assessment and Plan

Infectious Disease Assessment and Plan

Gastrointestinal/Stress Ulcer prophylaxis/Nutrition Assessment and Plan

DVT prophylaxis Assessment and Plan

Wounds/Pressure ulcer Assessment and Plan

Medications Reviewed

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

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Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

Action Plan

- [ ] New Action Plan Initiated
- [ ] Continue Same Course of Care
- [ ] Updated Action Plan Initiated

Plan:

I have Reviewed the Following Patient Data:

- [ ] Prior Medical Records & Test Results
- [ ] Medication Reconciliation
- [ ] Allergies
- [ ] Consultant Report(s)
- [ ] Current/New Medications
- [ ] Current Lab Result & Diagnostic Test
- [ ] I Have Personally Discussed Code Status with this Patient or their Surrogate Decision Maker and Believe they Understand their Medical Condition, Prognosis and the Consequences of their Code Status Decision.
- [x] All of the Above

Anticipated Post Hospital Service Needs:

- [ ] Home with Help
- [ ] Home with Home Health
- [ ] Assisted Living Facility
- [ ] SNF
- [ ] Acute Inpatient Rehab
- [ ] Home with Outpatient Therapy
- [ ] Hospice
- [ ] Group Home
- [ ] Post Acute Vent Care Unit

Other:

Coordinated care with

- [ ] Consultants
- [ ] Nursing
- [ ] Nutrition
- [ ] Therapist
- [ ] Family
- [ ] Attending
- [ ] PA

Other:

Plan Discussed in Detail with:

- [ ] Patient
- [ ] Family
- [ ] Nursing Staff
- [ ] Case Manager

Comment:

Other

50 minutes were spent reviewing history, labs, imaging and plan of care. This includes direct bedside attendance, interpretation of investigative studies, review of prior records, involvement of consultants.

This encounter was coordinated with supervising physician Dr. Eric Taylor. Half of the time (at least) was spent collaborating with supervising physician, Dr. Taylor.

Thank you for allowing me to participate in the care of this patient. Feel free to call me with any questions.

Anthony Populo, DNP, AGACNP - BC

Electronically Signed By:

Anthony Populo, FNP-C, 03/18/2026 18:05

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

3/18/2026	10:19	Kennedy Kelly, OT	Note Discipline: Occupational Therapist	Tag: Group Note

Form: OT Group Therapy Note [P56]

Occupational Therapy group Note

Initiated session (time):
10:00 AM

Duration of Session:
60

Late Entry
- [ ] This is a late entry

How many people were in group?
- (●) 2
- ( ) 3
- ( ) 4
- ( ) 5
- ( ) 6

Subjective:
Pt was seated in bedside chair upon therapist arrival. Pt was agreeable to tx group session.

Group Therapy Rationale:
- [x] Enhanced Motivation
- [x] Providing Encouragement
- [ ] Confidence in Skills Learned to Achieve Therapy Goals
- [ ] Community Reintegration
- [x] Peer Interaction
- [ ] Vicarious Learning
- [x] Decreased Social Isolation
- [x] Peer Communication in a Social Setting
- [x] Improved Functional Performance by Viewing Peer Performance
- [x] Feelings of Support by Peers in Similar Conditions
- [ ] Insight into condition
- [ ] Decreased Depression
- [ ] Improved Coping

Other: _____

Therapeutic Exercise:
- [ ] UB Theraband _____
- [ ] UB Weights _____
- [ ] LB Weights _____
- [ ] Cable Weights _____
- [ ] Coordination _____

Other: _____
Other: _____

Mobility:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Rehab Orientation: Plan of Care, Therapeutic Activities/Goals
- [ ] Ambulation
- [ ] W/C Mobility
- [x] Balance dynamic sitting balance
- [ ] Pre-Gait
- [ ] Fall Prevention
- [ ] Orthopedics
- [ ] Adaptive Equipment
- [ ] Energy Conservation
- [ ] Other

Transfers:

- [ ] Sit<>Stand
- [ ] Mat/Bed
- [ ] Wheelchair
- [ ] Other

ADL/IADL:

- [ ] Cooking
- [ ] Cleaning
- [ ] Financial Mgmt
- [ ] Medication Mgmt
- [ ] Meal Prep
- [ ] Crafts
- [ ] Eating
- [ ] Grooming
- [ ] Other

Cognitive:

- [ ] Memory
- [ ] Safety Awareness
- [ ] Judgment
- [ ] Attention/Concentration
- [ ] Communication/Social Interaction
- [ ] Discharge Planning
- [ ] Home Safety
- [ ] Precautions
- [ ] Problem-solving
- [ ] Other

Additional Treatment:

Cornhole activity w/ BUE while seated
RUE balloon tennis while seated

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Assessment

Assessment:

Pt was motivated to participate in tx session. Pt making improvements in dynamic sitting balance, hand-eye coordination, and BUE endurance. Pt benefited greatly from group session as seen by sharing personal stories/topics, talking about injury, engaging in friendly competition, and smiling/laughing throughout the course of tx. Session ended w/ pt seated in bedside chair w/ call button within reach.

Functional Goals Focused On:

Patient Making Progress In:

Needs Continued Focus On:

Education Provided/Patient's Response:

Plan

Plan:

Continue POC

Score: 0.00

Electronically Signed By:

Kennedy Kelly, OT, 03/18/2026 11:32

3/18/2026 10:36 Sylvia Valenzuela, RN Note Discipline: Case Manager Tag: Daily Note

CM submitted the order for FWW from Preferred Medical Equipment 480 446 9010 option 3 then 1.

Electronically Signed By:

Sylvia Valenzuela, RN, 03/18/2026 10:37

3/18/2026 15:26 Jose Garcia, PTA Note Discipline: Physical Therapist Assistant Tag: Daily Note

Form: PT Note (P33)

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 02:00 PM

Time Out: 02:30 PM

Total Time of Session: 30

Late Entry

- [ ] This is a late entry

Will this note include a Weekly Team Conference note?

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○ Yes ◉ No

PT Subjective

Subjective/Observations:

Patient supine in bed asleep upon entry. Agreeable for tx.

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness
Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vitals Taken

Did you take Vitals Pre-Treatment?

○ Yes ○ No

Did you take Vitals During Treatment?

○ Yes ○ No

Did you take Vitals Post-Treatment?

○ Yes ○ No

PT Objective for Daily Note

PT Mobility for Daily Note

	Assistance Level	Comment
Rolling Right		
Rolling Left		
Supine to Sit		
Sit to Supine		
Bed to Chair		
Sit to Stand		

Comments for Functional Mobility

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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PT Ambulation for Daily Note

	Distance	Device
Results		
Comments		
Gait Deviations		
Level of Assistance		

PT Ambulation for Daily Note (Cont.)

	Gait Deviations	Level of Assistance
Results		
Comments		
Gait Deviations		
Level of Assistance		

Gait Deviations:

Comments for Ambulation

PT Stairs for Daily Note

	Number of Steps	Device
Results		
Comments		

PT Stairs for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments for Stairs

Wheelchair Mobility for Daily Note

	Distance	Type
Results		
Comments		

Wheelchair Mobility for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments

Therapeutic Intervention

Therapeutic Exercise:

Therapeutic Activity:

bed mobility all SBA transfer to toilet from bed SBA with sit to stand transfer and walk to bathroom with no AD

Prosthetic/Orthotic Management

Exercise Equipment (Sci Fit / Nu Step, etc.):

Neuromuscular Re-Education:

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ambulation with head turns L and R rhythmically 2 x 100 ft
standing slow marching suit case carry in open space with CGA 1 x 50 ft
ambulation with head looking straight cues no AD x 150 ft

Gait:

FWW ambulation x 200 ft with SBA
FWW no AD x 200 ft with CGA and SBA

W/C Mobility/Management Training

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Additional LE Measurements taken?

What special tests were performed for PT

- [ ] None
- [ ] 2 Minute Walk Test
- [ ] 6 Minute Walk Test
- [ ] 10 Meter Walk Test
- [ ] Timed Up and Go (TUG)
- [ ] Berg
- [ ] Tinetti Assessment
- [ ] Activities-Specific Balance Test
- [ ] Function in Sitting Test (FIST) Results
- [ ] 5 Time Sit to Stand Test
- [ ] Amputee Mobility Predictor Assessment Tool
- [ ] Functional Gait Assessment
- [ ] Lower and Thoracic Evaluation

Other:

Additional Treatment:

PT Assessment

PT Assessment

Patient demonstrates improved dynamic standing balance as she ambulates without AD for >100 ft. Continues to have tendency to reach for furniture when walking in the hallways, however responds well to cues to slow down and walk with eyes forward. Minimal LOB occur without using AD. Will continue to benefit from continued therapy to address functional activity endurance.

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

PT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Decreased Strength
- [x] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [x] Impaired Safety Awareness
- [ ] Impaired Communication

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- [ ] Impaired Vision/Perception

DME Needed:

- [x] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers: .

PT Response to Treatment

Functional Goals Focused On:

PT Functional Progress

Needs Continued Focus On:

PT Plan of Care

PT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency:

Total number of days anticipated: 14

Patient/Family Goals:

Return to PLOF of Indep c ADLs, amb and grooming

PT Education

Education Provided On:

- [ ] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [ ] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Weight Bearing Precautions
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule

Use of Equipment:

Other:

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

- [ ] Knowledge Deficit
- [ ] Safety Awareness
- [ ] Motor Function Impairment

Other:

Family/Caregiver Training

none present.

PT Objective for Team Conference

Gait Deviations:

Score: 0.00

Electronically Signed By:

Jose Garcia, PTA, 03/18/2026 16:11

3/18/2026 16:40 Joseph Wong, MD Note Discipline: Physician Tag: Progress Note

Form: Rehab Progress Note [PPN]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

E785 - Hyperlipidemia, unspecified

I77811 - Abdominal aortic ectasia

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

Z6834 - Body mass index [BMI] 34.0-34.9, adult

Subjective

Subjective:

No acute events overnight. participating in therapies. denies pain at present. denies headaches, blurry vision.

Denies fever/chills, chest pain, SOB, cough, nausea, vomiting, diarrhea, constipation, dysuria, new weakness or changes in sensation.
Complete ROS negative unless mentioned above.

Physical Exam

Blood Pressure:

Systolic (mmHg): 153 Diastolic (mmHg): 63 Position: Sitting Side: Left

Comment:

Pulse:

B/min: 82 Position: Lying Site: Monitor Type: Regular

Comment:

Respiration:

Per Minute: 18 Vent: No

Comment:

Weight:

lb: 183.0 Scale: Bed

Comment:

Height:

Feet: 5 Inches: 1

Comment:

Physical Exam

General:

- [x] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative
- [ ] Confused
- [ ] Lethargic
- [ ] Obtunded
- [ ] Delusional
- [ ] Hallucinations
- [ ] Auditory
- [ ] Visual
- [ ] Anxious
- [ ] Oriented to person
- [ ] Oriented to place
- [ ] Oriented to time
- [ ] Oriented to situation

Comments:

Skin:

- [x] Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments:

- [ ] Abnormal

Abnormal findings:

HEENT:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: mm dry

- [ ] Abnormal

Abnormal findings:

Neck:

- [x] Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments:

- [ ] Abnormal

Abnormal findings:

Heart:

- [x] Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments:

- [ ] Abnormal

Abnormal findings:

Lungs/Chest:

- [x] Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments:

- [ ] Abnormal

Abnormal findings:

Abdomen:

- [ ] Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments:

- [x] Abnormal

Abnormal findings: obese
soft ntnd

Extremities:

- [x] Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments:

- [ ] Abnormal

Abnormal findings:

Back:

- [x] Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments:

- [ ] Abnormal

Abnormal findings:

Cranial Nerves: ii. Optic

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

- [ ] Normal

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- [ ] Abnormal

Other:

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

- [ ] Normal
- [ ] Abnormal

Other:

Cranial Nerves: xii. Hypoglossal

Can stick tongue out straight without tremors or fasciculation.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Gait and station are normal.

- [ ] Normal
- [x] Abnormal

Other:

fluent speech, follows commands
bilateral UE 5/5
bilateral LE 5-/5

Motor Functions: Romberg test is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Muscle tone is normal. No abnormal movements.

- [ ] Normal

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Abnormal

Other:

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Plantar responses are down going.

- [ ] Normal
- [ ] Abnormal

Other:

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Sensory: Sensory examination to pinprick and vibration is normal.

- [ ] Normal
- [ ] Abnormal

Other:

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed

Comment/Non-interfaced results:

Diagnostic Imaging Reviewed

Comment/Non-interfaced results:

Microbiology Tests Reviewed

Comment:

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Medical Conditions and Risks

Medical Conditions requiring close physician supervision:

- [x] Bowel & Bladder
- [x] Management of Pain
- [x] Medications
- [ ] Family Education/Counseling
- [x] Respiratory Complications
- [ ] Communication with Outside MDs

Other:

Risks being managed:

- [x] Infection
- [ ] Electrolyte Imbalance
- [x] Dehydration/Decrease Nutrition

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [x] DVT, PT/INR
- [x] Medication Management/Interactions

Other:

Functional Risks/Deficits

Functional Risks/Deficits needing close physician supervision:

- [x] Safety Awareness
- [x] Endurance for Activity
- [x] Transfers
- [ ] Bowel & Bladder Incontinence
- [x] ADL Performance
- [ ] Cognition/Language

Other:

Therapy and Nursing Focus

Continue PT/OT functional focus on:

- [x] Transfer Training
- [x] ADL Retraining
- [x] Ambulation Training
- [x] Safety Training
- [x] Balance Training
- [x] Improving ROM
- [x] Strengthening Exercises
- [x] Positioning
- [x] Fall Prevention
- [ ] Orthopedic Issues
- [x] Adaptive Equipment Training
- [ ] Caregiver Education

Other:

Continue ST functional focus on:

- [ ] NA
- [ ] Memory Strategies
- [ ] Swallowing Techniques
- [ ] Express/Comprehension
- [ ] Problem Solving
- [ ] Cognitive Retraining
- [ ] Safety Judgment
- [ ] Linguistics
- [ ] Caregiver education

Continue Rehab Nursing focus on:

- [ ] Bowel & Bladder Retraining
- [x] Pressure Relief/Skin Integrity/ Positioning
- [x] Fall Prevention
- [x] Carryover of Self-Care
- [x] Carryover of Cognitive Techniques
- [x] Med Management/Education
- [x] Carryover of Mobility Skills
- [ ] Carryover of Swallowing techniques
- [ ] Caregiver education

Other:

Barriers to Participating in Therapy

Barriers to Participating in Therapy

- [ ] None at this time
- [x] fatigue/decrease tolerance for activity
- [ ] orthostatic hypotension
- [ ] noncompliant behavior
- [x] pain
- [ ] agitation
- [ ] nausea/vomiting
- [ ] change in mental status
- [ ] altered sleep/wake cycle

Other:

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Functional and Medical Progress Since Last Note:

PT Assessment

Pt tolerated session this PM well. Pt demonstrates some difficulty with balance challenges performed this date, would benefit from continued balance training in future sessions. This session concluded with pt seated in room with son present, all needs met.

OT Assessment

See Conf Grid. Pt continues to demo dizziness

SLP Assessment

Medical Progress Since Last Note:

Current Medications

Medications:

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

Medication Changes

Medications Changes Listed and Described:

Medical Assessment & Plan (progress & goals):

Is the patient discharging within 24 hours?

○ Yes ○ No

Medical Assessment and Plan (progress & goals):

Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist

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HTN
- lisinopril

HLD
- on statin

Skin
- monitor per nursing protocol for decubitus ulcers
- frequent offloading as indicated

Bowel - constipation
- colace/senna
- lactulose, dulcolax suppository PRN

Bladder
- monitor for retention
- check PVRs

DVT ppx
- SCDs

Functional Assessment & Plan (progress & goals):

Functional Assessment and Plan (progress & goals):

Patient is making progress towards functional goals. Continue rehab plan as above. Discharge date: 3/23

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

Physician Addendum

Electronically Signed By:

Joseph Wong, MD, 03/18/2026 16:41

3/18/2026	17:29	Samer Hasson, MD	Note Discipline: Physician	Tag: Consult Note

Form: Provider Consult Note [39]

Coded Diagnoses

Admitting Diagnosis

R42 - Dizziness and giddiness

R5381 - Other malaise

I10 - Essential (primary) hypertension

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E785 - Hyperlipidemia, unspecified

I77811 - Abdominal aortic ectasia

K5900 - Constipation, unspecified

K5790 - Diverticulosis of intestine, part unspecified, without perforation or abscess without bleeding

K449 - Diaphragmatic hernia without obstruction or gangrene

E669 - Obesity, unspecified

Z6834 - Body mass index [BMI] 34.0-34.9, adult

Review of History

History of Present Illness

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/ neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

PMH/SH/FH reviewed

- [ ] No change since admission
- [ ] Changes/updates documented below

Past Medical History

Add comments to any medical history that needs further details

- [x] None
- [ ] AIDS/HIV
- [ ] A-Fib
- [ ] Allergic Rhinitis
- [ ] Alzheimer's
- [ ] Anemia
- [ ] Anxiety
- [ ] Arthritis (Osteo)
- [ ] Arthritis (Rheumatoid)
- [ ] Benign Prostate Hypertrophy
- [ ] CAD
- [ ] Cancer
- [ ] CHF
- [ ] Chronic Back Pain
- [ ] Cirrhosis
- [ ] Compression Fracture
- [ ] Dementia
- [ ] DJD
- [ ] Diabetes
- [ ] Diverticulitis/losis
- [ ] Depression
- [ ] Deep Vein Thrombosis
- [ ] Epilepsy
- [ ] GERD
- [ ] Glaucoma
- [ ] GI Bleed
- [ ] Gout
- [ ] Heart Disease
- [ ] Hiatal Hernia
- [ ] HTN
- [ ] Hyperlipidemia
- [ ] Kidney/Bladder
- [ ] Liver Disease
- [ ] Lung Disease
- [ ] Macular Degeneration
- [ ] MI (Not Primary Med Condtn)
- [ ] Obesity
- [ ] Orthopedic
- [ ] Osteoporosis
- [ ] Peptic Ulcer
- [ ] Peripheral Vascular Disease
- [ ] Pulmonary Embolis

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- [ ] Seizures
- [ ] Sleep Apnea
- [ ] Stroke
- [ ] TB History
- [ ] Thyroid Disease
- [ ] TIA

Other Medical History: _____

Additional Comments: _____

Past Surgical History

Add comments to any surgical history that needs further details

- [x] None
- [ ] AICD
- [ ] Amputation(s)
- [ ] Anesthesia difficulties
- [ ] Angioplasty
- [ ] Appendectomy
- [ ] Biopsy (site)
- [ ] Bronchoscopy
- [ ] CABG
- [ ] Carotid (CEA)
- [ ] Carpal Tunnel
- [ ] Cataract(s)
- [ ] Cholecystectomy
- [ ] Craniotomy
- [ ] Esophageal Surgery
- [ ] Gastric Surgeries (Type)
- [ ] Heart Valve Surgery
- [ ] Hemorrhoidectomy
- [ ] Hernia Repair (location)
- [ ] Hysterectomy
- [ ] Joint Replacement (location)
- [ ] Lithotripsy
- [ ] Mastectomy (R/L/B)
- [ ] Orthopedic
- [ ] Ostomies (Describe)
- [ ] Pacemaker
- [ ] PEG
- [ ] Pneumonectomy (lobes)
- [ ] Spinal Surgery
- [ ] Stent Placement (location)
- [ ] Tonsillectomy
- [ ] Thyroid Surgery
- [ ] Tracheostomy

Other Surgical History: _____

Additional Comments: _____

Family Medical History

Have any of these conditions occurred on either side of the patient's family? (Check if yes)

- [x] Not applicable
- [ ] Allergies
- [ ] Cardiovascular disease
- [ ] Gastrointestinal disease
- [ ] Pulmonary disease
- [ ] Bleeding tendencies
- [ ] Tuberculosis
- [ ] Psychiatric illness
- [ ] Bone disease
- [ ] Kidney disease
- [ ] Congenital deformities
- [ ] Alcohol/drug dependence
- [ ] Cancer/Tumors
- [ ] Diabetes

Other: _____

Comments: _____

Marital Status

Marital Status: Widowed

Current or past use of alcohol?

- ( ) Yes
- (•) No

Recreational/Illicit Drugs

- [x] Denies Use
- [ ] Past History of Use
- [ ] Current Use

Type: _____ Frequency: _____

Date last used: _____

Tobacco/Smoking

- [ ] Current Smoker

Packs/day: _____ How many years: _____

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- [x] Non-Smoker

Previous History of Smoking (indicate last time used and previous PPD): _____

- [ ] Vapes Vapes Frequency: _____
- [ ] Marijuana Marijuana Frequency: _____

Other: _____

Consult ROS

Review of System Status Update

- [ ] N/A (Admission Review of Systems)
- [x] NO CHANGES in Condition Since Last System Review.
- [ ] CHANGES HAVE Occurred & Amended Below to Reflect an Update to the Patient's Current Condition.
- [ ] Unable to Obtain Review of Systems. Reason: _____

Review of Systems

_____

Complications HEENT:

Check All that Apply.

- [ ] Jaundice
- [ ] Diplopia
- [ ] Photophobia
- [ ] Rhinitis
- [ ] Nose Bleeds
- [ ] Packed Left Nostril
- [ ] Packed Rt. Nostril
- [ ] Tooth ache
- [ ] Ear ache
- [ ] Sore throat
- [ ] Sinus tender
- [ ] Lymphadenopathy

Other: _____

- [ ] None of the Above

Complications of Respiratory System:

Check All that Apply.

- [ ] Intubated
- [ ] SOB at Rest
- [ ] SOB with Exertion
- [ ] Orthopnea
- [ ] Snores
- [ ] Hemoptysis
- [ ] Chills
- [ ] Night Sweats
- [ ] Fatigue
- [ ] Recent Weight Loss
- [ ] Ineffective & Non-Productive Cough
- [ ] Frequent Suctioning
- [ ] Persistent cough less than 2 weeks
- [ ] Persistent cough greater than 2 weeks
- [ ] Tracheostomy
- [ ] Sleep Apnea
- [ ] Oxygen Dependent

Other: _____

- [ ] None of the Above

Complications of Circulatory System:

Check all that Apply.

- [ ] HTN
- [ ] Hypotension
- [ ] Orthostatic Hypotension
- [ ] Tachycardia
- [ ] Bradycaria
- [ ] Dysrhythmia
- [ ] Edema
- [ ] History of Chest Pains at Rest
- [ ] History of Chest Pains with Activity

Other: _____

- [ ] None of the Above

Complications GI System:

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Check all that Apply.

- [ ] Vomiting in Past 72 Hours
- [ ] Diarrhea Within past 72 Hours
- [ ] Chronic Constipation
- [ ] Weight Loss
- [ ] Weight Gain
- [ ] Indigestion
- [ ] Reflux
- [ ] Rectal Bleeding
- [ ] Tarry Stools
- [ ] Rectal Tube
- [ ] Gastric Decompression
- [ ] Jaundice
- [ ] Encephalopathy

Other: _____

- [ ] None of the Above

Complications Endocrine System:

Check all that Apply.

- [ ] Episodes of Hypoglycemia
- [ ] Episodes of Hyperglycemic
- [ ] Frequent Urination
- [ ] Excessive Thirst
- [ ] Fatigue/Malaise
- [ ] Hyperactivity

Other: _____

- [ ] None of the Above

Complications Genitourinary System:

Check all that Apply.

- [ ] Incontinent
- [ ] Neurogenic Bladder
- [ ] Anuria
- [ ] Hematuria
- [ ] Cloudy
- [ ] Odor
- [ ] Urgency
- [ ] Retention
- [ ] Difficulty starting stream
- [ ] Vaginal/Penile Bleeding or Drainage

Other: _____

- [ ] None of the Above

Complications Musculoskeletal/Mobility:

Check all that Apply.

- [ ] Fall Risk
- [ ] Activity Intolerance
- [ ] Poor Balance
- [ ] Weakness/Fatigue
- [ ] Joint Swelling
- [ ] Joint Pain
- [ ] Contractures
- [ ] Dizziness
- [ ] Amputation
- [ ] Hemiplegia
- [ ] Monoplegia
- [ ] Quadrapeligia

Other: _____

- [ ] None of the Above

Complications of Pain:

Check all that Apply.

- [ ] Chronic Pain
- [ ] Intractable Pain
- [ ] Decreased Mental Status
- [ ] Agitation
- [ ] Inability to Concentrate
- [ ] Anxiety
- [ ] Nausea/Vomiting
- [ ] Constipation
- [ ] Itching

Other: _____

- [ ] None of the Above

Pain Management:

- [ ] N/A
- [ ] Heat
- [ ] Ice
- [ ] TENS Unit
- [ ] Positioning
- [ ] Biofeedback
- [ ] Medication

Comments: _____

Complications Skin/Wound:

Check all that Apply.

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Urinary Incontinence
- [ ] Fecal Incontinence
- [ ] Wound Drainage
- [ ] Erythema
- [ ] Skin Maceration
- [ ] Skin Tear
- [ ] Incision Dehiscense
- [ ] Incision Evisceration
- [ ] Stoma Care
- [ ] Poor Nutrition
- [ ] Immobility

Other: _____

- [ ] None of the Above

Complication Neuro/Mental Status:

Check all that Apply.

- [ ] Altered Mental Status
- [ ] Dementia Confusion
- [ ] Emotional Liability/Depression
- [ ] Seizures
- [ ] Headache
- [ ] Dizziness/Balance Loss
- [ ] Speech Problem
- [ ] Loss of Gag Reflex
- [ ] Loss of Memory
- [ ] Loss of Safety Awareness
- [ ] Paralysis - Location: _____

Other: _____

- [ ] None of the Above

All Recent Investigations Noted

- ○ All Recent Investigations Noted

Consult PE

Blood Pressure:

Systolic (mmHg): 153 Diastolic (mmHg): 63 Position: Sitting Side: Left

Comment: _____

Pulse:

B/min: 82 Position: Lying Site: Monitor Type: Regular

Comment: _____

Respiration:

Per Minute: 18 Vent: No

Comment: _____

Weight:

lb: 183.0 Scale: Bed

Comment: _____

Height:

Feet: 5 Inches: 1

Comment: _____

Physical Exam

HEENT: Normocephalic, atraumatic, moist oral mucosa.
NECK: Supple, No JVD, No carotid bruit, No thyromegaly.
Cardiovascular system: S1 S2 normal. Regular heart rate and rhythm. No murmurs
Respiratory system: Good air entry bilaterally in chest without any crackles. No chest wall tenderness. No use of accessory muscles.
Gastrointestinal system: Abdomen full soft, moves with respiration. No organomegaly, No tenderness. Bowel sounds present.
Musculoskeletal system: No deformities. No swelling.
Psychiatry: Good affect. Oriented to place, time and person.
Skin: No lesions, warm and dry. Wounds deferred per wound team

General:

- [ ] No complaints
- [ ] Acute distress
- [ ] Cooperative
- [ ] Uncooperative

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- ☐ Confused
- ☐ Lethargic
- ☐ Obtunded
- ☐ Delusional
- ☐ Hallucinations
- ☐ Auditory
- ☐ Visual
- ☐ Anxious
- ☐ Oriented to person
- ☐ Oriented to place
- ☐ Oriented to time
- ☐ Oriented to situation

Comments: _____

Skin:

☐ Normal: No rash, skin tears, injuries. Skin color appropriate to race, warm, dry and intact.

Comments: _____

☐ Abnormal

Abnormal findings: _____

HEENT:

☐ Normal: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EMO intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Neck:

☐ Normal: Supple. No lymphadenopathy, thyromegaly or masses. No carotid bruit. No JVD.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Heart:

☐ Normal: Regular rate and rhythm. No murmur, gallop or rub noted. No S3 or S4 heard. PMI nondisplaced.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Lungs/Chest:

☐ Normal: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Abdomen:

☐ Normal: Soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Extremities:

☐ Normal: No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally.

Comments: _____

☐ Abnormal

Abnormal findings: _____

Back:

☐ Normal: No scoliosis, kyphosis, or abnormal lordosis. No CVA tenderness.

Comments: _____

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Abnormal

Abnormal findings: _____

Cranial Nerves: ii. Optic

Distinguishes number of fingers in central field. Distinguishes movements in peripheral field.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: iii. Ocular motor, iv. Trochlear, vi. Abducens

Gazes symmetrically up, down, sideways. No diplopia. No disconjugate gaze.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: v. Trigeminal

Distinguishes 1 from 2 point touch symmetrically on forehead, cheek and chin.. Chews symmetrically. Opens mouth symmetrically. Clenched teeth force of contraction and bulk.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: Vii. Facial

Upper: Frowns symmetrically. Lower: Smile symmetrically. Both eyelids close on touching of cornea.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: vii. Auditor

Hears fingers rubbing or snapping equally in both ears. Hears watch ticking. Hears whispered voice.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: ix. Glosso-pharyngeal, x. Vagus

Has gag reflex. Says ah and uvula elevates symmetrically. Can make guttural sounds.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: xi. Accessory

Shrugs shoulders symmetrically.

☐ Normal

☐ Abnormal

Other: _____

Cranial Nerves: xii. Hypoglossal

Can stick tongue out straight without tremors or fasciculation.

☐ Normal

☐ Abnormal

Other: _____

Motor Functions: Gait and station are normal.

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Romberg test is normal.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Muscle tone is normal. No abnormal movements.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: There is no limb weakness, atrophy or fasciculation seen.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Deep tendon reflexes are 2+ and symmetrical.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Plantar responses are down going.

- ☐ Normal
- ☐ Abnormal

Other: _____

Motor Functions: Finger-to-nose and heel-to-shin is normal.

- ☐ Normal
- ☐ Abnormal

Other: _____

Sensory: Sensory examination to pinprick and vibration is normal.

- ☐ Normal
- ☐ Abnormal

Other: _____

Labs /Diagnostic Imaging Reviewed

Lab Results

Non-interfaced lab results reviewed

Comment/Non-interfaced results: _____

Diagnostic Imaging Reviewed

_____

Comment/Non-interfaced results: _____

Microbiology Tests Reviewed

Comment: _____

Decisions Made Based on Lab Values (purpose of additional tests, risks, advantages, impact on ability to hasten rehab participation and discharge):

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Consult Impression/Plan

Action Plan

☐ New Action Plan Initiated ☐ Continue Same Course of Care ☐ Updated Action Plan Initiated

Plan:

72 y.o female who was admitted to Valleywise on 3/12/26 with complaints of persistent dizziness, headaches. She reports ongoing for approximately 7 weeks, progressively worsening to the point she is unable to perform ADLs. CTA head/neck negative for acute findings. CT CAP negative for acute findings, but did show diverticulosis, small hiatal hernia, and fusiform ectasia of abdominal aorta measuring 2.6cm. She was noted to have HTN, started on lisinopril. Echo showed EF 74%. The patient was evaluated by therapies and found below functional baseline. Phillips, Priscila R was admitted to acute rehab at Reunion Phoenix on 03/13/2026.

#Debility - vertigo
- CTA head/neck, MRI brain no acute findings
- neuro consulted
- meclizine PRN
- comprehensive inpatient rehab with PT/OT, rehab nursing, CM, physiatrist
#HTN
- lisinopril
#HLD
- on statin

#DVT ppx
- SCDs

Impression

03/15/2026 seen in the common area, walking with therapy, no acute events, stable
03/16/2026: no acute issues she is resting in her room, no acute issues , no new concerns per patient or staff , vitals are stable
03/17/2026: no acute issues , she is stating she is doing well , no issues per staff , vitals are stable , all questions were addressed and answered
03/18/2025: seen in her bed, she is having headache , tylenol provided , no other complaints , will monitor , no issues per staff

Neuro/Psych Assessment and Plan

Respiratory Assessment and Plan

Cardiovascular Assessment and Plan

Renal/Genitourinary Assessment and Plan

Infectious Disease Assessment and Plan

Gastrointestinal/Stress Ulcer prophylaxis/Nutrition Assessment and Plan

DVT prophylaxis Assessment and Plan

Wounds/Pressure ulcer Assessment and Plan

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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Medications Reviewed

Acetaminophen (Tylenol) 325 MG, Give 650 mg (2 tablet(s)) by mouth Every Six Hours PRN Not to exceed more than 3 g/day from all acetaminophen doses combined Indication: headache mild pain 1-3

Aspirin (Aspirin) 81 MG, Chew 81 mg (1 tablet(s)) by mouth Daily Indication: myocardial infarction prevention

atorvastatin CALCIUM (Lipitor) 40 MG, Give 40 mg (1 tablet(s)) by mouth at Bedtime Indication: hyperlipidemia

bisacodyL (Dulcolax) 10 MG, Give 10 mg (1 suppositor(ies)) rectally Daily PRN PRN Use 3rd for Bowel Evacuation Indication: bowel evacuation

lisinopriL (Zestril) 5 MG, Give 5 mg (1 tablet(s)) by mouth Daily Hold for SBP less than 110 mmHg Indication: hypertension

Meclizine HCL - UD (Antivert) 25 MG, Give 25 mg (1 TABLET) by mouth Three Times a Day PRN Indication: vertigo

polyethylene glycoL 3350 (Miralax) 17G, Give 17 gram (1 packet(s)) by mouth Daily PRN PRN Use 1st for Bowel Evacuation Indication: constipation

Sennosides (Senokot) 8.6 MG, Give 17.2 mg (2 tablet(s)) by mouth at Bedtime Hold for Loose Stools Indication: constipation

Zinc Oxide (Zinc Oxide) 20 %, Apply OINTS. (G) topically As needed PRN Indication: skin irritation

I have Reviewed the Following Patient Data:

- ☐ Prior Medical Records & Test Results
- ☐ Medication Reconciliation
- ☐ Allergies
- ☐ Consultant Report(s)
- ☐ Current/New Medications
- ☐ Current Lab Result & Diagnostic Test
- ☐ I Have Personally Discussed Code Status with this Patient or their Surrogate Decision Maker and Believe they Understand their Medical Condition, Prognosis and the Consequences of their Code Status Decision.
- ☐ All of the Above

Anticipated Post Hospital Service Needs:

- ☐ Home with Help
- ☐ Home with Home Health
- ☐ Assisted Living Facility
- ☐ SNF
- ☐ Acute Inpatient Rehab
- ☐ Home with Outpatient Therapy
- ☐ Hospice
- ☐ Group Home
- ☐ Post Acute Vent Care Unit

Other: _____

Coordinated care with

- ☐ Consultants
- ☐ Nursing
- ☐ Nutrition
- ☐ Therapist
- ☐ Family
- ☐ Attending
- ☐ PA

Other: _____

Plan Discussed in Detail with:

- ☐ Patient
- ☐ Family
- ☐ Nursing Staff
- ☐ Case Manager

Comment: _____

Other

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Physician Attestation and Addendum

Physician Attestation

☐ I have reviewed this documentation and agree with findings. Exceptions, if any, are noted in the Addendum.

#Debility - vertigo
#HTN
#HLD
99232
Patient is Full Code

Physician Addendum

Electronically Signed By:

Samer Hasson, MD, 03/18/2026 17:30

3/18/2026	17:49	Mara Cleary, CCC-SLP	Note Discipline: Speech-Language Pathologists	Tag: Group Note

Form: SLP Rehabilitation Dining Group [GR2]

Rehab Dining Group

Initiated session (time):
12:00 PM

Duration of Session:
60

Mode of Therapy:
○ Individual ○ Concurrent ◉ Group ○ Co-Treatment

Group Therapy Rationale:
- ☐ Enhanced Motivation
- ☐ Providing Encouragement
- ☐ Confidence in Skills Learned to Achieve Therapy Goals
- ☐ Community Reintegration
- ☐ Peer Interaction
- ☐ Vicarious Learning
- ☐ Decreased Social Isolation
- ☐ Peer Communication in a Social Setting
- ☐ Improved Functional Performance by Viewing Peer Performance
- ☐ Feelings of Support by Peers in Similar Condition
- ☐ Decreased Depression
- ☐ Improved Coping

Other: _____

Patient Report (if able):

_____

Pain:
Before tx: _____
After tx: _____
Location: _____
Intervention: _____

Type Group Therapy:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Dysphagia
- [ ] Communication
- [ ] Cognitive-Linguistic
- [ ] ADL Training
- [ ] Neuro Re-Education
- [ ] Mobility

Diet Recommendations:

Foods	Drinks	Medications
Regular	Thin (0)	Regular

Diet Recommendation Comments:

Recommendations:

- [ ] Alternate Liquids/Solids
- [ ] Alter Bolus Size
- [ ] Alter Rate
- [ ] Effortful Swallows
- [ ] Supraglottic Swallo
- [ ] Chin Tuck
- [ ] Double Swallows
- [ ] Lingual Sweep
- [ ] Posture

Other:

Response to Treatment

Response to Treatment:

Patient consumed regular solids without overt s and s of aspiration 9/10 trials and with use of slow rate and amount, no talking with min assist. Patient consumed thin liquids without overt s and s of aspiration 10/10 trials with min assist to slow rate and amount..

Functional Goals Focused On:

swallowing strategies
GERD diet
dysphagia education

Patient Making Progress In:

swallowing

Needs Continued Focus On:

able to follow with min assist

Education Provided/Patient's Response:

able to follow with min assist

Score: 0.00

Electronically Signed By:

Mara Cleary, CCC-SLP, 03/18/2026 22:01

3/19/2026 6:27 Sylvia Valenzuela, RN Note Discipline: Case Manager Tag: Discharge Note

Form: Case Management Discharge Note [CM3]

CM Discharge Note

Discharge Instructions:

Discharge Date: _____

Hospital Physician Name: _____

Hospital Nurse's Station Phone Number: _____

Hospital Case Manager Phone Number: _____

If I have questions or problems, I should call:

Name: _____ Phone Number: _____

Final Discharge Summary:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Discharge Disposition:

06. Home under care of organized home health services organization

Discharge Disposition Details:

Medicare Important Message reviewed within 48 hours of discharge?

☐ Yes ☐ No ☐ N/A

Comment:

Currently Scheduled Appointments(s):

Specialty	Name	Date	Time	Address

Currently Scheduled Appointments(s): (Cont.)

Phone	Comments

Currently Scheduled Appointment(s) Comments:

Follow up appointments: ValleyWise 602 344 1300 at 2601 E. Roosevelt Street Phx

Has the Post-Discharge follow-up appointment(s) been reviewed with patient/caregiver?

☐ Yes ☐ No ☐ N/A

Comment:

Follow-Up Tests/Labs:

Test/Lab	Date	Time	Address

Follow-Up Tests/Labs: (Cont.)

Phone Number	Who Will Follow-Up

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Follow-Up Tests/Labs Comments:

Provision of Current Reconciled Medication List to Subsequenct Provider at Discharge: At the time of discharge to another provider, did your facility provide the patient's current reconciled medication list to the subsequent provider?

Route of Current Reconciled Medication List Transmission to Subsequent Provider: Indicate the route(s) of transmission of the current reconciled medication list to the subsequent provider. (Check all that apply)

- [ ] A. Electronic Health Record
- [ ] B. Health Information Exchange
- [ ] C. Verbal (e.g., in-person, telephone, video conferencing)
- [ ] D. Paper-based (e.g., fax, copies, printouts)
- [ ] E. Other Methods (e.g., texting, email, CDs)
- [ ] N/A

DME Needed:

- [x] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm
- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers: .

Reviewed how to access Medicare.gov Compare website and notified patient of their right to choose post-discharge providers and suppliers:

- ( ) Yes
- ( ) No
- ( ) N/A

Discharge Planning Process:

- [ ] Utilized teach back method in provision of education and patient able to effectively verbalize understanding of discharge planning process.
- [ ] Patient refused to participate in discharge planning process, discharged Against Medical Advice, or refused to discharge to PAC Provider based on basis of quality
- [ ] Reviewed how to access Medicare.gov Compare website and notified patient of their right to choose post-discharge providers and suppliers.

Comment:

Provided Medicare.gov lists with data directly from website based on Discharge Plan:

- [ ] Home Health Agency
- [ ] Skilled Nursing Facility
- [ ] Long-Term Acute Care Hospital
- [ ] Inpatient Rehabilitation Facility

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Patient Refused List
- [ ] N/A

Comment:

**Note: Patient was informed of hospital financial interest in:

- [ ] Not Applicable
- [ ] Home Health Agency
- [ ] Skilled Nursing Facility
- [ ] Outpatient
- [ ] Long-Term Acute Care Hospital
- [ ] Inpatient Rehabilitation Facility

Score: 0.00

Electronically Signed By:
Sylvia Valenzuela, RN, 03/19/2026 06:28

3/19/2026 7:09 Margaret M Demer, PT, DPT Note Discipline: Physical Therapist Tag: Daily Note

Form: PT Note (P33)

Session Information

Amount of the session patient attended

- (●) Entire Session
- ( ) Partial (missed some of the session)
- ( ) None (missed entire visit)

Session Time

Time In: 07:30 AM
Time Out: 08:30 AM
Total Time of Session: 60

Late Entry

- [ ] This is a late entry

Will this note include a Weekly Team Conference note?

- ( ) Yes
- (●) No

PT Subjective

Subjective/Observations:

Pt received in recliner in room, agreeable to PT session this AM. Pt denies HA/dizziness.

Precautions

Precautions:

Aspiration Precautions Every Shift, Request Type: Now, Comments: Maintain HOB elevated 90 degrees at mealtime and for 30 minutes afterwards

Yankhauer suction at bedside

Fall Precautions Every Shift, Request Type: Now

Therapy Precautions:

Fall
Dizziness

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Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

Light sensitivity
Cognitive communication deficits/Word retrieval deficits
History of esophageal dysphagia

FT: initiated with son 3/16 (MD)

Vitals Taken

Did you take Vitals Pre-Treatment?
○ Yes ○ No

Did you take Vitals During Treatment?
○ Yes ○ No

Did you take Vitals Post-Treatment?
○ Yes ○ No

PT Objective for Daily Note

PT Mobility for Daily Note

	Assistance Level	Comment
Rolling Right		
Rolling Left		
Supine to Sit		
Sit to Supine		
Bed to Chair		
Sit to Stand		

Comments for Functional Mobility

PT Ambulation for Daily Note

	Distance	Device
Results		
Comments		
Gait Deviations		
Level of Assistance		

PT Ambulation for Daily Note (Cont.)

	Gait Deviations	Level of Assistance
Results		
Comments		
Gait Deviations		
Level of Assistance		

Gait Deviations:

Comments for Ambulation

PT Stairs for Daily Note

	Number of Steps	Device
Results		
Comments		

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

PT Stairs for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments for Stairs

Wheelchair Mobility for Daily Note

	Distance	Type
Results		
Comments		

Wheelchair Mobility for Daily Note (Cont.)

	Level of Assistance
Results	
Comments	

Comments

Therapeutic Intervention

Therapeutic Exercise:

SCI fit to improve endurance L3 x10 min

Therapeutic Activity:

pt performed several STSs IND

Prosthetic/Orthotic Management

Exercise Equipment (Sci Fit / Nu Step, etc.):

Neuromuscular Re-Education:

to improve dynamic standing balance, decrease future fall risk:
-biodex maze game L1 and L2 x4 min close SBA
-OMNIVR sheep and wolf game x3 min, fox game x3 min CGA with max cueing

Gait:

pt ambulated with FWW ~250 ft x1, ~1000ft x1 SBA
pt ambulated with FWW with dual task of searching for cones and reaching OOBOS SBA ~500 ft total

W/C Mobility/Management Training

Endurance (e.g., Energy Conservation, Work Simplification, Pacing Techniques, Pursed Lip Breathing, etc.):

Additional LE Measurements taken?

What special tests were performed for PT

- ☐ None
- ☐ 2 Minute Walk Test
- ☐ 6 Minute Walk Test
- ☐ 10 Meter Walk Test
- ☐ Timed Up and Go (TUG)
- ☐ Berg
- ☐ Tinetti Assessment

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Activities-Specific Balance Test
- [ ] Function in Sitting Test (FIST) Results
- [ ] 5 Time Sit to Stand Test
- [ ] Amputee Mobility Predictor Assessment Tool
- [ ] Functional Gait Assessment
- [ ] Lower and Thoracic Evaluation

Other:

Additional Treatment:

PT Assessment

PT Assessment

Pt tolerated session this AM well. Pt able to tolerate up to 1000 ft of ambulation continuously this session, demonstrating increased ambulation tolerance. This session concluded with pt seated in recliner in room with breakfast present, call light in reach, all needs met.

Is patient being discharged in next 24 hours?

- (●) No
- ( ) Yes

PT Barriers to Discharge : (Home Environment, Equipment Needs etc.)

- [x] Decreased Strength
- [x] Decreased Motor Control
- [x] Decreased Balance/Coordination
- [x] Decreased Activity Tolerance
- [ ] Impaired Sensation
- [ ] Pain
- [x] Impaired Cognition/Memory
- [x] Impaired Safety Awareness
- [ ] Impaired Communication
- [ ] Impaired Vision/Perception

DME Needed:

- [x] None
- [ ] Walker - Standard
- [ ] Walker - Front Wheeled
- [ ] Walker- 4 Wheel w/Seat
- [ ] Walker - HD Front Wheeled
- [ ] Walker - HD 4-Wheel w/ Seat
- [ ] Platform - Right
- [ ] Platform - Left
- [ ] Hemi-Walker
- [ ] Straight Cane
- [ ] Quad Cane - Wide Base
- [ ] Quad Cane - Narrow Base
- [ ] Bedside Commode
- [ ] Drop Arm Commode
- [ ] Shower Chair with back
- [ ] Shower Chair w/o back
- [ ] Tub Transfer Bench
- [ ] Standard Wheelchair
- [ ] Light Weight Wheelchair
- [ ] Heavy Duty Wheelchair (>250 lbs)
- [ ] Reclining Wheelchair
- [ ] Motorized Wheelchair
- [ ] Custom Wheelchair
- [ ] 16"
- [ ] 18"
- [ ] 20"
- [ ] 22"
- [ ] 24"
- [ ] Anti-Tippers
- [ ] Elevating Leg Rests
- [ ] Arm Trough
- [ ] General Seat Cushion
- [ ] Pressure Relief/Reducing Cushion
- [ ] Swing Arm
- [ ] Removable Arm

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
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Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

- [ ] Sliding Board
- [ ] Hoyer
- [ ] Hospital Bed

Other: tbd

DME Providers: .

PT Response to Treatment

Functional Goals Focused On:

PT Functional Progress

Needs Continued Focus On:

PT Plan of Care

PT Intensity/Frequency

- [ ] Not required
- [ ] 1 hour/day minimum of 5 days per week
- [x] 1.5 hours/day minimum of 5 days per week

Other intensity/frequency:

Total number of days anticipated: 14

Patient/Family Goals:

Return to PLOF of Indep c ADLs, amb and grooming

PT Education

Education Provided On:

- [ ] Fall Precautions
- [ ] Goals for Inpatient Rehab / Plan of Care
- [ ] Therapy Schedule
- [ ] Safe Mobility
- [ ] Amputation Program Handbook
- [ ] SCI Program Handbook
- [ ] Stroke Program Handbook
- [ ] TBI Program Handbook
- [ ] Cardiac/Sternal Precautions
- [ ] Posterior Hip Precautions
- [ ] Pressure Relieving Technique
- [ ] Spinal Precautions
- [ ] Autonomic Dysreflexia
- [ ] Compensatory Strategies
- [ ] Energy Conservation
- [ ] Weight Bearing Precautions
- [ ] Home Exercise Program
- [ ] Orthosis management and wear schedule
- [ ] Prosthetic management and wear schedule

Use of Equipment:

Other:

Education Provided To:

- [ ] Patient
- [ ] Family
- [ ] Caregiver

Other:

Response to Education:

- [ ] Voiced Understanding
- [ ] Demonstrated Understanding
- [ ] Learner Requires More Education

Other:

Education Provided By:

- [ ] Demonstration
- [ ] Discussion
- [ ] Literature
- [ ] Video

Other:

Identified Problems/Deficits:

Name: Phillips, Priscila R	Account #: 203002787	M/R #: 9002683	Admit Date: 3/13/2026	Room: RPH200217A
Physician: Hasson, Samer	Age: 72Y	Sex: Female	Weight : 82.9 kg	Birth Date: 8/31/1953
Allergies: No Known Drug Intolerances		Diagnosis: Dizziness and giddiness		

☐ Knowledge Deficit ☐ Safety Awareness ☐ Motor Function Impairment

Other:

Family/Caregiver Training

none

PT Objective for Team Conference

Gait Deviations:

Score: 0.00

Electronically Signed By:

Margaret M Demer, PT, DPT, 03/19/2026 08:31